



STRATEGIC PLANNING MINISTRY OF HEALTH 2015-2019

DECREE OF THE MINISTER OF HEALTH OF THE REPUBLIC OF INDONESIA
NUMBER HK.02.02/MENKES/52/2015





PREFACE

MINISTER OF HEALTH OF THE REPUBLIC OF INDONESIA



The strategic plan (Renstra) of the Ministry of Health is a state document regulating the efforts on health development which are elaborated into programs/activities, indicators, targets, as well as funding and its regulation frameworks. Renstra serves as the basis in the implementation of health development.

Law Number 25/2004 on National Development Planning System mandates the Ministries/Agencies to formulate the strategic plan (Renstra) for five years periods. The Ministry of Health developed Renstra by referring to the Vision, Mission and the ninth development agenda (Nawa Cita) of the President which are stipulated in Presidential Regulation Number 2/2015 on National Medium Term Development Plan (RPJMN).

The 2015-2019 Renstra of the Ministry of Health is used as reference in planning and implementation of health development during 2015-2019, and it is applied to all stakeholders of health in the Central and Regional levels including cross-sectoral and business sector supports. The 2015-2019 Renstra of the Ministry of Health serves as the reference in planning and implementing health development within 2015-2019, and which should be carried out by all stakeholders of health sector in Central and Regional levels,



including in the cross-sectors and private sectors. Furthermore, the 2015-2019 Renstra of the Ministry of Health is elaborated into the program action plan (RAP) at Echelon I level and the activities action plan (RAK) at Echelon II level.

I would like to give my appreciation to all stakeholders that have contributed in preparing the 2015-2019 Renstra of the Ministry of Health. In this occasion, I would like to encourage to all stakeholders to synergize their efforts in health development in order to achieve the targets.

Hopefully the preparation and issuance of the 2015-2019 Renstra of the Ministry of Health would be blessed by the God Almighty. Aamiin.

Jakarta, February 6th
2015

SIGNED

The Minister of Health
of The Republic of
Indonesia,
NILA FARID MOELOEK



TABLE OF CONTENTS

Page

PREFACE	i
TABLE OF CONTENTS	v
DECREE OF THE MINISTER OF HEALTH OF THE REPUBLIC OF INDONESIA NUMBER HK.02.02/MENKES/52/2015 ON STRATEGIC PLAN OF THE MINISTRY OF HEALTH YEAR 2015-2019	vii
CHAPTER : INTRODUCTION	7
ONE	
A. Background	7
B. General Condition, Potential and Issues	9
C. Strategic Settings	31
CHAPTER : OBJECTIVES AND STRATEGIC TWO TARGETS ON THE MINISTRY OF HEALTH	43
A. Objectives	44
B. Strategic Targets	46
CHAPTER : POLICY DIRECTION, STRATEGY, THREE REGULATION AND INSTITUTIONAL FRAMEWORKS	53
A. National Policy and Strategy	53
B. Policy Direction and Strategy of the Ministry of Health	57
C. Regulation Framework	74
D. Institutional Framework	75



CHAPTER FOUR	:	PERFORMANCE TARGET AND FUNDING FRAMEWORK	79
		A. Performance Target	80
		B. Funding Framework	109
CHAPTER FIVE	:	CLOSING	113
APPENDIXES	:	I. Performance Target Matrix	119
		II. Budget Allocation Matrix	175
		III. Regulation Framework Matrix	235
		IV. List of Abbreviations	253
		V. Preparatory Team	269



**DECREE OF THE MINISTER OF HEALTH OF
THE REPUBLIC OF INDONESIA
NUMBER HK.02.02/MENKES/52/2015**

ON

**THE STRATEGIC PLAN
OF THE MINISTRY OF HEALTH
YEAR 2015-2019**





**DECREE OF THE MINISTER OF HEALTH OF
THE REPUBLIC OF INDONESIA
NUMBER HK.02.02/MENKES/52/2015
ON
THE STRATEGIC PLAN OF THE MINISTRY OF HEALTH
YEAR 2015-2019
WITH THE BLESSING OF THE GOD ALMIGHTY
MINISTER OF HEALTH OF THE REPUBLIC OF
INDONESIA,**

- Considering:
- a. that in order to achieve the national development goals in health sector in relevant to the mandate of Law number 25/2004 on National Development Planning System, it is required to prepare the Strategic Plan of the Ministry of Health;
 - b. the development of policies set up by the Ministry of Health in order to ensure the health of the people at the highest degree, the it is require to establish the goal, policy and strategy in the Strategic Plan of the Ministry of Health 2015-2019;
 - c. that the strategic plan aforementioned in point a and by has been developed as one indicative planning document that contains development programs in health sector which will be carried out by the Ministry of Health;



- d. based on the consideration stated in point a,b and c, it is necessary to stipulate the Decree of the Minister of Health on the Strategic Plan of the Ministry of Health 2015-2019;

In view of:

1. Law Number 25/2004 on National Development Planning System (State Gazette 2004 No. 104, Supplement to State Gazette No. 4421);
2. Law number 17/2007 on National Long-Term Development Plan 2005-2025 (State Gazette 2007 No. 33, Supplement to State Gazette Number 4700);
3. Law Number 36/2009 on Health (State Gazette 2009 No. 144, Supplement to State Gazette Number 5063);
4. Law Number 23/2014 on Regional Government (State Gazette of the Republic of Indonesia 2014 Number 244, Supplement to State Gazette of the Republic of Indonesia Number 5587);
5. Presidential Regulation Number 47/2009 on Position, Task, Function, Organizational Structure, and Procedure of the Ministry of Health of the Republic of Indonesia;
6. Presidential Regulation Number 24/2010 on Position, Task and Function of the State Ministry as well as the Organizational Structure, Task and Function of the Echelon I of the State



Ministry as it has been amended several times by the Presidential Regulation Number 135/2014;

7. Presidential Regulation Number 72/2012 on National Health System (State Gazette of the Republic of Indonesia 2012 Number 193);
8. Presidential Regulation Number 165/2014 on the Structuring of Task and Function of the Working Cabinet (State Gazette of the Republic of Indonesia 2014 Number 339);
9. Presidential Regulation Number 2/2015 on National Medium-Term Development Plan (State Gazette of the Republic of Indonesia 2015 Number 3);
10. Regulation of the Minister of Health Number 1144/Menkes/Per/VIII/2010 on Organization and Working Procedure of the Ministry of Health (Official Gazette of the Republic of Indonesia 2010 Number 585), as amended by the Regulation of the Minister of Health Number 35/2013 (Official Gazette of the Republic of Indonesia 2013 Number 741);

DECIDES:

Stipulate : The Decree of the Minister of Health on Strategic Plan of the Ministry of Health 2015-2019.



- FIRST : Strategic Plan of the Ministry of Health 2015-2019 is stated in Appendix I to Appendix III which is an inseparable part of this Ministerial Decree.
- SECOND : Strategic Plan of the Ministry of Health 2015-2019 as stated in the First Dictum that is used as reference by the Ministry of Health for annual planning and implementation of health development program.
- THIRD : This Ministerial Decree comes into force on the date it is stipulated.

Jakarta, February 6th 2015

THE MINISTER OF HEALTH
OF THE REPUBLIC OF INDONESIA,

[SIGNED]

NILA FARID MOELOEK



APPENDIXES

DECREE OF THE MINISTER OF HEALTH NUMBER HK.02.02/MENKES/52/2015 ON STRATEGIC PLAN OF THE MINISTRY OF HEALTH 2015-2019





APPENDIXES
DECREE OF THE MINISTER OF HEALTH
NUMBER HK.02.02/MENKES/52/2015 ON
STRATEGIC PLAN OF THE MINISTRY OF
HEALTH 2015-2019

**THE STRATEGIC PLAN OF THE MINISTRY OF HEALTH
YEAR 2015-2019**

**CHAPTER ONE
INTRODUCTION**

A. BACKGROUND

The essential of Indonesia's health development is the multifaceted efforts which are implemented by all components of the Nation that aims to raise the awareness, willingness and to increase the capability of all Indonesian to live a healthy life at the highest level. It is also considered as the investment for a productive human resources development socially and economically. Its successful results may be determined by the alignment of efforts within program and cross-sectoral program and other sustainable efforts which have been executed in the previous period. Therefore, there is a substantial need for developing a sustainable health development planning.

According to the Law Number 25/2004 on National Development Planning System (SPPN) mandates that every line ministries should formulate their Strategic Plan (Renstra) which refers to the National Medium Term Development Plan (RPJMN). As the stipulation of RPJMN



2015-2019 was finalized, furthermore, the Ministry of Health need to develop a Renstra of 2015-2019 period. It is an indicative planning document which comprises the programs of health development that will be carried out by the Ministry of Health and should be the reference in preparing the annual planning. It is also prepared by utilizing the technocratic, political, participative, top-down and bottom-up approaches.

The health development of 2015-2019 is named as the Healthy Indonesia Program which targets to improve the health outcomes and nutrition status of all Indonesians through efforts in healthcare provision and community empowerment that are supported by both financial protection and healthcare equalization. the Principle Targets of RPJMN 2015-2019 are: (1) the improvement of health outcomes and nutrition status of mothers and children; (2) the enhancement of disease control; (3) an increased access and quality of primary and referral healthcare, especially in the remote, underdeveloped and border areas; (4) a wider coverage of universal healthcare through the implementation of Kartu Indonesia Sehat (Healthy Indonesia Card) and advanced quality of National Social Security System's management, (5) the fulfillment of needs for human resources on health, medicines, and vaccines; and (6) the escalation of health system' responsiveness.

The implementation of Healthy Indonesia Program is based on 3 main pillars, i.e. healthy paradigm, the strengthening of healthcare provision and national health insurance: 1) the healthy paradigm pillar is executed by



deploying the strategy of mainstreaming health sector in the national development, strengthening the promotive and preventive intervention and community empowerment; 2) strengthening healthcare provision is conducted by applying the strategy of improving the access to healthcare provision, optimizing the referral system and improving the quality of healthcare provision, as well as exercising the continuum of care approach and health risk-based intervention; 3) the national health insurance is implemented by using the strategy of expanding the targets, benefits, quality and cost control.

B. GENERAL CONDITION, POTENTIALS AND ISSUES

The overview of general condition, potentials and issues in health development is presented based on the results of health program outcomes, condition of strategic environment, population, education, poverty and other recent progress. Potentials and issues in health development will be considered as inputs in determining policy direction and strategy of the Ministry of Health.

1. Healthcare Provision

Maternal and Child Health. Maternal mortality rate has decreased slightly, nevertheless, it is still far from the MDGs target of 2015, and despite of the number of births attended by human resource on health has increased. This condition might be caused by some factors such as the poor quality of maternal healthcare provision, unfit condition of the pregnant mothers and other determinant factors. The main causes of maternal mortality are



hypertension during pregnancy and post-partum hemorrhage. These could be minimized if the quality of antenatal care provision is properly executed.

Some circumstances that might lead to the poor maternal health condition are the complication intervention, anemia, pregnant mothers associated with diseases or problems such as diabetes, hypertension, malaria and the four main risk factors (young pregnant mother aged <20 years, older pregnant mothers aged >35 years, having short period e.g. 2 years between two pregnancies and having excessive children >3 per years). There is 54.2 per 1,000 young women aged under 20 years has deliver live births, while older women aged above 40 years old who has giving birth is 207 per 1,000 live births. The number has been confirmed and corroborated with a dataset that depicts the fact that the first marriage held in an early stage of womanhood (aged <20 year old), it is accounted for a 46.7% of total married women population.

Several potentials and challenges of the maternal and child mortality rate reduction program is that the number of deployed human resources on health for the maternal health provision especially midwives has been relatively spread out across Indonesia's regions, however, their competencies are not sufficient according to the existing standard. In quantity' aspect, the number of health centers with Basic Emergency Neonatal Obstetric Care (BENOC/PONED) and Comprehensive Emergency Neonatal Obstetric Care'



(CENOC/PONEK) hospitals have been increased but those increment do not in line with the improvement of service quality. The improvement of pre-pregnancy stage of maternal health condition especially during at adolescents period is became as an important factor in lowering the maternal and infant mortality rates.

A significant numbers of family planning (FP/KB) participants are recognized as another potential in reducing the maternal deaths, nevertheless, they need to be promoted for a long term use of contraception in order to have an effective results. The diversity of food options is also potentially able to enhance the nutritional status of pregnant mothers, thus, the supplementary food scheme which contains high calories, protein and micronutrient should be developed.

Under-five and Infant Mortality within the last five years. The Neonatal Mortality Rate (NMR/AKN) remains the same, at 19/1000 of live births. Meanwhile, There was a slightly Post Neonatal Mortality Rate (PNMR/AKPN) reduction from 15/1,000 to 13/1,000 live births, the under-five years infant mortality rate is declined from 44/1,000 to 40/1,000 live births. The causes of death in perinatal group are the intra uterine fetal death (IUFD) of 29.5% and underweight newborn (BBLR) of 11.2%, these figures imply that mother's condition before and during pregnancy affect the condition of her baby. The next challenge would be on how to prepare the expectant mothers in anticipating their pregnancy, and deliver



their baby as well as to secure a healthy environment for protecting babies from any infections.

The major causes of death for neonatal to one-year old group are any infection occurred, especially pneumonia and diarrhea. It's closely related to the mother's healthy lifestyle and her surroundings.

School Age and Adolescent. The leading causes of death at this age are transportation accident, dengue fever and tuberculosis. Other health problems are tobacco consumption and early marriages (10-15 years old) which are consist of 0.1% boys and 0.2% girls.

In respect to adolescent nutrition status, based on Basic Health Research (Riskesdas) 2010, the prevalence of stunting rate of adolescent aged 13-15 years (moderate and severe level) is 35.2% nationally and 31.2% for adolescent aged 16-18 years. About half of the total adolescent is having any energy deficiencies and one third of the total adolescent suffers from protein and micronutrient deficiencies.

The implementation of Occupational Health School (OHS/UKS) should be made compulsory at schools and madrasah starting from Kindergarten/Islamic School (TK/RA) to Senior High School/Vocational Senior High School/Islamic Senior High School (SMA/SMK/MA), considering that UKS possess a role in promoting all health-related matters. This platform is very important and strategic due to its administration brings effective and efficient outcomes



and also it results a multiplier effect. UKS should become a compulsory intervention in Health centers. Moreover, they need to increase the quantity and quality of Adolescent Healthcare' (AH/PKPR) provision that reaches teenagers at and out-of-school. Some priorities of the program are nutritional improvement of the school-age children, reproductive health and early detection of non-communicable diseases.

Occupational Age and Elderly People. Besides the non-communicable diseases that threaten the occupational age group, work-related illness and occupational accidents are also increasing. The total mortality due to occupational accidents was increasing to almost 10% for the last 5 years. The occupational accidents mostly occurred on the age group of 31-45 years old. Thus, the occupational-age group healthcare' program should become the priority, in order to control the risk factors at the beginning phase. The priority for occupational-age healthcare comprises developing primary healthcare and implementing occupational health and safety in workplaces, as well as establishing Occupational Healthcare Post as one of the forms of Community based Healthcare (CBH/UKBM) for workers and improving the healthcare for the vulnerable one, for instance, fishermen, Indonesian migrant workers (IMW/TKI) and female workers.

Nutrition of the Community. The progress of nutrition related issues in Indonesia are getting more complicated right now, besides the under nutrition, over nutrition is another problem that we must address



seriously as well. In the 2010-2014 National Medium Term Development Plan, the improvement of nutrition status of community becomes one of the priorities, by lowering the prevalence of underweight infant to 15% and prevalence of stunting infant to 32% by 2014. The Riskesdas' results within 2007 to 2013 points out some alarming facts where underweight rates increased from 18.4% to 19.6%, stunting rates also increased from 36.8% to 37.2%, while wasting rates dropped from 13.6% to 12.1%. Riskesdas 2010 and 2013 shows that underweight newborn (BBLR) <2,500 gram decreased from 11.1% to 10.2%. Stunting is induced by chronic malnutrition due to poverty and poor parenting method, it impairs the cognitive development, triggers someone to get sick easily and it leads to low competitiveness that consequently frame people into poverty lines. The first 1,000 days of life of a child is the critical times that may shape their future, and during this period, Indonesian children would face several serious growth disorders. The problem is, beyond a thousand days, the effect from a malnutrition status will be extremely difficult to be treated. To overcome stunting-related issues, the community should be educated to comprehend the importance of an adequate nutrition for pregnant mother and infant. Actively engaged in the global commitment (SUN-Scaling up Nutrition) in reducing stunting, Indonesia now focuses on the first 1,000 days of life from conception to the second birthday. In order to cope with stunting in an integrated way because malnutrition-related problems cannot be solved solely by a health sector (specific intervention) but it requires



several external sectors involvement (sensitive intervention). This is stipulated in the Government Regulation Number 42/2013 on National Movement to Accelerate Nutrition Improvement.

The increased prevalence of obesity happens not only to under-five but to adult people as well. It was proven by the increasing prevalence of central obesity (waist circumference >90 cm for men and >80 cm for women) from 2007 to 2013 throughout the provinces. The highest prevalence in 2013 is the Jakarta Province (39.7%) 2.5 times doubled compare to the lowest prevalence rate in East Nusa Tenggara Province (15.2%). The prevalence of central obesity is increasing in all provinces but the increment was also varies, the highest ones were in Jakarta, Maluku and South Sumatera Provinces. By examining these facts, a balanced and proactive nutrition education along with a clean and healthy lifestyle (PHBS) becomes compulsory to be implemented in the community.

Communicable Diseases. The priorities for communicable diseases still focus on HIV/AIDS, tuberculosis, malaria, dengue fever, influenza and avian flu. Moreover, Indonesia has not completely successful in controlling the neglected diseases like leprosy, Lymphatic filariasis, leptospirosis, and so forth. The morbidity and mortality rates were caused by communicable diseases like polio, measles, diphtheria, pertusis, hepatitis B, and tetanus which all can be prevented by immunization on maternal or neonatal have been substantially decreased; Indonesia



had declared polio-free in 2014.

The prevalence of HIV cases on age group of 15-49 years old tends to increase. In early 2009, the prevalence of HIV cases on this age group was only 0.16% and increased to 0.30% in 2011 it increased again to 0.32% in 2012, and keep increased to 0.43% in 2013. The AIDS' CFR rate was also decreased from 13.65% in 2004 to 0.85% in 2013.

Some potentials that Indonesia owned in controlling HIV-AIDS are having good preparation, including procedures to treat patient, HRH, healthcare providers (especially hospital), and health laboratories. There are at least four laboratories that have been accredited with biosafety level 3 (BSL 3), which consist of the laboratory of Health Research and Development Agency, the Institute of Human Virology and Cancer Biology (IHVCB) of Indonesia University, the Institute of Tropical Diseases of Airlangga University, and Eijkman Institute for Molecular Biology.

Existing hard works were successfully brought Indonesia to be the first country in South East Asia region that achieved the global TB target. The achievements were the Crude Detection Rate/CDR above 70% and Treatment Success Rate/TSR above 85% in 2006. In the RPJMN 2015-2019, Indonesia insists to utilize the TB' prevalence as 272 per 100,000 people absolutely (680,000 patients with TB) and the survey findings of TB prevalence of 2013-2014 which aims to compute the prevalence of pulmonary TB with a bacteriological confirmation on an Indonesian



population aged above 15 years. The findings are: 1) sputum smear-positive pulmonary TB prevalence per 100,000 people aged over 15 years was 257 (confidence intervals at 95% level were 210-303); 2) the prevalence of pulmonary TB with a bacteriological confirmation per 100,000 people aged 15 years was 759 (confidence intervals at 95% level were 590-961); 3) the prevalence of pulmonary TB with a bacteriological confirmation in all ages per 100,000 people was 601 (confidence intervals at 95% level were 466-758) and 4) the prevalence of pulmonary TB in all form for all ages per 100,000 people was 660 (confidence intervals at 95% level were 523-813), it is estimated that there are 1,600,000 Indonesian people with TB (confidence intervals at 95% level are 1,300,000 - 2,000,000)

Control on communicable diseases such as malaria which is part of the global commitment also presents a pretty good achievement. Annual Parasite Incidence (API), the indicator to measure the impact of malaria control, shows a decreasing tendency from year to year. Nationally, the malaria case from 2005-2012 tends to decrease, in 1990 the API was 4.69 per 1,000 population and it declined to 1.38 per 1,000 in 2013 and by 2014, it is expected to attain MDG's target, $API < 1$ per 1,000 population. The initial index in 2009 was 1.85% and it decreased to 1.75% in 2011, and continued to decrease to 1.69% by 2012, and it got down to 1.38% in 2013, which is closer to the target of 1% by 2014.



As for DBD, the national target of DBD morbidity rate in 2012 was 53 per 100,000 population or lower. In 2013, it was 45.85 per 100,000 population which means beyond the specified target. The DBD mortality rate has also diminished, where in 1968 the CFR rate was 41.30% and it was 0.77% in 2013.

In order to mitigate outbreaks (KLB) of any communicable diseases, a system namely the Early Warning and Response System (EWARS) or in Indonesian is called as *Sistem Kewaspadaan Dini dan Respon* (SKDR) has been developed; this is a reinforcement form for the Early Warning System - Extraordinary Incident (SKD-KLB). It is expected that by exercising EWARS, there will be an improvement in early detection and response against the increasing trend of diseases cases particularly those that are potential to promote KLB.

In the last couple of decades, a number of new diseases emerged and some of them were succeeded to spread in Indonesia, for example, SARS and avian flu. While in Middle East Countries there has been an outbreak of MERS diseases, and Ebola outbreak strikes Africa. These emerging diseases are mostly provoked by virus, whereas it was originated in animals but it was finally contracted to human. Some of which have evolved to a communicable diseases transmitted from human to human.



Non-Communicable Diseases. The communicable diseases tend to continuously spreading out and have posed a threat from the early age. For the last two decades, there has been a significant epidemiological transition, the non-communicable diseases have become the major burden with the communicable diseases remains as threat. Indonesia is facing double burden diseases, suffering from non-communicable diseases and communicable diseases at once. The major non-communicable diseases include hypertension, diabetes mellitus, cancer and chronic obstructive pulmonary disease (COPD). Smoking' death toll keeps increasing from 41.75% in 1995 to 59.7% in 2007. In addition to that the 2006 national economic survey stated that the poor population spent 12.6% of their income for smoking.

Therefore, an early detection should be conducted proactively by approaching the targets, hence, most of people do not aware that they suffer from non-communicable diseases. The non-communicable diseases (PTM) control intervention comprises the establishment of integrated development post for Non-Communicable Diseases Control (Posbindu-PTM) to monitor and detect the risk factors of non-communicable diseases among community. Since its development in 2011 Posbindu-PTM has developed to 7,225 Posbindu throughout Indonesia in 2013.

Environmental Health. The intervention also shows a sufficient result. The percentage of household with



access to drinkable water is increased from 47.7% in 2009 to 55.04% in 2011. This number decreased to 41.66% in 2012 but it increased to 66.8% in 2013. It is almost meet the target of 68% by 2014.

In 2013, the proportion of household with sustainable access to drinkable water is escalated to 59.8% from 45.1% in 2010; access to improved basic sanitation is also increased from 55.5% in 2010 to 66.8% in 2013. Additionally, the village' developments that implement the Community Based Total Sanitation (STBM) in order to improve environmental sanitation is also experienced a continuous improvement.

Mental Health. There are some problems of mental health which are considered as a significant health issue and posed a huge health burden. Based on Riskesdas 2013, the prevalence of emotional and mental disorder (symptoms of depressions and anxiety disorder), is 6% for age group of 15 years old and above. It means that more than 14 million people in Indonesia suffer from emotional and mental disorders. Meanwhile, the prevalence of severe mental disorders like psychosis disorder is 1.7 per 1,000 populations. It indicates that more than 400,000 people suffer from severe mental disorder (psychotic). The percentage of shackled people with severe mental disorder is 14.3% or in other words, there are about 57,000 of shackled mentally ill cases.

Mental disorder and narcotics, psychotropic, and addictive substances' abuses (Napza) are related to a



self-harming behavior such as suicides. Based on the report from Indonesian National Police in 2012 reveals the number of suicides around 0.5% from 100,000 population, it conveys that there are approximately 1,170 cases of suicides reported in a year. The Priority of mental health intervention is a development of community based mental health program (UKJBM) where the main essence are Puskesmas, collaborative efforts with the community to prevent the increasing cases of mental illness.

Access and Quality of Health Services. From 2009 to 2013, the amount of Puskesmas is increased although the annual growth rate is not high 3-3.5%. In 2009, there were 8,737 Puskesmas (3.74 per 100,000 population), it increased up to 9,655 (3.89 per 100,000 population) in 2013. From that number some of those were Puskesmas with inpatient care, which were also increased from 2,704 in 2009 to 3,317 in 2013. The Rifaskes data in 2011 depicts that there were 2,492 Puskesmas located in remote and very remote areas across 353 districts/cities.

Total numbers of General Hospitals (RSU) and Specialized Hospitals (RSK) along with the beds (TT) are also increased. In 2009, there were 1,202 RSU with 141,603 TT, then, it increased to 1,725 RSU with 245,340 TT in 2013. In 2013 most of 53% RSU are privates (profitable and non-profitable hospital), and 30.4% owned by district/city governments. Similar improvement was also occurred in RSK, from 321 RSK with 22,877 TT in 2009, it increased to 503 RSK with



33,110 TT in 2013. In 2013, more than half of RSK 51.3% consist of Maternity Hospitals, Maternity and Children Hospitals. Data on October 2014 illustrates that currently there are 2,368 hospitals and it is predicted that the number will increase to 2,809 by 2017, with the growth rate of 147 hospitals per year. Based on the readiness of primary healthcare in Puskesmas, Rifaskes data 2011 portrays an unsatisfactory outcome. The total percentage of admitted patients per 10,000 populations was only 1.9%. The average of bed occupancy rate (BOR) was only 65%. There were only 25% of district/city hospitals that provides comprehensive emergency neonatal obstetric care; and the readiness of state hospitals in delivering comprehensive emergency neonatal obstetric care was only 86%. Furthermore, capability of hospital in delivering blood transfusion service in general is remained low (in average 55%), particularly sufficient blood stock component (41% State Hospital and 13% Private Hospital).

The readiness of primary healthcare in Puskesmas only reached 71%, basic emergency neonatal obstetric care met 62% and non-communicable diseases care attained 79%. The poor readiness of supply side is mainly caused by the insufficiency of facilities; shortage of medicines supply, medical means and equipments; unequal distribution of HRH; and inadequate healthcare quality. In Puskesmas, the readiness of basic equipments is quite high at 84% but the capability to determine a diagnosis is still low at 61%. Some of these low capabilities that need to be improved



are pregnancy test (47%), glucose urine test (47%) and blood glucose test (54%). There are only 24% of Puskesmas which able to deliver the complete diagnosis components.

2. Community Empowerment in the Health Sector.

The percentages of households that practice clean and healthy lifestyle was increased from 50.1% (2010) to 53.9% (2011), and 56.5% (2012), it slightly decreased to 55.0% (2013). As the 2014 target was 70% then the 2013 outcome' attainment seems far from expectation. The active and alert villages program was also improved from 16% (2010) to 32.3% (2011), 65.3% (2012), and 67.1% (2013). The target in 2014 was 70%, thus, the achievement in 2013 was pretty close to the specified target. The numbers of operating Village Health Posts (Poskesdes) are also enhanced from 52,279 (2010) to 52,850 (2011), 54,142 (2012) and to 54,731 (2013). The target of 2014 is 58,500; so based on these outcomes there are still 45% of households that have not implemented clean and healthy lifestyle; around 30% of alert villages (desa siaga) program are not active yet, and around 13, 500 (18.75%) of Poskesdes are not operating (it is assumed that there are 72,000 of Poskesdes). There is a quite significant change in household members' defecation behavior, a group aged ≥ 10 years has exercising an appropriate defecation behavior was accounted for 71.1% in 2007, and then, it escalated to 82.6% in 2013. However, it indicates that there is still around 17.4% of household members' aged ≥ 10 years old applying the opposite one.



The reason for health promotion and community empowerment intervention was not optimum is the limited capacity of health promotion in the sub-national level, due to lack of health promotion staffs. Based on 2011 Rifaskes report, there is only 4,144 public health counselors across Indonesia.

They are spread over 3,085 Puskesmas (34.4%). In average, there is 0.46 health promotion staff per Puskesmas; and only 1% of them have health education background/health promotion training.

3. Accessibility and Quality of Pharmaceutical and Medical Devices Stocks.

Drug accessibility is determined by the supplies of drugs for healthcare provision. In 2013, medicines and vaccines stocks level reach 96.82%, increased from 92.5% in the previous year. Nevertheless, the stocks were not well-distributed throughout all provinces. Data of 2012 describes that there were three provinces with availability level below 80%, and there were six provinces with drug availability level over 100%. The disparity reflects poor logistics management of drugs and vaccines. Consequently, the use of online logistic management system needs to be promoted, including a flexible and accountable relocation scheme for drugs and vaccines between province/district/city.

During 2010-2014, an intervention to improve logistic management for drugs and vaccines has been started; it involved the implementation of e-catalog and



initiation of e-logistic for drugs. In 2013, e-catalog has been utilized by 432 Health Offices at province/district/city level and also by State Hospitals; it saved drug supplies budget utilization up to 30%. Until 2013, there were 405 pharmacies facilities at the district/city level which adopt e-logistic application. The use of e-logistic facilitates a real-time monitoring on drugs and vaccines supplies, and eases the supply management in implementing health program.

Although the supplies of drugs and vaccines are quite sufficient, the pharmacies services at healthcare facilities have not fulfill the standard. In 2013, there were only 35.15% Puskesmas and 41.72 Hospitals' Pharmacy Installations offer standardized pharmaceutical services. The utilization of generic drugs is quite substantial, however, the use of rational drugs at healthcare facilities were only 61.9%. This fact is particularly triggered by the low implementation of formulary and guideline on drug use rationally. In the other side, the community who understand the details use and benefit of generic drugs is still very weak, especially, 17.4% in rural areas and 46.1% in urban areas. Public knowledge on drugs in general is still below expectation; evidently around 35% of households were storing antibiotics without any physician's prescription (Riskesdas 2013).

The implementation of National Health Insurance has the potential to increase the need for essential medicines and medical equipments. In order to increase the supply of drugs and medical equipments



that is safe, contained with good quality, and efficacious, the government has developed the National Formulary and e-catalog to ensure the implementation of rational drug use. The concept of Essential Medicines is applied on National Formulary as reference for healthcare provision, so that pharmaceutical service can be a cost-effective one.

The percentage of drugs that meet quality standards, and are efficacious and safe continues to increase and by 2011 attained 96.79%. Meanwhile, the household health supplies (PKRT) that meet security requirement, quality and benefit standards continues to improve and by 2013 it reached 90.12% (2013).

In the meantime, the quality of drugs production' facilities, other pharmaceutical products, medical devices and food are generally still inadequate due to ineffective monitoring and assistance. In 2013, there were only 67.8% of drugs production facilities and only 78.18% of medical devices and PKRT which hold the latest certified Good Manufacturing Practices and met the appropriate production methods. Poor quality of drugs is still exacerbated by the high drug price due to inefficient distribution chain and imported medicinal raw materials.

Imported medicinal raw materials, other pharmaceutical products and medical devices lead to a lack of self-reliance in healthcare provision. Almost 90% of national drug's need could be fulfilled by domestic production. However, pharmacy industry still



relies on imported medicinal raw materials. Around 96% of raw materials deployed by pharmacy industry are imported. The raw material component contributes to 25-30% total cost of drug production, thus, some interventions on this component will affect medicine price.

From a natural resources aspect, Indonesia is rich with medicinal plants. The result of Research on Medicinal Plants and Herbs (Ristoja) 2012 which only covers 20% of the archipelago, found 1, 740 species of medicinal plants. If the government support consistently for national local-reliance on drugs production, several dedicated researchers would be able to generate raw materials for drugs from our own land. The history of local-reliance on drugs' raw materials demonstrated the major role of health regulation and strong cross-sector commitment in gaining successful results. During 1982-1990, the production of paracetamol had 100% protection from the government. As a consequence, the priorities that should be carried out include not only the development of e-catalog and e-logistic, but also the local-reliance of drugs raw materials.

- 4. Human Resource for Health (HRH).** In 2012, there were 707,234 HRHs, whom were increased to 877,088 in 2013; who around 40% of them work in Puskesmas. The numbers of HRHs are quite sufficient but the deployment is not equally distributed. In addition, the composition of types of deployed HRHs in Puskesmas is not balanced. Most of HRHs in Puskesmas are



medical staffs (9.37 person per Puskesmas), nurse-including dental nurses (13 person per Puskesmas), midwives (10.6 person per Puskesmas). Meanwhile, there are only 2.3 public health workers, 1.1 sanitarians, and 0.9 nutritionists per Puskesmas. Rifaskes also reveals that there are only 0.46 health counselors per Puskesmas.

Some hospitals experiences shortcomings in HRH's recruitment which are considered as a challenge in improving healthcare provision. In 2013, there were 29% pediatric specialists, 27% obgyn specialists, 32% surgical specialists and 33% geriatric specialists. There were 88,309 general practitioners that hold the Letter of Registration (STR), thus, the ratio for general practitioners is 3.61 doctors per 10,000 populations. Meanwhile, the ideal number according to WHO recommendation is ten doctors per 10,000 populations. Additionally, the quality of the recent graduates on HRH is not yet satisfying. The percentage of HRHs that passed competency test is still inadequate, 71.3% for physicians, 76% for dentists, 63% of nurses, 67.5% of vocational nurses (D3 Keperawatan) and 53.5% of vocational midwives.

5. Research and Development. Research and development in health sector is directed on researches that provide information to support health program in the form of studies, national health researches, regular monitoring, product oriented breakthrough researches and research on development and network. This intervention is reflected from several breakthrough



researches, such as Basic Health Research (Riskesdas), Health Facility Research (Rifaskes), Research on Medicinal Plants and Herbs (Ristoja), Specific Research on Environmental Pollution (Rikus Cemarling), Research on Health Culture, Cohort Study on Growth and Non-Communicable Diseases (PTM), Research on registration of diseases and study on total diet.

6. Health Sector Financing. Budget allocation for health sector from state budget (APBN) or local budget (APBD) has not complied the mandate of Law number 36/2009 on Health, which is 5% of APBN and 10% of APBD (outside salary) needs to be allocated by both central and local government. The budget of Ministry of Health in the last couple of years has shown an increasing trend. In 2008, the Ministry of Health received budget allocation from the State Budget of IDR 18.55 trillion, and the allocation keeps increasing for the following years. In 2009, the budget allocation of Ministry of Health was 20.93 trillion and escalated to IDR 38.61 trillion in 2013, and rose to IDR 46.459 trillion in 2014. The increment in 2014 is allocated for the implementation of National Health Insurance, while health intervention funds have been declined. Although the budget allocation has been increased, the proportion is relatively unchanged, around 2.5%.

Health development is funded not only by the Ministry of Health but also by local budget. Law Number 36/2009 on Health mandates the Local Governments (province, district, and city) to allocate a minimum of 10% from their Regional Budget (outside employee's



salary) for health development. However the allocation only reaches 9.37% in general, with some provinces were able to allocate up to 10-16% in 2012. Several new provinces can only allocate 2-8% of their local budget for health development and it still includes employee's salary. A better condition can be found at district/city level where 221 (42.2%) of district/city has allocated >10% of their local budget for health.

Besides that, a specific effort to help the local government of district/city levels in improving the access and equality to public health interventions through Puskesmas, the Ministry of Health as the representative of the government initiates another fund channeling known as the Health Operational Assistance fund (BOK). The utilization of this fund is focused on some promotive and preventive health interventions, for instance maternal and child health-family planning (MCH-FP/KIA-KB), immunization, improvement of public health nutrition, diseases control, etc. which are in line with the Minimum Standard of Service and the MDGs on the health sector.

The problem in the budgeting area is the allocation for curative and rehabilitative measures is way higher than the allocation for promotive and preventive measures; in fact, the last mentioned measures actually aims to maintain and improve the wellbeing and healthy states of the community and avoid them from falling sick easily. This situation is potential in leading to the allocative inefficiency of health

intervention.

- 7. Management, Regulation and Health Information System.** The implementation of health planning at the Ministry of Health basically has executed well which highlighted by the use of IT on e-planning, e-budgeting and e-money system. The challenges in health planning are the lack of adequate data and information that fits into the needs and timeframe; no mechanism that can guarantee the harmony and alignment between the plan and budget of the Ministry of Health with the plan and budget of relevant ministries/agencies and the Local Government or Pemda (district, city and province); lastly, the utilization of evaluation results or studies as input for planning preparation process.

Regarding the regulation, there were lots of various laws, presidential regulations, and regulations of Minister of Health have been stipulated to reinforce the equal distributed on HRHs, health financing, community empowerment, planning and health information system, local-reliance on medicine, vaccines supplies and medical equipments, as well as the implementation of national health insurance (JKN) and other health interventions.

C. STRATEGIC SETTINGS

1. National Strategic Settings

Population Growth. The growth of Indonesian people is marked by the emergence of window opportunity whereas the positive dependency ratio. It is the number



of productive age population that exceeds the non-productive part, the culmination time will take place in 2030. The total Indonesian population in 2015 is 256,461,700 people, with the growth rate of 1.19% per year, the population in 2019 will increase to 268,074,600 people.

The numbers of women of childbearing age will increase from around 68.1 million in 2015 to 71.2 million in 2019. From this number, it is estimated there will be five million expected pregnant mothers every year. This number constitutes an estimation number of total deliveries and child births. It also considered as a referral for ANC burden, total deliveries, and neonatus/newborns healthcare provision. The occupational-age group will increase from 120.3 million in 2015 to 127.3 million in 2019. Total population aged over 60 years will increase from 21.6 million in 2015 to 25.9 million in 2019. The current population size of elderly people in Indonesia surpasses the total population of Australia that is approximately 19 million.

The implications of the increased elderly population towards health system are (1) the increasing needs for secondary and tertiary healthcare provision, (2) the increasing needs for home care provision and (3) the rapid growth of health-related costs. Consequently, the government must provide numbers of elderly people-friendly and the disabled people' facilities, considering the large proportion of people with disabilities within this age group.



The difficulties in reducing poverty rate will still become a crucial issue. In quantity, the total of poor population is increasing which leads a problem for the government in terms of incurred costs to be covered. In 2014, the government should disburse the premium fund for the health insurance scheme of 86.4 million specifically targeting the poor and near-poor people. The BPS data discloses that during 2013 there was an increase in the poverty gap index from 1.75% to 1.89% and the poverty severity index from 0.43% to 0.48%. This exhibits that the level of poverty is worsening, because it keeps away from the poverty line, and the financial gap between the poor and the non-poor people is largely widening.

The education level of people is one of the vital indicators that determine the human development index. Apart from health, education also plays a great role in actualizing the quality human resources of Indonesia. However, despite of having the increased average of school attendance period within couples of years; it has not fulfilled the target of 9-years compulsory education program. According to a national survey (Susenas) calculation on quarter I 2013, the average of school attendance period for age group of 15 years old and above in Indonesia is 8.14 years. This condition is closely related to the School Participation Rate (APS), which is the percentage of school student in various levels of education against the number of relevant school-age population.



Disparity of Health Status. Although nationally the quality of public health has been enhanced, the disparity of health status between socio-economic level, regions and urban-rural areas remains high. Infant and under-five mortality rate in the poorest group is almost four times higher than the richest. Moreover, infant mortality rate and maternal death rate during child birth is higher in rural areas, in eastern Indonesia, and among population with low education level. Percentage of under-five with under nutrition and malnutrition status in rural areas is higher than in urban areas.

Disparity of Health Status between Regions. Some data on gap in health sector can be looked at the Riskesdas 2013. The proportion of stunting infants: the lowest one is in Bali province (9.6%) and the highest one is in East Nusa Tenggara province (28.7%) or three times compare to the lowest one. A concerning gap is reflected from the form of community participation in health sector, there is a weighing frequency for the under-five (under-five weighing >4 times in the last 6 months). The lowest frequency for weighing the under-five is in North Sumatera province (only 12.5%) and the highest is 6 times fold in Yogyakarta province (79.0%). It indicates a large gap of Integrated Health Post (Posyandu) activities among provinces. Compare to 2007, the gap is considered wider that implies a dropped number of activities in Posyandu, and the variation between provinces follows the increased gap.



The Implementation of National Social Security system (SJSN). Based on the road map towards the National Health Insurance Scheme, by 2019 all of Indonesians should be covered by Universal Health Coverage (UHC/JKN). The imposition of JKN clearly required an improvement of access and quality in healthcare, either at the primary level of healthcare or at the following ones, and it is also required to enhance the service referral healthcare system. In order to control the burden of state budget that is required by JKN program, it needs a supporting role from public health intervention that is promotive and preventive, so that the people will remain fit and will not get sick easily. The progress of JKN membership has achieved a favorable result. Until the beginning of September 2014, the number of participants has reached 127,763,851 people (105.1% from the target). The fast growing participants do not come with the increasing number of healthcare facilities and it leads to a long queue which if it's not immediately overcome might impair the service quality.

Gender Equality. The human resources quality of women still needs to be improved, especially because: (1) women will be an active working partner for men in addressing social, economy and political issues and (2) women influence the quality of the next generation due to the reproductive function of women plays a role in developing human resources in the future.



The Indonesian Gender Development Index (IPG) has been increased from 63.94 in 2004 to 68.52 in 2012. The increase of IPG is basically induced by improvement in some of IPG's components indicators which comprise of health, education and viability.

The Enactment of Village Law. The Law Number. 6/2014 on village has been stipulated on January 2014. Since then, every 77,548 villages will receive a quite large allocated fund every year. Based on the simulation of 2015 state budget, each village will receive approximately a billion rupiahs. The transferred funds would bring a great impact for the empowerment of village community. The clean and healthy lifestyle behaviour (PHBS) and community based healthcare (UKBM) will posed a better opportunity to be implemented at household level in the village because there will be adequate infrastructure as its enabling factors.

The Strengthening Role of Province. By the enactment of Law Number 23/2014 as the replacement for Law No. 32/2004 on local government, the province, in addition to hold status as a region, is now also an administrative working territory for the governor as the representative of central government. The Minimum Service Standard (SPM) in health sector that has been regulated by the Minister of Health Law Number 23/2014 that gives a pretty strong role for the province to control the districts and cities in their territories. The monitoring on SPM implementation in



health sector can be fully handed over to the province by the Ministry of Health because now the province has the authority to impose sanction on district/city related to the implementation of SPM.

The Enactment of Health Information System Regulation In 2014, Government Regulation (PP) Number 46 on Health Information System (SIK) was enacted. It requires dataset on health should be accessible for the working units of Government Agencies and Local Government that manage SIK according to their respective authorities.

2. Regional Strategic Settings

The ASEAN Economic Community (MEA) will be effective started by 1st January 2016. The implementation of ASEAN Community that encompasses the total population of 560 million of people will encourage the opportunities (market access) and challenges at the same time for Indonesia. The implementation of ASEAN Economic Community includes trade liberalization on goods and services and also investment on health sector. Thus, an intervention to improve the competitiveness of domestic healthcare facilities is required. The improvement of the existing healthcare facilities, for example, human resources, equipments, infrastructures, and management should be carried out. The accreditation of healthcare facilities (hospital, Puskesmas, and etc.) should be implemented seriously, immediately and well-planned.



This is related to the Mutual Recognition Agreement (MRA) on some types of profession that associated with the mobility coverage. The MRA specified not only the engineers and accountants but also the HRHs/physicians, dentists and nurses. It is possible that in the future there will be an extension of included professions' lists.

Nevertheless, the competitiveness level of domestic HRHs should be strengthened. The quality of the institutes for HRH's education should be enhanced through intense development and accreditation.

3. Global Strategic Settings

Millennium Development Goals (MDGs) will come to the end by 2015 there are many countries that acknowledge the success of MDGs as the driver for reducing poverty and advancing community development. The acknowledgement comes especially in the form of political support. The follow up of this program is known as the Sustainable Development Goals (SDGs) which covers 17 goals. In terms of health sector, the fact points out a healthy individual would like to have a stronger physical and intellectual capabilities, thus, they are able to contribute a productive output to community development.

Accession to the Framework Convention on Tobacco Control. Framework Convention on Tobacco Control (FCTC) is the most impactful global response against tobacco and tobacco's product (cigarettes)



which are the cause of various fatal diseases.

Until now, there are 179 countries in the world that have ratified the FCTC. Indonesia is one of the initiator countries and had participated in formulating the FCTC. However, Indonesia has not adopted it despite all the pressures from many parties to Indonesian government to do so. The reasons are not only for the benefit of community health but also to protect the reputation of Indonesia in the world.

The trade liberalization on goods and services in WTO context - especially in General Agreement on Trade in Service, Trade Related Aspects on Intellectual Property Rights and Genetic Resources, Traditional Knowledge and Folklores (GRTKF) are all global commitment forms that must be carefully responded.

The priority includes accelerating the completion of MoU towards an agreement that is more operational oriented so that the result of the cooperation between countries can be immediately obtained.





CHAPTER TWO

OBJECTIVE AND STRATEGIC TARGETS

MINISTRY OF HEALTH



MINISTRY OF HEALTH
OF THE REPUBLIC OF INDONESIA



CHAPTER TWO

OBJECTIVE AND STRATEGIC TARGETS

MINISTRY OF HEALTH

The 2015-2019 Strategic Plan of Ministry of Health does not define vision and mission but it follows the vision and mission of the President of the Republic of Indonesia that is “the Realization of Indonesia as a sovereign, independent state, with a strong character and the mutual cooperation as its basis”. This vision can be realized by implementing seven missions of development:

1. The realization of national security that is able to protect the integrity of territory, support the economic independence by securing the maritime resources and reflects Indonesia’s character as archipelagic state.
2. Realize community that is developed, continuous and democratic based on constitutional state.
 1. Actualize free and active foreign policies and enhance the national identity as maritime state.
 2. Creating the quality of Indonesian people that is excellent, developed and prosperous.
 3. Shaping a competitive nation.
 4. Raise up Indonesia as maritime state that is independent, developed, strong and based on national interest, and
 5. Forming Indonesian people who have the personality in culture.



And furthermore there are nine agenda of priorities known as NAWA CITA that should be materialized by the Working Cabinet, those are:

1. To renew the state's obligation to protect all people and provide security to all citizens.
2. The presence of good governance through developing clean, effective, democratic and reliable governance.
3. To build Indonesia from its periphery by strengthening rural areas within the framework of a unitary state.
4. To reject a weak state by reforming the system and law enforcement that is corruption-free, dignified and reliable.
5. To improve the quality of Indonesian.
6. To improve people's productivity and competitiveness in the international market.
7. To achieve economic independence by moving the strategic sectors to domestic economy.
8. To revolutionize the nation's character.
9. To strengthen diversity and social restoration of Indonesia.

The Ministry of Health has the role and contribution in achieving the whole Nawa Cita especially in improving the quality of life of Indonesian.

A. OBJECTIVES

There are two objectives of the Ministry of Health in 2015-2019 which are: 1) improved status of community health and; 2) improved responsiveness and social and financial protection of the community in health sector.



The improvement of community health status is implemented in all life cycle continuum which are started from infant, under-five, school-age children, adolescent, working-age group, maternal and elderly people.

The indicators objective of Ministry of Health is impact or outcome oriented.

In improving the community health status, indicators to be achieved are:

1. The reduction of maternal mortality rate from 359 per 100,000 live births (SP 2010) 346 to 306 per 100,000 live births (SDKI 2012).
2. Reduction of infant mortality rate from 32 to 24 per 1,000 live births.
3. Reduction of BBLR percentage from 10.2% to 8%.
4. Improvement of health promotion intervention and community empowerment as well as the financing of promotive and preventive activities.
5. The improvement of clean and healthy lifestyle intervention.

Meanwhile, in order to enhance the responsiveness and protection of people against social and financial risks in health sector, the desirable indicators are:

1. Reduction of household burden to finance health services after having the health insurance, from 37% to 10%
2. The increasing of responsiveness index against health services from 6.80 to 8.00.



B. STRATEGIC TARGETS

The strategic targets of Ministry of Health are:

1. The improvement of community health, targets to be achieved are:
 - a. The increasing percentage of healthcare facility based deliveries of 85%.
 - b. The decrease percentage of pregnant mothers with chronic energy deficiency to 18.2%.
 - c. The increasing percentage of district and city that applies the policies of clean and healthy lifestyle (PHBS) to 80%.
2. The improvements in controlling diseases, with targets to be achieved are:
 - a. 40% of district/city fulfill the environmental health quality
 - b. 40% of cases of preventable diseases by executing certain immunization (PD3I) can be decreased.
 - c. 100% of district/city has the response preparedness of health emergency circumstances that has outbreaks potentials.
 - d. Decreasing prevalence of smoking in age group ≤ 18 years by 5.4%
3. Better access and improved quality of health service facilities, with targets to be achieved are:
 - a. 5,600 sub-districts that have at minimum one accredited Puskesmas each.
 - b. 481 districts/cities that have at minimum one accredited RSUD (Local General Hospital) each.



4. The improvement on access, self-reliance and quality of pharmaceutical care and medical devices, with targets to be achieved are:
 - a. 90% availability of drugs and vaccines at Puskesmas.
 - b. 35 types of medicinal raw materials, traditional medicines and medical devices that are manufactured in the country.
 - c. 83% of medical devices products and household medical supplies in the market are qualified.
5. The increasing of number, types, and improvement of quality and equality of human resources for health (HRH), with targets to be achieved are:
 - a. 5,600 Puskesmas that has at minimum 5 types of health workers each.
 - b. 60% of district/city hospitals class C that has four basic specialist doctor and three supporting specialist doctor.
 - c. Competency improvement of 56,910 HRHs.
6. The improvement of synergy between ministries/agencies, targets to be achieved are:
 - a. The increasing number of other relevant line ministries that support health development.
 - b. The increasing percentage of district/city that receives the best reputation in implementing SPM by 80%.
7. The improvement of utilization of partnership nationally and internationally, targets to be achieved are:

- a. 20% of business sector that utilizes the CSR for health program.
 - b. 15 civil society organizations that utilize their resources for supporting health sector.
 - c. 40 international cooperation agreements in health sector that are implemented.
8. The improvement of integrated planning, technical guidance and monitoring-evaluation, with targets to be achieved are:
- a. 34 provinces that have 5 years terms plan and an integrated health budget from various sources.
 - b. 100 recommendations of integrated monitoring-evaluation
9. The improvement of the effectiveness on health research and development, with the targets to be achieved are:
- a. 35 research results are registered under HKI.
 - b. 120 recommendations of policies that are based on health research and development which are being advocated to the health program manager and or to stakeholders.
 - c. Five reports on National Health Research (Riskesmas) in health and public nutrition.
10. The improvement of good and clean governance, with targets to be achieved is:
- 100% of Work Units that undergo an audit and the findings on state loss is $\leq 1\%$.



11. The improvement of competency and performance of Ministry of Health's public officials, with the targets to be achieved are:
 - a. The increasing percentage of structural officials in the scope of Ministry of Health whose competency are relevant with the requirements of their position by 90%.
 - b. The increasing percentages of employees of Ministry of Health whose performance values are at minimum good by 94%.
12. The improvement of the integrated Health Information System, with targets to be achieved are:
 - a. The increasing percentage of district/city that report the priorities of health data in a complete and timely manner by 80%.
 - b. The provision of data communication network that is designated for e-health service access by 50%.



MINISTRY OF HEALTH
OF THE REPUBLIC OF INDONESIA



CHAPTER THREE

POLICY DIRECTION, STRATEGY, REGULATION AND INSTITUTIONAL FRAMEWORK



MINISTRY OF HEALTH
OF THE REPUBLIC OF INDONESIA



CHAPTER THREE

POLICY DIRECTION, STRATEGY, REGULATION AND INSTITUTIONAL FRAMEWORK

A. NATIONAL POLICY DIRECTION AND STRATEGY

The national policy direction and health development strategies of 2015-2019 are part of Long-Term Development Plan in health sector (RPJPK) of 2005-2025, which aims to enhance the awareness, willingness, and capability of people to live a healthy lifestyle, thus the realization of the ultimate societies' health status can be achieved through the creation of healthy behavior and environmental health of Indonesian people, nation and state; concurrently, it possess the ability to demonstrate equal and fair quality of healthcare provision whereas provides the highest level across Indonesia.

The targets of health development that must be achieved by 2025 are improved health community' status which is highlighted by the increasing age of life expectancy, the decreased infant and maternal mortality rate; as well as the decreased prevalence of under nutrition among under-five.

In order to accomplish both goals and targets of health development, thus, the 2005-2025 health development strategies are: 1) health-oriented national development; 2) the empowerment of community and regionals; 3) the development of health intervention and financing; 4) development and empowerment of human resources for health; and 5) health emergency response.

In the RPJMN of 2015-2019, the defined targets are enhancing the health and public nutrition status through the implementation of health intervention and community empowerment that are supported by financial protection and health service equality.

The health development targets stated in RPJMN 2015-2019 are as follow:

No	Indicators	Initial Status	Target 2019
1	Improvement of Health Status and Nutrition		
	a. Maternal mortality rate per 100,000 live births	346 (SP 2010)	306
	b. Infant mortality rate per 1,000 live births	32 (2012/2013)	24
	c. Prevalence of underweighted under-five children(percent)	19.6 (2013)	17.0
	d. Prevalence of stunting (moderate and severe) for children aged under 2 years (percent)	32.9 (2013)	28.0

No	Indicators	Initial Status	2019 Target
2	Enhanced Control on Communicable and Non-Communicable Diseases		
	a. The prevalence of Tuberculosis (TB) per 100,000 populations	297 (2013)	245
	b. The prevalence of HIV (percent)	0.46 (2014)	<0.50
	c. Numbers of districts/cities that are succeeded in eliminating malaria	212 (2013)	300
	d. The prevalence of high blood pressure (percent)	25.8 (2013)	23.4
	e. The prevalence of obesity among population aged 18+ years (percent)	15.4 (2013)	15.4
	f. The prevalence of smoked individuals aged <18 years	7.2 (2013)	5.4
3	The Improvement of Equal Distribution and Quality of Healthcare Provision		
	a. Number of sub-districts that acquires at minimum an accredited Puskesmas	0 (2014)	5,600
	b. Number of districts/cities that have at minimum a national accredited certification local hospital	10 (2014)	481
	c. Percentage of districts/cities that reaches 80% of full basic immunization for the	71.2 (2013)	95

No	Indicator	Initial Status	Target 2019
4	The Improvement on the Financial Protection, Availability, Distribution, and Quality of Medicine as well as Human Resources for Health (HRH)		
	a. The percentage of SJSN's Participants (percent)	51.8 (October 2014)	Min 95
	b. Numbers of Puskesmas that has at least five types of HRHs	1,015 (2013)	5,600
	c. The percentage of Class-C local hospitals in districts/cities which have seven specialists	25 (2013)	60
	d. The percentage of medicine and vaccines' stocks at Puskesmas	75.5 (2014)	90.0
	e. Percentage of qualified medicine	92 (2014)	94

The health development policies are focused on the strengthening of quality of primary healthcare especially through the enhancement of health insurance, improvement of access and quality of primary and referral healthcare that are supported by the health system strengthening and increased health financing. Healthy Indonesia Card is one of the major medium in encouraging the health reform in order to actualize an optimum healthcare provision including the enhancement of promotive and preventive interventions.

The strategies of 2015-2019 health development cover:

1. Acceleration of improved access to quality maternal,



- child, adolescents and elderly people healthcare provision.
2. Accelerated nutrition status improvement.
 3. Enhancing the control of disease and environmental health.
 4. Improving the access to quality primary healthcare provision.
 5. Upgrading the access to quality referral healthcare provision.
 6. Increasing the availability, affordability, distribution, and quality of pharmaceutical products and medical devices.
 7. Strengthening the monitoring on foods and medicine.
 8. Improving the availability, distribution and quality of deployed human resources for health.
 9. Reinforcing the promotion of health and community empowerment
 10. Enhancing the management, research and development as well as the information system
 11. Stabilizing the implementation of National Social Security System (SJSN) on health sector
 12. Developing and enhancing the effectiveness of health financing.

B. POLICY DIRECTION AND STRATEGIES OF MINISTRY OF HEALTH

The policy direction and strategies of Ministry of Health are based on the national policy direction and strategy as set out in the National Medium Term Development Plan (RPJMN) of 2015-2019. To ensure and promote the implementation of various effective and efficient health



interventions, thus, the prioritized interventions embed with high leverage in achieving the goal of health development will be executed in integrated manner on the focus and locus of activities, health and health development.

The policy direction of Ministry of Health refers to three major things:

1. The Strengthening of Primary Healthcare Provision

Puskesmas conducts the function as health gate keeper in the region through four types of interventions which include:

- a. Improving and empowering the community.
- b. Enhancing public health intervention.
- c. Implementing individual healthcare provision.
- d. Monitoring and encouraging a health-oriented development.

In order to reinforce those three functions, the revitalization of Puskesmas is considered as a vital step. It focuses on five matters: 1) improvement on HRHs; 2) enhancement of technical and management capacity of Puskesmas Provision; 3) upgrading the health financing; 4) improvement on Puskesmas' Information System (SIP); and 5) implementation of Puskesmas accreditation.

The enhancement of deployed HRHs in Puskesmas is prioritized on the availability of five types of HRHs, which are: Public health specialists, sanitarian, nutritionist, pharmacists, and medical analyst. In order



to promote the accomplishment of national health development targets; particularly is the strengthening of primary healthcare provision. The Ministry of Health develops a Healthy Nation Program (Nusantara Sehat). This program specially assigned HRH at primary healthcare level utilizing a team-based method.

The management capacity of Puskesmas is directed to improve Health Information System quality, planning quality at Puskesmas level and technical capability in carrying out an early detection in health related issues, community empowerment, and improve the quality of environmental health.

The Financing of Puskesmas aims to reinforce the implementation of effective promotive and preventive activities by the optimization Puskesmas' financing sources.

The Development of health information system refers to data and information acquisition on health problems as well as on health development outcomes in timely and accurate manner.

Puskesmas' accreditation aims to improve the quality of healthcare provision and it focuses on the regions that are covered in health development priorities.

2. The Implementation of Continuum of Care Approach

The approach is implemented by expanding the scope, quality and sustainability of diseases control



intervention and healthcare provision for mothers, infants, under-five children, adolescents, working-age and elderly people.

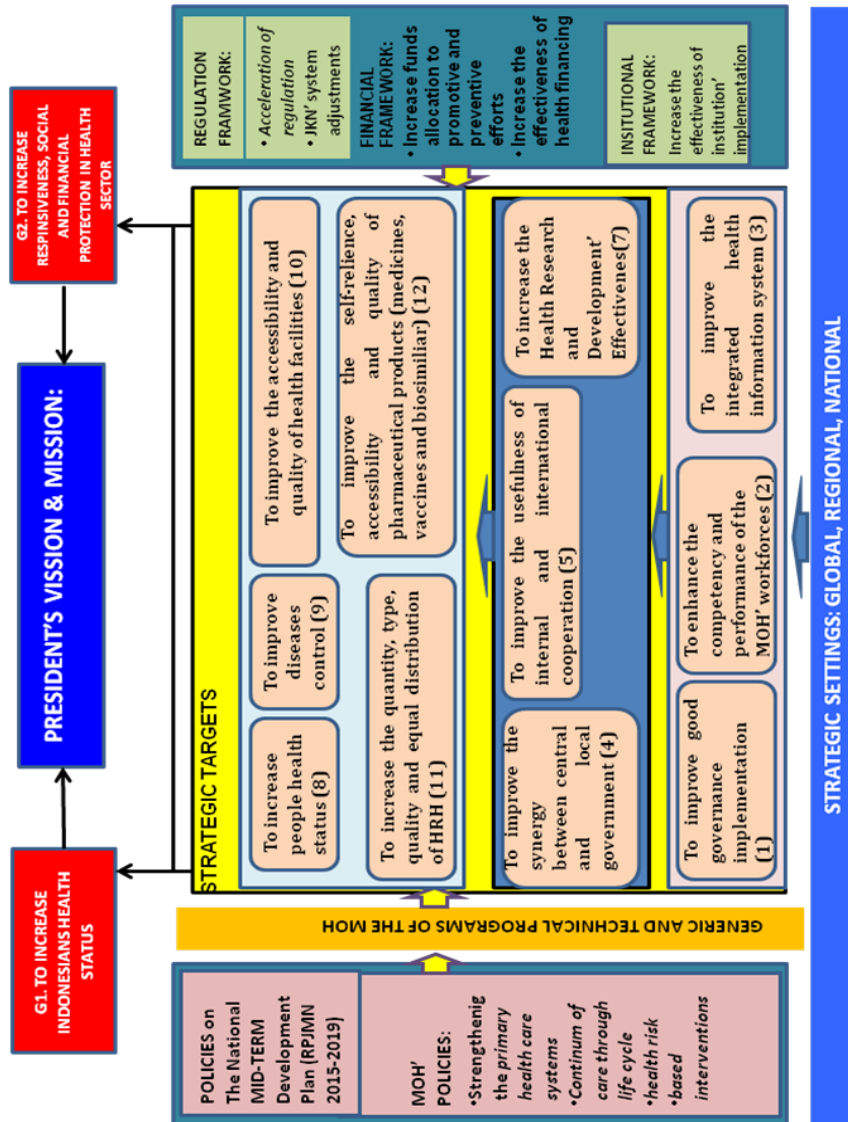
3. Health Risk Based Intervention.

The specific programs to address health related issues suffered by infant, under-five children and elderly people, pregnant mothers, refugees and disadvantages families, risk-groups, and community who lives in remote areas, border territories, islands, and regions embed with health issues.

In order to achieve the goal, the Ministry of Health set up the strategy as illustrated in Figure 1.

The detailed strategies are developed as a part of the overall strategy and stages to accomplish the goal of the Ministry of Health, whether those are specified in Goal 1 (T1) or Goal 2 (T2). The goals were set up to accomplish the President's vision and mission. In order to actualize both goals, The Ministry of Health must ensure that there are 12 strategic targets to be realized as direction and strategic priorities in the following five years. The 12 strategic targets form a hypothesis of cause-consequence in realizing the T1 and T2.

Figure 1. The Strategic Map to Achieve the Vision of the Ministry of Health



The Ministry of Health specified twelve detailed strategic targets that are divided into three groups: strategic targets on input aspects (organization, human resources, and management); on institutional strengthening aspect; and on strategic intervention aspect.

- **Strategic targets on input aspects:**

- 1. Improving Good Governance**

The strategies in improving good governance include:

- a. Promoting the effective, efficient, economical financial management that comply the rules and regulation.
- b. Improving transparency and accountability by taking into consideration the principle of justice and compliances.
- c. Demonstrating good quality of auditing system that enables to generate a relevant statutory auditing report (LHP) to the stakeholders' needs.
- d. Realizing a transparent and accountable management of the Inspectorate General.

- 2. Improving competency and performance of Ministry of Health's Public Officials**

The strategies include multiple efforts:

- a. Develop a competency standard of structural position for all echelons.
- b. Develop an open formation system in the internal of the Ministry of Health, such as, job bidding for echelon 1 and 2.

3. Enhancing an integrated Health Information System The strategies include multiple efforts:

- a. Developing “real time monitoring” for all Program Performance Indicator (IKP) and Activity Performance Indicators (IKK) of the Ministry of Health.
- b. Improving the capacity of information specialists at district/city and provincial level, so that the health profile can be published T+4 months, or can be published in every April.

The next strategy is to manage the internal strategic process in the ministry in an excellent manner, which includes the enhanced synergy between line ministries/agency (K/L), central and local government (SS6), increased domestic and international partnership (SS7), an improved integration of planning, technical guidance and monitoring and evaluation (SS8), as well as optimized effectiveness on health research and development (SS9).

- **Strategic target group on institutional strengthening aspect:**

4. Enhancing the Synergy between Line Ministries/Agencies

The strategies include multiple efforts:

- a. Developing the national action plan of prioritized health development program.



- b. Creating a communication forum to ensure the synergy between line ministries/agencies (K/L).

5. Improving the Efficiency of Partnership (Domestically and Internationally)

The strategies include multiple efforts:

- a. Developing a roadmap for domestic and international cooperation.
- b. Establishing rules of cooperation that completes the established roadmap.
- a. Organizing a communication forum among stakeholders to identify the effectiveness of the institutional partnership both domestically and globally.

6. Improving the Integrated Planning, Technical Guidance and Monitoring-Evaluation

The strategies include multiple efforts:

- b. Designation of focus and locus on health development
- c. Establishing the technical policies for integrated planning, monitoring and evaluation.
- d. The competency building of planners and evaluators at central and regional level.
- e. Technical assistance on health planning at regional level.
- f. Improving the quality and utilization of integrated monitoring and evaluation results.

7. Improving the effectiveness of Health Research and Development

The strategies include multiple efforts:

- a. Expanding domestic and international cooperation on research that involves other line ministries/agencies, universities and local governments based on mutually beneficial agreement and acceleration of technology transferred process.
- b. Strengthening the network of both research and laboratory in promoting national research and healthcare system.
- c. Actively developing strategic partner alliances with non-ministries state agencies, local governments, business entities and academics.
- d. Extending dissemination and advocacy of the utilization of research and development results for health programs and policies' needs.
- e. Conducting research and development that refers to the policies of the Ministry of Health and to the Prioritized Policy Plan of the Health Agency Research and Development 2015-2019.
- f. Development of infrastructure, facilities, resources and regulations in implementing research and development.

In order to achieve the goals of the Ministry of Health, the five inter-related strategic targets as a result the implementation of various integrated

technical program must be realized first, those targets are: 1) Improving the public health provision (SS1); 2) Improving diseases control (SS2); 3) Improving access to and quality of health facilities (SS3); 4) The increase of amount, types, qualities, and equal distributed of HRHs (SS4); and 5) Improving the access, self-reliance and quality of pharmaceutical supplies and medical devices (SS5).

- **Strategic targets group on strategic intervention:**

- **8. Improving Public Health Status**

The strategy aims to improve public health that covers the healthcare provision for all age groups, following the life cycle starting from infant, child, adolescent, productive age, maternal and elderly people. It will be achieved by:

- a. Organizing health consultation, advocacy and partnership development with various development agents including local government.
- b. Empowering the community and increasing the role of people in health sector.
- c. Increasing the number and enhancing the capability of public health counselors/and other HRHs in promoting health.
- d. Developing method and technology of health promotion that is relevant to the dynamic changes in community

9. Improving the Control of Diseases

- 1) In order to control the communicable diseases, the strategies include:
 - a. Expanding the coverage of access to healthcare for all Indonesians (including rapid screening test if there is an allegation of potential increase of communicable diseases outbreaks such as conducting a Mass Blood Survey for malaria infection) in related to the communicable diseases especially for regions in the borders, islands and remote areas to reinforce the outbreaks cycle cutting off.
 - b. In order to enhance the quality of communicable diseases' control intervention, an innovative strategy for instance, granting the authority to public health officers, especially the access right for observing the risk factors, diseases and determination of its countermeasures is required to be executed.
 - c. Boosting the involvement of community in assisting diseases control intervention through a community based surveillance system that puts a scrutiny of some matters that might contain health-related issues, then, they report it to the health officers for an early response, thus it mitigates any health problems.

- d. Improve the competency of HRHs, including the epidemiologist, sanitarian and laboratory staffs in controlling communicable diseases.
 - e. Enhancing the role of local sector especially several designated districts/cities as the entry point for the country in implementing International Health Regulation (IHR) for preventing any diseases which are potential in inducing any medical emergency situation.
 - f. Ensuring the availability of drugs and vaccines as well as instant diagnostic devices for prompt control against communicable diseases.
- 2) In terms of non-communicable diseases, an early detection intervention which proactively reaching the community because 3/4 of people does not realize that they are suffering from non-communicable diseases particularly the workers. In addition to that it is required to encourage district/city that implements clean and healthy lifestyle to enact a smoke-free areas to limit smoking spaces for the cigarettes smokers.
- 3) Improved environmental health, the strategies include:
- a. Formulation of local legislation in the form of governor, mayor/district head regulation that may drive other sector in the region to play an active role in implementing environmental health

activities such as increasing the availability of good sanitation, quality drinking water and healthy areas.

- b. Increasing an effective use of technology according to the capacity and condition of environmental health issues in each region.
- c. Enhancing the involvement of community in sanitation businesses.
- d. Improving the working group of drinking water and environment health (AMPL) through holding AMPL's network meetings, role division of Regional Working Unit (SKPD) in supporting drinking water access and sanitation improvement.
- e. Reinforcing the role of Puskesmas in organizing the movement of Stop Improper Defecation (SBS) in sub-district/district areas; at minimally a Puskesmas should have a SBS village.
- f. Extending the role of potential regions that carry out the strategy of health outcome adaptation due to climate change.

10. Improving Access and Quality of Health Services Facility

In order to improve access and quality of Primary Healthcare (FKTP), some implemented interventions consist of:

- a. Establishing a well-defined budget allocation for providing quality medical facilities and devices.



- b. Optimizing the function of FKTP, whereas each sub-district has minimally a standardized Puskesmas.
- c. Actualizing healthcare innovations, for example, a flying healthcare provision (the targets are some provinces with remote and the most remote areas as well as districts/cities that do not have specialists, telemedicine, primary hospital, etc.
- d. Realizing the support on regulation by stipulating policies and NSPK (norm, standard, procedure and criteria) of FKTP.
- e. Conducting the collaboration system on education of HRHs through a strengthening effort on the concept and competency of Primary Healthcare Doctors (DLP) and strategic HRHs.
- f. Actualizing the enhancement of quality of advocacy, development and monitoring on local government in order to strengthen the management of Puskesmas by the health office at district/city level.
- g. Promoting the performance management system of FKTP through the implementation of a performance assessment instrument.

In order to improve the access and quality of referral healthcare provision, some interventions to be implemented should include:

- a. Determining a well-defined budget allocation for providing quality and standardized medical facilities and devices at hospitals.

- b. Executing the implementation of hospitals' performance management system to ensure patient safety, as well as a standardized healthcare delivery by doctors and nurses.
- c. Strengthening the quality of advocacy, facilitation and monitoring to accelerate quality improvement of healthcare provision and promote the leveling up of local hospital (RSUD) to local public service agency (BLUD)
- d. Optimizing the role of technical unit (UPT) in administering local healthcare facilities provision (Fasyankes).
- e. Delivering various excellent services (the handling of tertiary cases) at the national referral hospitals in integrated manner with the academic health system.
- f. Strengthening the referral system by developing regionalized referral system in each province (one regionalized referral hospital for several districts/cities) and national referral system (one national referral hospital for several provinces)
- g. Establishing a highly efficient partnership through the implementation of sister hospital program, partnership with private sectors, operational cooperation (KSO) on medical devices and so forth.
- h. Establishing a collaboration system of HRH's education.

11. Increasing the Number, Type, Quality and Equality Distribution of HRHs

The strategies include multiple efforts:

- a. The deployment of team based specialized health workers.
- b. Increasing the distribution of integrated, bounded and locally specific workers.
- c. Developing an incentive system which includes both material and non-material for all HRHs.
- d. Enhancing of HRH' quality production.
- e. Implementation of registration and professional licensing mechanism using a competency test against all HRHs.
- f. Leveling up training quality through accreditation mechanism of trainings.
- g. Controlling the participants and results of education.
- h. Escalating the numbers of long distance education and trainings.
- i. Improving the numbers of competency-and-job requirement-based trainings.
- j. The development of performance system.

12. Improving the Access, Self-reliance, and Quality of Pharmaceutical and Medical Devices Stocks

High political commitment is required to actualize the locally drug-reliance on medicinal raw materials. Some defined strategies from various interventions include:

- a. Regulations on pharmaceutical companies in producing raw materials and traditional medicines and its utilization of those materials in domestic drugs and traditional medicines production; and finally, on the form of incentives for national locally drug-reliance acceleration intervention.
- b. Regulation on strengthening the institutions and pre- and post-monitoring system on market of medical devices.
- c. The working group of Academy, Business, Government dan Community (ABGC) in development and production of medicinal raw materials, traditional medicines and medical devices in the country.
- d. The stipulation of regulation that reinforces the utilization and development of domestic medical devices industries.
- e. Raising awareness and concerns of community and HRHs on the importance of domestically raw materials-reliance on medicine, traditional medicines and medical devices which have the quality and affordability.
- f. Establishing national pharmacy installation as the center of excellence for the management of medicine, vaccines and medical supplies (perbekkes) in public sector.

- g. Strengthening the administration and implementation of Health Technology Assessment (HTA) on medicine and medical devices selection for government programs as well as for the beneficial of JKN.
- h. The acceleration of generic products' availability for all expired patent drugs.
- i. Setting up an integrated information system and network in pharmaceutical and medical devices sector.
- j. Transforming the pharmacist as strategic HRHs, including organizing the contracted employment program (PTT) to promote the equally deployed HRH's distribution.
- k. Improving the quality of pharmaceutical care delivery and utilization of rational use of medicine through the strengthening of managerial, regulation, education as well as monitoring and evaluation system

C. REGULATION FRAMEWORK

An Adequate regulation is considered as an essential to lever the good implementation of the program and activity. It is required to align and prepare the regulation with the global, regional and national challenges. The regulation framework is directed to: 1) set up regulation from the derivates of legislation that are relevant to health sector; 2) improve a equal distribution of deployed HRHs; 3) diseases control and environmental sanitation; 4) the improvement of community empowerment and health oriented development; 5) the strengthening of locally



medicine and medical equipments-reliance; 6) A better quality of the implementation of national health insurance; 7) the strengthening on government role in decentralization era; and 8) an enhancement of health financing.

The regulation framework that will be developed includes the formulation of government regulation, presidential regulation, and relevant ministerial regulation, as well as developing the synchronization, integration of health development implementation between central and local levels.

D. INSTITUTIONAL FRAMEWORK

The established organization design was based on the constitutional mandates, various legislations, development and challenges in strategic environment in health development sector, national health system, as well as it takes into account any progress in governance issues, decentralization and regional autonomy policies, and bureaucratic reform principles (effective and efficient institutional structuring).

The most fundamental function of government is to serve the people. The Ministry of Health will deliver an effective administration by setting up an organization design with the right function and right sizing, removing any overlapping task and function by clearly defining the role, responsibility and coordination mechanism (horizontally and vertically) in carrying out all 2015-2019 Renstra's programs.



The institutional framework consists of: 1) synchronization institution nomenclature with the program of the Ministry of Health; 2) the strengthening of health policies to support the norm, standard, procedure and criteria and mainstreaming of health oriented development; 3) reinforcement of monitoring, control, surveillance and evaluation of health development; 4) reinforcement of internal business of the Ministry of Health that covers the arrangement of HRHs, management, regulation and health information; 5) improvement of access and quality of health services; 6) enhancement of health development synergy; 7) enhancement on prioritized program of health development; and 8) the screening of health technologies.



CHAPTER FOUR

PERFORMANCE TARGETS AND FUNDING FRAMEWORK





CHAPTER FOUR

PERFORMANCE TARGETS AND FUNDING FRAMEWORK

By taking into account the preliminary design of RPJMN 2015-2019, the vision and mission, goals, strategies and strategic targets aforementioned in previous chapters, the performance targets and funding framework are developed for 2015-2019 programs. There are two programs of the MOH-Indonesia, the generic and the technical programs.

The generic programs include:

1. Supporting program for management and implementation of other technical tasks.
2. Enhancement program for the implementation of National Health Insurance (JKN)/Healthy Indonesia Card (KIS).
3. Program for enhancing the surveillance and accountability of the Indonesia's MOH apparatus.
4. Health research and development program.

The technical programs include:

1. Mother and child health and nutrition development program.
2. Diseases control and environmental sanitation program.
3. Healthcare provision development program.
4. Pharmaceutical and medical devices program.
5. Development and empowerment of human resource for health program.

A. PERFORMANCE TARGET

Performance targets are assessment of program achievements that are regularly measured and evaluated at the end of 2019. The targets are calculated in cumulative during five years period and will come to the end of 2019.

1. Supporting Program for Management and Implementation of other Technical Tasks

The targets for this program are improved coordination of tasks implementation, development and delivery of management support of the MOH. The indicators of targets achievement are:

- a. The formulation of 15 health-oriented public policies.
- b. The percentage of the alignment between management support and implementation of other technical tasks reaches 98%.

In order to achieve those results, activities that will be conducted are:

1) Planning and Budgeting of Health Development Program

The target of these activities is the increasing quality of planning and budgeting of health development program. The indicators of targets achievement are:

- a) 34 provinces that have five years terms plan and an integrated health budget from various sources.



- b) The number of quality document of planning policies, budget and evaluation of health development should reach 127 documents.
- c) 170 recommendations of integrated monitoring-evaluation.

2) Personnel Administration Development

The target of this activity is the improving service delivery of personnel administration. The indicators of target achievement are:

- a) The percentage of the fulfillment for human resource for health reaches 90%.
- b) The percentage of structural officials in the scope of Ministry of Health whose competency are relevant to job requirement reaches 90%.
- c) The percentages of Ministry of Health personnel whose performance values are at minimum good reaches 94%.

3) Development of Financial Administration and State Assets Management

The target of this activity is the increasing quality of Financial and State Assets (BMN) management of the Ministry of Health that is effective and efficient and reported according to the rules. The indicators of targets achievement are:

- a) The percentage of work units that submit a timely and quality financial statement according to the Government Accounting System (SAP) in order to maintain the Unqualified Opinion (WTP) reaches 100%.

- b) The percentages of fixed asset values that have been granted by the Stipulation of Utilization Status (PSP) reaches 100 %.
- c) The percentage of procurement that uses e-procurement reaches 100 %.

4) The Formulation of Laws and Regulation and Organization

The target is the increasing number of laws and regulation, delivery of legal services, organization and procedures. The indicators of targets achievement are:

- a) Number of legal products in health sector:
 - 35 drafts bill (RUU), drafts of government regulations (RPP), draft of presidential regulation/decrees (Keppres)/presidential instruction (Inpres) that have been completed.
 - 375 drafts of the Minister of Health Regulations/Decrees.
- b) Number of handling case and of health cooperation agreement:
 - 300 handlings on legal matters related to assets.
 - 175 handlings of legal cases.
 - 150 cooperation agreements in health sector.
- c) 44 products of organization and job procedures and analysis.
- d) Number of procedures, state affairs organization products in health sector, performance accountability and functional position reach 75 products.



5) The Management of Administration, Protocols, Household, Financial and Salary

The targets of this activity are: 1) the increasing quality of correspondence administration, well-organization of events and activities of the leadership; 2) the increasing quality of documentation service for overseas official travel, official paperwork and archive management in the Ministry of Health environment; 3) an improve management of MOH's office; 4) improving the quality of management of the salaries payment and/or strategic human resources of health in order to support the achievement indicators of health development program 2015-2019. Indicators of those target achievements in 2019 are:

- a) 95% of correspondence administrations, organization of events and activities for the decision maker are well and smoothly implemented according to the rules.
- b) 95% of documents service for overseas official travel is delivered in timely manner.
- c) 90% of central work unit's archives are well-developed and 80% of regional technical implementing unit's archives and official paperwork are well-developed.
- d) 100% provision of office means and facilities.
- e) 96% of payment and/or incentives for strategic human resources of health are well-targeted.

6) Health Data and Information Management

The target of this activity is the improving management of health data and information. The indicators of that target achievement are:

- a) Percentage of districts/cities that report prioritized health data reaches 70%.
- b) The percentage of provision of data communication network that is designated for e-health service access reaches 50%.

7) Promotion of Health and Community Empowerment

The target of this activity is the improving implementation of community empowerment and health promotion. The indicators of that target achievement are:

- a) The formulation of 15 health-oriented public policies to enhance the life quality of Indonesian people.
- b) The percentage of districts/cities that implement clean and healthy lifestyle policies reaches 80%.
- c) The percentage of villages that utilize the 10% village fund for community based healthcare program reaches 50%.
- d) 60 business entities utilize their CSR for health program.
- e) 45 civil society organizations utilize their resources to support health sector.

8) Health Crisis Management

The target of this activity is the improving intervention of health crisis risks reduction. The indicators of that target achievement are:

- a) 170 districts/cities receive support in order to implement health crisis risk reduction intervention in their areas.
- b) 34 provinces receive advocacy and outreach to support the implementation of health crisis risk reduction intervention in their areas.

9) Public Communication Management

The target of this activity is the improving management of public communication. The indicators of that target achievement are:

- a) 38,633 publications on health are disseminated to public.
- b) The percentage of information request and complaint services that are completed reaches 90%.

10) Improvement on Health Intellectual

The target of this activity is the optimum improvement on intellectual health in order to prepare quality manpower. The indicator of that target achievement is the production of 35 instruments for increasing and countering intellectual health problems according to the lifecycle stages to support the development of public education in preparing quality manpower.

11) The improving Health of the Hajj Pilgrims

The target of this activity is the increasing preparedness of effective and required health services in Saudi Arabia. The indicator of that target achievement is the percentages of health checkup results of the pilgrims (3 months before the operation) reaches 80%.

12) Enhancement of International Cooperation

The target of this activity is the better role and position of Indonesia in international cooperation in health sector. The indicator of that target achievement is the formulation of 40 cooperation agreements in health sector.

13) The Management of Indonesian medical Council

The target of this activity is the improvement of registration service and standardization of professional education, development and handling of disciplinary violation cases on doctors and dentists. The indicators of the target are:

- a) 185 cases of disciplinary violation of doctors and dentists are solved.
- c) 167,000 Letters of Registration (STR) of doctors and dentists are registered and delivered in timely manner.

2. Enhancement Program for the Implementation of National Health Insurance (JKN)/ Healthy Indonesia Card (KIS)

The target is the enhancement of National Health Insurance (JKN)/Healthy Indonesia Card (KIS).



The indicator is having 109.9 million people as Contribution Beneficiaries (PBI) by participating in national health insurance (JKN)/Healthy Indonesia Card (KIS).

Development of Health Financing and National Health Insurance (JKN)/Healthy Indonesia Card (KIS).

The target of this activity is the formulation of technical policies on development of Health Financing and National Health Insurance (JKN)/Healthy Indonesia Card (KIS). The indicators of target achievement are:

- a. The productions of 52 documents of study/monitoring and evaluation results on JKN/KIS implementation.
- b. 16 documents of Health Technology Assessment (HTA) results are submitted to the Minister of Health.
- c. 14 policies documents on the realization of contribution fee of the JKN/KIS participants are produced.

3. Program for enhancing the Surveillance and Accountability of the Ministry of Health Public Officials

The target of this program is the increasing transparency of state governance and the implementation of bureaucratic reform. The indicator of this target is 100% of working units have <1% of state losses.

In order to achieve those results, activities that will be conducted are:

1) Reinforce the Surveillance on programs/activities in working units under Inspectorate I

The target is the increasing transparency of state governance and the implementation of bureaucratic reform at working units under Inspectorate I. The indicator of this target is 100% of working units in Inspectorate I have < 1% of state losses.

2) Reinforce the Surveillance on programs/activities in working units under Inspectorate II

The target is the increasing transparency of state governance and the implementation of bureaucratic reform at working units under Inspectorate II. The indicator of this target is 100% of working units in Inspectorate II have <1% of state losses.

3) Reinforce the Surveillance on programs/activities in working units under Inspectorate III

The target of this activity is the increasing transparency of state governance and the implementation of bureaucratic reform at working units under Inspectorate III. The indicator of this target is 100% of working units in Inspectorate III have <1% of state losses.

4) Reinforce the Surveillance on programs/activities in working units under Inspectorate IV

The target is the increasing transparency of state governance and the implementation of bureaucratic reform at working units under Inspectorate IV. The indicator of this target is 100% of working units in Inspectorate IV have <1% of state losses.

5) Improvement of Public Complaint Handling Service at Ministry of Health

The target of this activity is having an improvement in handling public complaints that indicate any state losses. The indicator of the target is 100% of public complaints are handled according to the authority of the Inspectorate General.

6) Management support and implementation of other technical tasks in the enhancement program of Surveillance and Accountability of the Ministry of Health Public Officials

The target is the increasing support on management and implementation of other technical tasks in the enhancement program of surveillance and accountability of the ministry of health's apparatus. The indicator of target achievement is 100% of working units have implemented the prevention and eradication of corruption program.

4. Maternal and Child Health and Nutrition Development Program

The target of this program is the increasing provision and access of quality health services for all people. The indicators are:

- a. The increasing percentage of labor at healthcare facilities by 85%.
- b. The decreasing percentage of pregnant mothers that have chronic energy deficiency to 18.2%.

In order to achieve those results, activities that will be conducted are:

1) Public Nutrition Improvement Development

The target of this activity is the improvement of public nutrition service delivery. The indicators of target achievement are:

- a) 95% of pregnant mothers with chronic energy deficiency receive supplementary food.
- b) 98% of pregnant mothers receive iron supplements tablet (TTD).
- c) 50% of infants less than 6 months old have exclusive breastfeeding.
- d) 50 % of newborn babies have early initiation of breastfeeding (IMD).
- e) 90% of under-fives suffer from wasting receive supplementary food.
- f) 30% of girls receive iron supplements tablet (TTD).

2) Health Development for Infant, Children and Adolescent

The target of this activity is the increasing access and quality of health services for infants, children and adolescents. The indicators of this target are:

- a) The percentage of Neonatus First Visit (KN1) reaches 90%.
- b) The percentage of Puskesmas that runs medical screening on first grade students reaches 70%.
- c) The percentage of Puskesmas that runs medical screening on VII and X grade students reaches 60%.
- d) The percentage of Puskesmas that organizes health activities for adolescent reaches 45%.

3) Maternal and Reproductive Health Development

The target of this activity is the increasing access and quality of maternal and reproductive health services. The indicators of this target are:

- a) The percentage of Puskesmas that organizes class program for pregnant mothers reaches 90%.
- b) 100% of Puskesmas implements the labor planning and complication prevention (P4K) program.
- c) 80% of pregnant mothers have a minimum of 4 times of antenatal care.

4) Development of Occupational Health and Sport Intervention

The target of this activity is the improvement of intervention on occupational health and sport.

The indicators of this target are:

- a) 80 % of Puskesmas implements basic occupational health program.
- b) 730 of Occupational Health Care (UKK) posts are established in Fish Landing Base/the Fish Auction Places (PPI/TPI) areas.
- c) 100% of medical checkup facilities for Indonesian workers (TKI) meet the standard.
- d) 60% of Puskesmas organizes health activities for community in their working areas.

5) Traditional and Complementary Medical Treatment Development

The target is to increase the fostering, development and monitoring on traditional and complementary medical treatments. The indicator of this target is 75% of Puskesmas delivers traditional treatment.

6) Health Operational Assistance (BOK)

The target is the provision of Health Operational Assistance (BOK) for Puskesmas. The indicators of target achievement are:

- a) 10,315 Puskesmas receives BOK.
- b) 7,737 Puskesmas publishes the utilization report of BOK on announcement board at Puskesmas or sub-district head's offices.

7) Management support and implementation of other technical tasks in Maternal and Child Health and Nutrition Development Program

The target of this activity is the improvement of management support and implementation of other

technical tasks in maternal and child health and nutrition development program. The indicator of the target is 94% of administrative activities of management support and implementation of the program are realized.

5. Diseases Control and Environmental Health Program

The target of this program is the decreasing of communicable and non-communicable diseases, and the improvement of environment health quality. The indicators of this target are:

- a. 40% of district/city meets the environment health quality.
- b. 40% of cases of diseases that can be prevented by certain immunization (PD3I) can be decreased.
- c. 100% of districts/cities have the preparedness policies to response medical emergency circumstances with outbreaks potentials.
- d. Decreasing prevalence of smoking in age group of ≤ 18 years by 5,4%

In order to achieve those results, activities that will be conducted are:

1) Development of Surveillance, Immunization, Quarantine and Comprehensive Health Intervention

The target of this activity is to decrease morbidity rate due to diseases that can be prevented by immunization enhance the surveillance, health quarantine and comprehensive health intervention. The indicators of this target are:

- a) 93% of children age 0-11 months old receives full basic immunization.
- b) 90% of early warning signals are responded.
- c) 60% of districts/cities with diving area implement comprehensive health intervention.

2) Vector Borne Diseases Control (P2B2)

The target of this activity is the reinforcement of prevention and control of diseases of animal origin. The indicators of this target are:

- a) 80% of districts/cities implement integrated vectors control.
- b) 400 of districts/cities with API <1/1,000 population.
- c) 75 districts/cities of filarial endemic areas succeed in decreasing microfilaria rate to <1%.
- d) 68% of districts/cities have IR DBD <49 per 100,000 populations.
- e) 85% of districts/cities have eliminated rabies.

3) Control of Communicable Diseases

The target is to decrease the morbidity and mortality rate due to direct communicable diseases. The indicators of this target are:

- a) The percentage of coverage in finding new cases of leprosy without defects reaches 95%.
- b) 90% of districts/cities have the success rate of positive BTA pulmonary tuberculosis treatment of minimum 85%.
- c) 55% of HIV cases are treated.

- d) 60% of districts/cities have 50% of their Puskesmas runs pneumonia medical checkups and procedures using MTBS program.
- e) 80% of districts/cities that implement early detection of hepatitis B to risk groups.

4) Non-communicable Diseases Control

Target of this activity is the decreasing morbidity and mortality rate due to non-communicable diseases; the increasing prevention and control of non-communicable diseases. The indicators of this target are:

- a) 50% of Puskesmas implement integrated non-communicable diseases (PTM) control.
- b) At minimum 50% of districts/cities and 50% of schools enforce the policy of non-smoking areas (KTR).
- c) 50% of villages/sub-districts organizes activities of the integrated health posts for non-communicable diseases.
- d) 50% of women age 30-50 undergo early detection of cervical and breast cancer.
- e) 50% of districts/cities run medical check-up for drivers at main terminal.

5) Environmental Health

The target of this activity is the improvement of environmental health and quality control. The indicators of this target are:

- a) 45,000 villages/sub-districts implement community based total sanitation (STBM).
- b) 50% of drinking water facilities are being monitored.

- c) 58% of public places meet the sanitation requirements.
- d) 36% of hospitals attain the standard in conducting medical waste treatment.
- e) 32% of food processing facility (TPM) meets a qualified of health.
- f) 386 of districts/cities create healthy environment layout.

6) Management support and implementation of other technical task in Diseases Control and Environmental Health Program

The target is the improvement of management support and implementation of other technical task in diseases control and environmental health program. The indicators of this target are:

- a) 85% of implementing-units of diseases control and environmental sanitation program obtain performance accountability system for government institution (SAKIP) score of a minimum 85%.
- b) 69% of facilities of central and regional implementing-units are improved in order to meet the standard.

6. Healthcare Provision Development Program

The target of this development program is the increasing access to quality primary and referral healthcare provision for community. The indicators of this target are:

- a. 5,600 sub-districts have at minimum one certified accredited Puskesmas each.



- b. 481 districts/cities have at minimum one accredited local hospital (RSUD) each.

In order to achieve those results, activities that will be conducted are:

1) Medical Support Intervention and Health Facilities Development

The target of this activity is the increased equality, and quality service of medical support, health facilities and medical devices. The indicators of this target are:

- a) 32% of regional hospitals as the provider of telemedicine services.
- b) 50 vertical technical implementing units and regional referral hospitals have operation cooperation (KSO) arrangement.

2) Nursing Care and Medical Technician' Services Development

The target of this activity is the improvement of quality and access to nursing, midwifery care and medical technician services. The indicator of this target is 1,015 of Puskesmas implementation of public health nursing program (Perkesmas).

3) Primary Health Intervention Development

The target of this development program is the increasing access to quality primary healthcare for community. The indicators of this target are:

- a) 6,000 of Puskesmas with outpatient and inpatient care that deliver standardized services.

- b) 150 districts/cities that organize mobile health services (PKB) in remote and the most remote areas.
- c) 9,414 units of Puskesmas that have applied Puskesmas management.
- d) 318 of districts/cities with remote/most remote areas (T/ST) have regulations on stipulation of Puskesmas with T/ST areas.
- e) 366 districts/cities are ready to undergo accreditation on primary health facilities.
- f) 5,600 of Puskesmas have collaborated with blood transfusion unit (UTD) and hospitals through the health office.

4) Referral Healthcare Intervention Development

The target of this development program is the availability of quality referral healthcare facilities that accessible by the people. The indicators of this target are:

- a) 60 units of national and regional referral hospitals implement the integration of medical records.
- b) 125 of regional referral hospitals facilities and devices (SPA) meet the specified standard.
- c) 95% of districts/cities are prepared with referral services access.
- d) 14 national referral central hospitals facilities are being upscale.
- e) The formulation of one document concerning the needs of hospital boat in archipelagic districts by 2016.

- f) 97 of regional hospitals meet the standard and specific criteria.
- g) Constructing 64 primary hospitals.

5) Mental Healthcare Development

The target of this activity is the improved quality and access to mental healthcare and narcotics, psychotropic, and addictive substances healthcare. The indicators of this target are:

- a) 50% of healthcare facilities at the recipient obligation report institution (IPWL) of active narcotic drug users.
- b) 280 of districts/cities Puskesmas administer the mental healthcare.
- c) 60% of regional referral general hospitals provide mental health/psychiatry care.

6) Management Support and Implementation of other technical tasks in Healthcare Provision Development Program

The target of this activity is the improvement of management support and implementation of other technical tasks in healthcare Provision development program. The indicators of this target are:

- a) 100% of the integrated monitoring and evaluation that runs effectively.
- b) 100% of working units receive budget allocation according to the priorities criteria.
- c) 70% of vertically technical implementing units have Renstra-based performance management system.

- d) 100% of organized vertically technical implementing units based on good performance index that are in line with the performance contract.
- e) 90% of directorate programs refer to national target areas.

7. Pharmaceutical and Medical Devices Program

The target of pharmaceutical and medical devices program is the improved access and quality of pharmacy, and medical devices stocks as well as the household medical supplies. The indicators of this target are:

- a. 90% availability of drugs and vaccines at Puskesmas level.
- b. 35 types of medicinal raw materials, traditional medicines and medical devices that are manufactured in the country.
- c. 83% of medical devices products and household medical supplies in the market are qualified.

In order to achieve those results, the following activities shall be carried out:

1) Pharmaceutical Care Improvement

The target of this activity is the improvement of pharmacy service and the rational drug use at healthcare facilities. The indicators of this target are:

- a) 60% of Puskesmas delivers pharmaceutical care according to the standard.
- b) 70% of rational drug use at Puskesmas

2) Increasing Availability of Public Medicines and Medical Supplies

The target of this activity is the provision of quality, well-distributed and affordable drugs, vaccines and medical supplies at state healthcare facilities. The indicators of this target are:

- a) 90% availability of drugs and vaccines at Puskesmas.
- b) The percentage of pharmaceutical installation in districts/cities that apply standardized management for drugs and vaccines reaches 75%.

3) Improvement of Medical Devices Production and Distribution

The target of this activity is the improvement in control of pre and post marketing of medical devices and household health supplies (PKRT). The indicators of this target are:

- a) 83% of qualified medical devices products and household health supplies in the market.
- b) 10 medical devices are manufactured in the country.
- c) 55% of medical devices and household medical supplies meet the good manufacturing standard (GMP/CPAKB).
- d) 75% of pre-market assessments are in line with the good review practices.

4) Improvement on Pharmacy Production and Distribution of Medical Devices

The target is the increasing production of raw materials and local medicines and quality improvement of pharmaceutical production and distribution facilities. The indicators of this target are:

- a) 25 types of medicinal raw materials and traditional medicines are manufactured in the country.
- b) 10 industries utilize medicinal raw materials and traditional medicines that are manufactured in the country.

5) Management support and implementation of other technical tasks in Pharmaceutical and Medical Devices Program

The target of this activity is the increasing management support and implementation of other technical tasks in pharmaceutical and medical devices program. The indicator of this target is 95% of client satisfaction is satisfied with management support.

8. Human Resources for Health Development and Empowerment Program

The target of this program is the improvement on availability and quality of human resource for health according to the health care standard. The indicators of this target are:

- a. 5,600 Puskesmas that has at minimum 5 types of human resource of health each.

- b. 60% of class C district/city hospitals that have had four basic specialist doctor and three ancillary specialist doctor.
- c. Competency improvement of 56,910 human resources for health.

In order to achieve those results, activities that will be conducted are:

1) Standardization, Certification and Continuous Education of Human Resources for Health

The target of this activity is the organization of standardization, certification, and continuous education of human resource for health. The indicator of this target is 690,000 registered human resources of health.

2) Higher Education and Quality Improvement of Human Resource for Health

The target of this activity is the increasing organization of advanced education and quality improvement of human resource for health. The indicator of this target is having 5,000 new participants of education assistance aid and scholarship.

3) Education and Training of Public Officials for Health

The target of this activity is the improvement of public officials for health education and training. The indicator of this target is 45,000 public officials for health obtaining certificate from accredited trainings.

4) Education and Training of Human Resources of Health

The target is the increasing number of education and training for Human Resource for Health. The indicator of this target is 11,910 of educators, human resources for health and members of community participate in such trainings.

5) The Management of Higher Education Quality

The target of this activity is the quality improvement of higher education. Indicator of this target is 80% of study programs in Health of Polytechnic (Poltekkes) of the Ministry of Health that have been accredited.

6) Planning and Efficient use of Human Resource for health

The target of this activity is having a better planning and efficient use of human resource for health. Indicator of this target is 24,000 human resources for health are absorbed to work in health services facilities.

7) Human Resource for Health Planning

The target of this activity is the increasing implementation of health professional planning. The indicator of this target is the creation of 15 planning documents of human resource for health.

8) Internship for Human Resources of Health

The target of this activity is to establish internship opportunity for human resources of health. Indicator for this target is 32,500 human resources of health conduct internship.

9) Management support and implementation of other technical tasks in the Development and Empowerment of Human Resource for health

The target of this activity is the improvement on management support and implementation of other technical tasks in the development and empowerment of human resource for health. The indicators of this target are:

- a) The formulation of 100 documents regulating norms, standards, procedures and criteria of the development and empowerment of human resource for health (PPSDM).
- b) The formulation of 34 documents containing data and information of PPSDM programs.

10) Development and Management of Higher Education

The improvement in higher education development and management. Indicators of this target are:

- a) Having 100,000 graduates from MOH's Poltekkes.
- b) 38 working units have their facilities upscale.

9. Health Research and Development Program

The target of this program is the quality improvement of research and development in health sector. Indicators of this target are:

- a. 35 documents containing research results are registered under HKI.
- b. Formulation of 120 recommendations of policies that are based on health research and development which are being advocated to health program manager and or to stakeholders.



- c. 5 report documents on national health research (Risksnas) in health and public nutrition.

In order to achieve those results, activities that will be conducted are:

1) Research and Development in Biomedical and Basic Medical Technology

Target of this activity is the increasing number of researches and developments in biomedical and basic medical technology. The indicators of this target are:

- a) The formulation of 25 policies recommendations which are derived from researches and development in biomedical and basic medical technology.
- b) 100 publications of scientific papers on biomedical and basic medical technology are published in national and international printed and or electronic media.

2) Research and Development on Public Health Intervention Technology

Target of this activity is the increasing number of researches and developments on public health intervention technology. The indicators of this target are:

- a) The formulation of 40 policies recommendations which are derived from researches and development in public health intervention technology.

- b) 219 publications of scientific papers on public health intervention technology are published in national and international printed and or electronic media.

3) Research and Development on Medical Applied Technology and Clinical Epidemiology

Target of this activity is the increasing number of researches and developments in medical applied technology and clinical epidemiology. The indicators of this target are:

- a) The formulation of 40 policies recommendations which are derived from researches and development in medical applied technology and clinical epidemiology.
- b) 93 publications of scientific papers on medical applied technology and clinic epidemiology are published in national and international printed and or electronic media.

4) Research and Development on Humanity, Health Policies and Community Empowerment

Target of this activity is the increasing number of researches and developments on humanity, health policies and community empowerment. The indicators of this target are:

- a) The formulation of 45 policies recommendations which are derived from researches and development in humanity, health policies and community empowerment.

- b) 125 publications of scientific papers on humanity, health policies and community empowerment are published in national and international printed and or electronic media.

5) Research and Development on Medicinal Plants and Traditional Medicines

Target of this activity is the increasing number of research and development on medicinal plants and traditional medicines. The indicators of this target are:

- a) The formulation of 10 policies recommendations which are derived from researches and development on medicinal plants and traditional medicines.
- b) 120 publications of scientific papers on medicinal plants and traditional medicines are published in national and international printed and or electronic media.

6) Research and Development on Vector and Reservoir of Diseases

The target is the increasing number of research and development on vector and reservoir of diseases. The indicators of this target are:

- a) The formulation of 10 policies recommendations which are derived from researches and development on vector and reservoir of diseases.
- b) 85 publications of scientific papers on vector and reservoir of diseases are published in national and international printed and or electronic media.

7) Management Support and Implementation of other technical tasks in Research and Development Program in Health Sector

The target of this activity is the improvement of management support and implementation of other technical tasks towards research and development program. The indicators of this target are:

- a) Formulation of 25 reports concerning management support towards research and development in health sector.
- b) Formulation of 20 reports concerning management support towards technical research and development in health sector.

B. FUNDING FRAMEWORK

The funding framework includes the increasing funding and effectiveness of funding. The increasing health funding is conducted by increasing health budget proportion significantly to reach 5% of APBN by 2019. The increasing of funding can also come from other funding assistance among others the local government, private sector and community as well as other sources from tariff/taxes and customs. In order to improve the effectiveness of health development funding, the role and authority of central-regional should also be more effective, as well as the synergy of health development between central-regional and special allocation fund (DAK) management should be well-targeted.



In order to improve the effectiveness of health financing then health funding is prioritized to increase the access and quality of health services for the poor through National Health Insurance, reinforcement of health intervention for community lives in isolated areas, islands and borders territory, the strengthening of sub-systems in national health system for supporting the intervention to reduce maternal, infant and under-five mortality rates as well as diseases control and environmental sanitation.

In order to support health interventions in the regions, the Ministry of Health allocates larger budget proportion through Special Allocation Fund (DAK), Co-Administration (TP), deconcentration, social aid and other activities designated for the regions.



CHAPTER FIVE CLOSING





CHAPTER FIVE

CLOSING

The 2015-2019 strategic plan (Renstra) of the Ministry of Health is prepared as the reference for planning, implementing and appraising the intervention of the Ministry of Health within the next five years. Thus the main unit and working unit in the Ministry of Health will have specified performance targets that will be evaluated in the middle (2017) and at the end of 5 years period (2019) according to the existing provisions.

If in the future changes are made to the 2015-2019 Ministry of Health of Strategic Plan (Renstra), appropriate amendment will be applied.

MINISTER OF HEALTH
OF THE REPUBLIC OF
INDONESIA,

[SIGNED]

NILA FARID MOELOEK





APPENDIXES



APPENDIX I

MINISTER OF HEALTH DECREE
NUMBER CONCERNING HK.02.02/MENKES/52/2015
ON
THE MINISTRY OF HEALTH STRATEGIC PLAN
YEARS 2015-2019

**PERFORMANCE TARGET MATRIX OF THE
MINISTRY OF HEALTH STRATEGIC PLAN YEARS
2015-2019**

APPENDIX I
MINISTER OF HEALTH DECREE
NUMBER CONCERNING HK.02.02/MENKES/52/2015
ON
THE MINISTRY OF HEALTH STRATEGIC PLAN
YEARS 2015-2019

PERFORMANCE TARGET MATRIX OF THE MINISTRY OF HEALTH STRATEGIC PLAN YEARS 2015-2019

NO	PROGRAM/ACTIVITY	OBJECTIVE	INDICATOR		CALCULATION METHOD	BASELINE (2014)	TARGET					EXECUTING ORGANIZATION UNIT
							2015	2016	2017	2018	2019	
(1)	(2)	(3)	(4)		(5)	(6)	(7)	(8)	(9)	(10)	(11)	(12)
I	SUPPORTING PROGRAM FOR MANAGEMENT AND IMPLEMENTATION OF OTHER TECHNICAL TASKS OF MOH	Improved coordination of tasks implementation, development and delivery of management support of the MOH	1	The number of formulation health-oriented public policies			3	3	3	3	3	SECRETARIAT GENERAL
			2	The percentage of the alignment between management support and implementation of other technical tasks	The number of work achievements of the Central/Bureau offices divided by the number of Central/Bureau offices	-	90%	92%	94%	96%	98%	
1	Planning and Budgeting of Health Development Program	The increasing quality of planning and budgeting of health development program	1	The number of provinces that have five years terms plan and an integrated health budget from various sources	Provinces with a five-year plan and integrated health budget from various sources	-	9	16	25	30	34	BUREAU OF PLANNING AND BUDGETING

NO	PROGRAM/ACTIVITY	OBJECTIVE	INDICATOR		CALCULATION METHOD	BASELINE (2014)	TARGET					EXECUTING ORGANIZATION UNIT
							2015	2016	2017	2018	2019	
(1)	(2)	(3)	(4)		(5)	(6)	(7)	(8)	(9)	(10)	(11)	(12)
			2	The number of quality document of planning policies, budget and evaluation of health development	Documents categorized based on the duties and functions of the work units	24	25	25	25	26	26	BUREAU OF PLANNING AND BUDGETING
			3	The number of recommendations of integrated monitoring-evaluation	Integrated monitoring and evaluation recommendations that are produced	-	34	34	34	34	34	
2	Personnel Administration Development	The improving service delivery of personnel administration	1	The percentage of the fulfillment for public officials for health	The employment of Civil Servant Candidates (CPNS) and Contract Workers (PTT/P3K) to the number of formation of CPNS and PTT/P3K per year	90%	90%	90%	90%	90%	90%	BUREAU OF PERSONNEL
			2	The percentage of structural officials in the scope of Ministry of Health whose competency are relevant to the job	The number of structural officials with the required competency standards to the total number of	-	60%	70%	80%	85%	90%	
			3	The percentages of Ministry of Health personnel whose performance values are at minimum satisfactory level	The number of CPNS and PNS with an SKP score of, at minimum satisfactory level to the total number of CPNS and PNS	-	80%	85%	88%	91%	94%	

NO	PROGRAM/ACTIVITY	OBJECTIVE	INDICATOR	CALCULATION METHOD	BASELINE (2014)	TARGET					EXECUTING ORGANIZATION UNIT
						2015	2016	2017	2018	2019	
(1)	(2)	(3)	(4)	(5)	(6)	(7)	(8)	(9)	(10)	(11)	(12)
3	Development of Financial Administration and State Assets Management	The increasing quality of Financial and State Assets (BMN) management of the Ministry of Health that is effective and efficient and reported according to the rules	1	The percentage of work units that submit a timely and quality financial statement according to the Government Accounting System (SAP) in order to maintain the Unqualified Opinion (WTP)	The number of Work Units in the Central and Regional Offices that deliver financial reports divided by the total number of Work Units in the Central and Regional Offices	2%	100%	100%	100%	100%	BUREAU OF FINANCE AND STATE ASSETS
			2	The percentages of fixed asset values that have been granted by the Stipulation of Utilization Status (PSP)	Fixed assets valuation that have gained PSP divided by fixed assets valuation of audited financial reports	-	30%	50%	70%	90%	
			3	The percentage of procurement that uses e-procurement	The number of Work Units in the Central and Regional Office using LPSE divided by the total number of Work Units in the Central and Regional Office	90%	65%	80%	90%	100%	

NO	PROGRAM/ACTIVITY	OBJECTIVE	INDICATOR	CALCULATION METHOD	BASELINE (2014)	TARGET					EXECUTING ORGANIZATION UNIT
						2015	2016	2017	2018	2019	
(1)	(2)	(3)	(4)	(5)	(6)	(7)	(8)	(9)	(10)	(11)	(12)
4	The Formulation of Laws and Regulation and Organization	The increasing number of laws and regulation, delivery of legal services, organization and procedures	1	The number of legal products in health sector:							BUREAU OF LAW ORGANIZATION
			a	The number of drafts bill (RUU), drafts of government regulations (RPP), draft of presidential regulation/decrees (Keppres)/presidential instruction (Inpres) that have	The trends of the results from the previous years compared to the estimated needs of the upcoming years	15	5	5	5	5	
			b	The number of drafts of the Minister of Health Regulations/Decrees	The trends of the results from the previous years compared to the estimated for the upcoming years	75	75	75	75	75	
			2	a. Number of case handling related to assets	The trends of the results from the previous years compared to the estimated needs in the upcoming years	60	60	60	60	60	
			b	Number of legal case handling	The trends of the results from the previous years compared to the estimated needs in the upcoming years	35	35	35	35	35	
			c	The number of work agreements in the health sector	The trends of the results from the previous years compared to the estimated needs in the upcoming years	30	30	30	30	30	

NO	PROGRAM/ACTIVITY	OBJECTIVE	INDICATOR		CALCULATION METHOD	BASELINE (2014)	TARGET					EXECUTING ORGANIZATION UNIT
							2015	2016	2017	2018	2019	
(1)	(2)	(3)	(4)		(5)	(6)	(7)	(8)	(9)	(10)	(11)	(12)
			3	The number of products of organization and job procedures and analysis	The trends of the results from the previous years compared to the estimated needs in the upcoming years		12	8	8	8	8	BUREAU OF LAW AND ORGANIZATION
			4	The number of procedures, state affairs organization products in health sector, performance accountability and functional position	The trends of the results from the previous years compared to the estimated needs in the upcoming years		15	15	15	15	15	

NO	PROGRAM/ACTIVITY	OBJECTIVE	INDICATOR		CALCULATION METHOD	BASELINE (2014)	TARGET					EXECUTING ORGANIZATION UNIT
							2015	2016	2017	2018	2019	
(1)	(2)	(3)	(4)		(5)	(6)	(7)	(8)	(9)	(10)	(11)	(12)
5	The Management of Administration, Protocols, Household Affairs, Financial and Payroll	The increasing quality of correspondence administration, well-organization of events and activities of the leadership	1	The Percentage of correspondence administrations, organization of events and activities for the decision maker are well and smoothly implemented according to the rules	A = The number of completed correspondence divided by the total number of correspondence multiplied by one hundred percent B = The number of daily events that are well-executed divided by the total number of daily activities multiplied by one hundred percent. The cumulative of A plus B divided by 2 equals to the target	90%	91%	92%	93%	94%	95%	BUREAU OF GENERAL AFFAIRS
		The increasing quality of documentation service for overseas official travel, official paperwork and archive management in the Ministry of Health environment	1	The percentage of documents service for overseas official travel is delivered in timely manner	The number of staff documents for overseas trips prepared maximum 10 days after the date of proposal	90%	91%	92%	93%	94%	95%	

NO	PROGRAM/ACTIVITY	OBJECTIVE	INDICATOR	CALCULATION METHOD	BASELINE (2014)	TARGET					EXECUTING ORGANIZATION UNIT
						2015	2016	2017	2018	2019	
(1)	(2)	(3)	(4)	(5)	(6)	(7)	(8)	(9)	(10)	(11)	(12)
			2 The percentage of central work unit's archives are well-developed and The percentage of regional technical implementing unit's archives and official paperwork are well-developed	The number of Central Office Work Units with organized archiving divided by the total number of Work Units multiplied by one hundred percent	75%	80%	82%	85%	88%	90%	BUREAU OF GENERAL AFFAIRS
				The number of UPT Work Units with organized archives and documentation of missions divided by the total number of Work Units multiplied by one hundred percent	60%	60%	65%	70%	75%	80%	

NO	PROGRAM/ ACTIVITY	OBJECTIVE	INDICATOR	CALCULATION METHOD	BASELINE (2014)	TARGET					EXECUTING ORGANIZATION UNIT
						2015	2016	2017	2018	2019	
(1)	(2)	(3)	(4)	(5)	(6)	(7)	(8)	(9)	(10)	(11)	(12)
		An improve management of MOH's office	1	The percentage provision of office means and facilities	SP -1 = The number of M2 development, renovation, rehabilitation of the office buildings divided by the number of actual M2 development, renovation, and rehabilitation of the office buildings divided by one hundred percent. SP-2 = The number of procurement of office equipment divided by the number of the actual office equipment multiplied by one hundred percent. SP-3 = The number of services and maintenance of the office equipment and facilities divided by the number of actual services and maintenance multiplied by one hundred percent. IKK-2=The total sum of SP divided by three	100%	100%	100%	100%	100%	BUREAU OF GENERAL AFFAIRS
		Improving the quality of management of the salaries payment and/or strategic human resources of health in order to support the achievement indicators of health development program 2015-2019	1	Total percentage of payment and/or incentives for strategic human resources of health are well-targeted.	The number of strategic and well-targeted health workers divided by the total number of employees multiplied by 100 percent	90%	92%	93%	94%	95%	

NO	PROGRAM/ ACTIVITY	OBJECTIVE	INDICATOR	CALCULATION METHOD	BASELINE (2014)	TARGET					EXECUTING ORGANIZATION UNIT	
						2015	2016	2017	2018	2019		
(1)	(2)	(3)	(4)		(5)	(6)	(7)	(8)	(9)	(10)	(11)	(12)
6	Health Data and Management Information	The improving management of health data and information	1	The percentage of districts/cities that report prioritized health data	The number of Regencies/Cities that submit reports divided by the total number of Regencies/Cities	20%	30%	40%	50%	60%	70%	CENTER FOR HEALTH DATA AND INFORMATION
			2	The percentage of provision of data communication network that is designated for e-health service access	The number of Regencies/Cities with available connections divided by the total number of Regencies/Districts	-	10%	20%	30%	40%	50%	
7	Health Promotion and Community Empowerment	The improving implementation of community empowerment and health promotion	1	The formulation of health-oriented public policies to enhance the life quality of Indonesian people.	The number of health-concerned public policies	3	3	3	3	3	3	CENTER OF HEALTH PROMOTION
			2	The percentage of districts/cities that implement clean and healthy lifestyle policies	(The number of Regencies/Cities that create policies which support PHBS, minimum 1 new policy per year divided by the number of regencies and cities) x 100%	30%	40%	50%	60%	70%	80%	
			3	The percentage of villages that utilize the 10% village fund for community based healthcare program	(The number of Villages that use at least 10% of their village funds for Community-Based Health Efforts (UKMB) divided by the total number of villages) x 100%	-	10%	20%	30%	40%	50%	

NO	PROGRAM/ ACTIVITY	OBJECTIVE	INDICATOR	CALCULATION METHOD	BASELINE (2014)	TARGET					EXECUTING ORGANIZATION UNIT
						2015	2016	2017	2018	2019	
(1)	(2)	(3)	(4)	(5)	(6)	(7)	(8)	(9)	(10)	(11)	(12)
			4 The number of business entities utilize their CSR for health program	The number of business sectors that create MOUs with the Ministry of Health in supporting health programs	4	4	8	12	16	20	CENTER OF HEALTH PROMOTION
			5 The number of civil society organizations utilize their resources to support health sector	The number of community organizations that create MOUs with the Ministry of Health in support of health programs	3	3	6	9	12	15	
8	Health Crisis Management	The improving intervention of health crisis risks reduction	1 The number of districts/cities receive support in order to implement health crisis risk reduction intervention in their areas	Calculate the number of Regencies/Cities that have received counseling in an effort to reduce health crisis risks in their areas	300	34	34	34	34	34	CENTER FOR HEALTH CRISIS MANAGEMENT
			2 The number of provinces receive advocacy and outreach to support the implementation of health crisis risk reduction intervention in their areas	The number of Provinces that have received advocacy and socialization to support efforts in reducing health crisis risks in the areas	34	7	7	7	7	6	

NO	PROGRAM/ACTIVITY	OBJECTIVE	INDICATOR	CALCULATION METHOD	BASELINE (2014)	TARGET					EXECUTING ORGANIZATION UNIT	
						2015	2016	2017	2018	2019		
(1)	(2)	(3)	(4)		(5)	(6)	(7)	(8)	(9)	(10)	(11)	(12)
9	Public Communication Management	The improving management of public communication	1	The number of publications on health are disseminated to public	The total number of publications distributed in the society by the Center of Public Communication through print and electronic media, press release, social media (facebook, twitter, YouTube, websites, publishing companies, and face-to-face media (socialization, meetings).	1,050	7,499	7,614	7,727	7,840	7,953	CENTER OF PUBLIC COMMUNICATION
			2	The percentage of information request and complaint services that are completed	The number of information services and settled public complaints divided by the number of information requests and complaints received through the SIAP application (Halo Kemkes, email, pojok info, PPID, LAPOR, incoming letters and SMS)	90%	90%	90%	90%	90%	90%	

NO	PROGRAM/ACTIVITY	OBJECTIVE	INDICATOR		CALCULATION METHOD	BASELINE (2014)	TARGET					EXECUTING ORGANIZATION UNIT
							2015	2016	2017	2018	2019	
(1)	(2)	(3)	(4)		(5)	(6)	(7)	(8)	(9)	(10)	(11)	(12)
10	Health Intelligence Enhancement	The optimum improvement on intellectual health in order to prepare quality manpower.	1	The number of production of instruments for increasing and countering intellectual health problems according to the lifecycle stages to support the development of public education in preparing quality manpower	According to the 7 stages in the life cycle, namely: 1. Pre-birth/maternal stage; 2. Infant stage; 3. Under five stage; 4. Childhood stage; 5. Adolescent stage; 6. Adulthood stage 7. Elderly stage	9	7	7	7	7	7	CENTER OF HEALTH INTELLIGENCE
11	The Improving Health of the Hajj Pilgrims	The increasing preparedness of effective and required health services in Saudi Arabia	1	The percentages of health checkup results of the pilgrims (3 months before the operation)	The number of health examination results inserted in the SISKOHATKES three months before operations divided by the PILGRIM quota of the current year multiplied by 100%	-	60%	65%	70%	75%	80%	HAJJ HEALTH CENTER

NO	PROGRAM/ACTIVITY	OBJECTIVE	INDICATOR	CALCULATION METHOD	BASELINE (2014)	TARGET					EXECUTING ORGANIZATION UNIT
						2015	2016	2017	2018	2019	
(1)	(2)	(3)	(4)	(5)	(6)	(7)	(8)	(9)	(10)	(11)	(12)
12	Enhancement International Cooperation	The better role and position of Indonesia in international cooperation in health sector	1 The number of formulation of cooperation agreements in health sector	The number of signed international agreements, including international conferences agreements related to governance that have been implemented by the Ministry of Health to achieve the strategic objectives of health development measured through periodic and comprehensive	30	8	9	8	7	8	CENTER FOR INTERNATIONAL COOPERATION
13	The Management of Indonesian Medical Council	The improvement of registration service and standardization of professional education, development and handling of disciplinary violation cases on doctors and dentists	1 The number of cases of disciplinary violation of doctors and dentists are solved	The average number of complaints within the last 3 years amounting to 30 to 40 complaints. And which are followed up as disciplinary violations by Doctors/Dentists	30	37	37	37	37	37	INDONESIAN MEDICAL COUNCIL
			2 The number of Letters of Registration (STR) of doctors and dentists are registered and delivered in timely manner	The estimated number of freshly graduated Doctors and Dentists, plus re-enrolled students, Indonesian citizens who graduated abroad, foreign citizens who graduated in Indonesia, and local graduated Doctors and Dentists who want to work or study abroad	12,000	20,000	72,000	35,000	20,000	20,000	

NO	PROGRAM/ACTIVITY	OBJECTIVE	INDICATOR		CALCULATION METHOD	BASELINE (2014)	TARGET					EXECUTING ORGANIZATION UNIT
							2015	2016	2017	2018	2019	
(1)	(2)	(3)	(4)		(5)	(6)	(7)	(8)	(9)	(10)	(11)	(12)
II	ENHANCEMENT PROGRAM FOR THE IMPLEMENTATION OF NATIONAL HEALTH INSURANCE (JKN)/ HEALTHY INDONESIA CARD (KIS)	The enhancement of National Health Insurance (JKN)/Healthy Indonesia Card (KIS)	1	The number of people as Contribution Beneficiaries (PBI) by participating in national health insurance (JKN)/Healthy Indonesia Card (KIS)	The number of PBI participants as stipulated by law multiplied by the pre-determined premium amount and multiplied by twelve years	86.4	92.2	103.5	105.6	107.8	109.9	CENTER FOR HEALTH FUNDING AND HEALTH INSURANCE
1	Development of Health Financing and National Health Insurance (JKN)/Healthy Indonesia Card (KIS)	The formulation of technical policies on development of Health Financing and National Health Insurance (JKN)/Healthy Indonesia Card (KIS)	1	The number of productions of documents study/monitoring and evaluation results on JKN/KIS implementation	Documents categorized based on studies/reviews/monitoring and evaluation of health funding and JKN	6	10	10	10	10	12	
			2	The number of documents of Health Technology Assessment (HTA) results are submitted to the Minister of Health	Documents categorized based on the produced HTA studies/analysis	-	2	2	4	4	4	
			3	The number of policies documents on the realization of contribution fee of the JKN/KIS participants are produced	Documents categorized according to the policies for the funding of PBI for JKN/KIS	1	2	3	3	3	3	
III	PROGRAM FOR ENHANCING THE INTERNAL CONTROL AND ACCOUNTABILITY OF THE MINISTRY OF HEALTH' PUBLIC OFFICIALS	The increasing transparency of good governance and the implementation of bureaucratic reform	1	The percentage of of working units have <1% of state losses	(The total number of Work Units managing the APBN of the Ministry of Health with state loss findings score of <1% based on the audit results) x 100% of the total audited work units managing the APBN for the Ministry of Health	85%	88%	91%	94%	97%	100%	INSPEKTORAT GENERAL

NO	PROGRAM/ACTIVITY	OBJECTIVE	INDICATOR	CALCULATION METHOD	BASELINE (2014)	TARGET					EXECUTING ORGANIZATION UNIT	
						2015	2016	2017	2018	2019		
(1)	(2)	(3)	(4)		(5)	(6)	(7)	(8)	(9)	(10)	(11)	(12)
1	Reinforce the internal control on program/activities in working unit under Inspectorate I	The increasing transparency of state governance and the implementation of bureaucratic reform at working units under Inspectorate I	1	The percentage of working units in Inspectorate I have < 1% of state losses	(The number of Work Units under Inspectorate I managing the APBN for the Ministry of Health with state loss findings score of <1% based on audit results) x 100% of the number of audited Work Units under Inspectorate I	80%	84%	88%	92%	96%	100%	INSPECTORATE I
2	Reinforce the internal control on program/activities in working unit under Inspectorate II	The increasing transparency of state governance and the implementation of bureaucratic reform at working units under Inspectorate II	1	The percentage of working units in Inspectorate II have < 1% of state losses	(The number of Work Units under Inspectorate II managing the APBN of the Ministry of Health with state loss findings score of <1% based on audit results) x 100% of the number of audited Work Units under Inspectorate II managing the APBN for the Ministry of Health	88%	90%	92%	94%	96%	100%	INSPECTORATE II
3	Reinforce the internal control on program/activities in working unit under Inspectorate III	The increasing transparency of state governance and the implementation of bureaucratic reform at working units under Inspectorate III	1	The percentage of working units in Inspectorate III have < 1% of state losses	(The number of Work Units under Inspectorate III managing the APBN for the Ministry of Health with state loss findings score of <1% based on the audit results) x 100% Number of audited Work Units managing the APBN for the Ministry of Health in under the Counsel of Inspectorate III	93%	94%	95%	96%	97%	100%	INSPECTORATE III

NO	PROGRAM/ACTIVITY	OBJECTIVE	INDICATOR	CALCULATION METHOD	BASELINE (2014)	TARGET					EXECUTING ORGANIZATION UNIT	
						2015	2016	2017	2018	2019		
(1)	(2)	(3)	(4)		(5)	(6)	(7)	(8)	(9)	(10)	(11)	(12)
4	Reinforce the internal control on program/activities in working unit under Inspectorate IV	The increasing transparency of state governance and the implementation of bureaucratic reform at working units under Inspectorate IV	1	The percentage of working units in Inspectorate IV have < 1% of state losses	(The number of work units under the Inspectorate IV managing the APBN for the Ministry of Health with state loss findings score of <1% based on the audit results) x 100% number of audited Work Units under Inspectorate IV managing the APBN for the Ministry of Health	78%	80%	85%	90%	95%	100%	INSPEKTORAT IV
5	Improvement of Public Complaint Handling Service at Ministry of Health	Improvement in handling public complaints that indicate any state losses	1	The percentage of public complaints are handled according to the authority of the Inspectorate General	(The number of public complaints indicating state loss that have been handled by authority of the Inspectorate General) x 100% Number of public complaints indicating state loss received by authority of the Inspectorate General	100%	100%	100%	100%	100%	100%	INSPEKTORATE OF INVESTIGATION
6	Management support and implementation of other technical tasks in the enhancement program of Internal Control and Accountability of the Ministry of Health Public Officials	The increasing support on management and implementation of other technical tasks in the enhancement program of Internal Control and accountability of the ministry of health's public officials	1	The percentage of working units have implemented the prevention and eradication of corruption program	The number of Work Units at the Central and Regional Offices that have applied the corruption prevention and eradication action program) x 100% of the Number of Work Units at Central and Regional Offices of the Ministry of Health	20%	20%	40%	60%	80%	100%	SECRETARIATE OF THE INSPEKTORATE GENERAL

NO	PROGRAM/ACTIVITY	OBJECTIVE	INDICATOR		CALCULATION METHOD	BASELINE (2014)	TARGET					EXECUTING ORGANIZATION UNIT
							2015	2016	2017	2018	2019	
(1)	(2)	(3)	(4)		(5)	(6)	(7)	(8)	(9)	(10)	(11)	(12)
IV	MATERNAL AND CHILD HEALTH AND NUTRITION PROGRAM	Increased availability and reach of high-quality health services for the whole society.	1	The percentage of deliveries in healthcare facilities	(The number of women giving birth in Puskesmas operational areas and receiving standard assistance by health workers in health facilities within a one year period divided by the number of targeted deliveries in Puskesmas operational areas within the same one year period) x 100%	70.4%	75%	77%	79%	82%	85%	DIRECTORATE GENERAL OF MATERNAL, CHILD HEALTH AND NUTRITION
			2	The percentage of pregnant women with Chronic Energy Deficiency (CED)	(The number of pregnant women with upper arm circumference of < 23,5cm / number of pregnant women with measured upper arm circumference x 100%	24.2%	24.2%	22.7%	21.2%	19.7%	18.2%	
1	Nutrition Improvement Program	Improved nutritional services to the community	1	The percentage of pregnant women with CED receiving food supplement (PMT)	(The percentage of pregnant women with CED that receive food supplement (PMT) within a certain area/the total number of pregnant women with CED within the	N/A	13%	50%	65%	80%	95%	DIRECTORATE OF NUTRITION
			2	The percentage of pregnant women receiving Tablet for Blood Supplement (TTD)	(The number of pregnant women receiving TTD within a certain area/the entire number of pregnant women within the same area) x 100%	82%	82%	85%	90%	95%	98%	

NO	PROGRAM/ACTIVITY	OBJECTIVE	INDICATOR	CALCULATION METHOD	BASELINE (2014)	TARGET					EXECUTING ORGANIZATION UNIT	
						2015	2016	2017	2018	2019		
(1)	(2)	(3)	(4)		(5)	(6)	(7)	(8)	(9)	(10)	(11)	(12)
			3	The percentage of Infants under six months receiving an exclusive breastfeeding	(The number of infants under six months receiving exclusive breastfeeding within a certain area / total number of infants under six months within the area) x 100%	38%	39%	42%	44%	47%	50%	DIRECTORATE OF NUTRITION
			4	The percentage of newborn infants receiving Early Initiation of Breastfeeding (IMD)	(The number of newborn infants receiving IMD / total number of babies x 100%	35%	38%	41%	44%	47%	50%	
			5	The percentage of underweight children under five receiving food supplement	(The number of underweight children under five receiving food supplement / the total number of weighed children under five) x 100%	N/A	70%	75%	80%	85%	90%	
			6	The percentage of adolescent girls receiving Tablet for Blood Supplement (TTD)	(The number of adolescent girls receiving TTD in a certain area / total number of adolescent girls in the area) x 100%	N/A	10%	15%	20%	25%	30%	

NO	PROGRAM/ACTIVITY	OBJECTIVE	INDICATOR		CALCULATION METHOD	BASELINE (2014)	TARGET					EXECUTING ORGANIZATION UNIT
							2015	2016	2017	2018	2019	
(1)	(2)	(3)	(4)		(5)	(6)	(7)	(8)	(9)	(10)	(11)	(12)
2	Healthcare provision for Infants, Children and Adolescents	Improved access and quality of healthcare provision for infants, children, and adolescents	1	The percentage of first prenatal visit (KN1)	(The number of newborn infants receiving 1 standard service in a Prenatal Visit at the age of six to 48 hours in a certain area within a certain period/the total number of targeted infants in the area within the same period of time) x 100%	75%	75%	78%	81%	85%	90%	DIRECTORATE OF CHILD HEALTH
			2	The percentage of Puskesmas conducting health screenings for grade-one students	(The number of Puskesmas conducting health screening for grade-one elementary school (SD/MI) students in an area within one year/the number of Puskesmas in the area within the same year) x 100%	N/A	50%	55%	60%	65%	70%	
			3	The percentage of Puskesmas conducting health screening for grade seven and grade ten students	(The number of Puskesmas conducting health screening for grade seven junior high school (SMP/MTs) students and grade ten high school (SMA/MA) students in an area within a year / the number of Puskesmas in the area within the same year) x100%	N/A	30%	40%	50%	55%	60%	

NO	PROGRAM/ACTIVITY	OBJECTIVE	INDICATOR	CALCULATION METHOD	BASELINE (2014)	TARGET					EXECUTING ORGANIZATION UNIT
						2015	2016	2017	2018	2019	
(1)	(2)	(3)	(4)	(5)	(6)	(7)	(8)	(9)	(10)	(11)	(12)
			4 The percentage of Puskesmas conducting health services for adolescents	The number of Puskesmas qualified to conduct health services for adolescents in their operational areas in a year/the number of Puskesmas in the area in the same year) x 100%	21%	25%	30%	35%	40%	45%	DIRECTORATE OF CHILD HEALTH
3	Maternal and Reproductive Healthcare	Improved access and quality of Maternal and Reproductive Healthcare Provision	1 Percentage of Puskesmas conducting maternity classes	(The number of Puskesmas conducting maternal classes / total number of District Puskesmas x 100%	27%	78%	81%	84%	87%	90%	DIRECTORATE OF CHILD HEALTHCARE
			2 The percentage of Puskesmas conducting orientations for the Birth Planning and Complication Prevention Program (P4K)	(The number of Puskesmas implementing orientations for the Birth Planning and Complication Prevention Program / total number of Puskesmas) x 100%	72%	77%	83%	88%	95%	100%	
			3 Percentage of pregnant women receiving antenatal services minimum 4 times (K4)	(The number of pregnant women that have received antenatal services minimum 4 times by health workers in an operation area within 1 year)/(number of targeted pregnant women in the area within 1 year) x100%	70%	72%	74%	76%	78%	80%	

NO	PROGRAM/ACTIVITY	OBJECTIVE	INDICATOR	CALCULATION METHOD	BASELINE (2014)	TARGET					EXECUTING ORGANIZATION UNIT	
						2015	2016	2017	2018	2019		
(1)	(2)	(3)	(4)		(5)	(6)	(7)	(8)	(9)	(10)	(11)	(12)
4	Development of occupational health and sports	Improved efforts for occupational health and sports	1	Percentage of Puskesmas implementing basic occupational health	(The number of Puskesmas that have applied Work Safety and Security (K3) and conducted promotive and/or preventive and/or curative and/or rehabilitative measures for workers in their area of operation)/(number of Puskesmas in Indonesia) x 100%	1034	40%	50%	60%	70%	80%	DIRECTORATE OF OCCUPATIONAL HEALTH AND SPORTS
			2	Number of UKK posts established in PPI/TPI areas	The Numbers of UKK posts established by communities and facilitated by Puskesmas in PPI/TPI areas	105	230	355	480	605	730	
			3	The percentage of qualified facilities for the health examination of Indonesian migrant workers (TKI)	(The numbers of qualified health examination facilities for TKI) / the number of health examination facilities for TKI) x 100%	101	100%	100%	100%	100%	100%	
			4	The percentage of Puskesmas conducting sports health activities in communities within their areas of operation	(The numbers of Puskesmas conducting qualified sports and health activities / The number of Puskesmas in Indonesia x 100%	671	20%	30%	40%	50%	60%	

NO	PROGRAM/ACTIVITY	OBJECTIVE	INDICATOR	CALCULATION METHOD	BASELINE (2014)	TARGET					EXECUTING ORGANIZATION UNIT
						2015	2016	2017	2018	2019	
(1)	(2)	(3)	(4)	(5)	(6)	(7)	(8)	(9)	(10)	(11)	(12)
5	Traditional and Complementary Health Development	Improved efforts in the management, development, and monitoring of traditional and complementary health	1 The percentage of Puskesmas implementing traditional health	(The percentage of Puskesmas implementing traditional health) / (the total number of Puskesmas) x 100%	12%	15%	25%	45%	60%	75%	DIRECTORATE OF OCCUPATIONAL HEALTH AND SPORTS
6	Health Operational Assistance (BOK)	The Provision of Health Operational Assistance (BOK) for Puskesmas	1 The number of Puskesmas that receive BOK	The number of Puskesmas that use BOK funds	9,655	9,719	9,865	10,013	10,163	10,315	SECRETARIATE OF THE DIRECTORATE GENERAL OF NUTRITION AND MATERNAL AND CHILD HEALTH
			2 The number of Puskesmas that publish BOK utilization reports on the Puskesmas message board or at the district office	The number of Puskesmas that publish the utilization of BOK reports on the Puskesmas message board or at the district office	5,000	7,289	7,399	7,510	7,622	7,737	

NO	PROGRAM/ACTIVITY	OBJECTIVE	INDICATOR	CALCULATION METHOD	BASELINE (2014)	TARGET					EXECUTING ORGANIZATION UNIT
						2015	2016	2017	2018	2019	
(1)	(2)	(3)	(4)	(5)	(6)	(7)	(8)	(9)	(10)	(11)	(12)
7	Management Support and Implementation of Other Technical Tasks in Maternal, Child Health and Nutrition Program	Improvement in management support and implementation of other technical tasks in Maternal, Child Health and Nutrition Program	Realization percentage of administration activities in management support and implementation of other technical tasks in Maternal, Child Health and Nutrition Program	(Amount of used budget and amount of activities carried out) / (total budget and total activity output) x 100	85%	90%	91%	92%	93%	94%	SECRETARIATE OF THE DIRECTORATE GENERAL OF NUTRITION AND MATERNAL AND CHILD HEALTH
V	DISEASE CONTROL AND ENVIRONMENTAL HEALTH PROGRAM	Decreasing numbers in communicable disease, non-communicable disease and improvement of the environment quality	1	Percentage of Districts/cities that meet environmental health quality	(Cumulative number of Districts/cities that meet at least 4 criteria) / (total number of Districts/cities) x 100% in a certain period	15.3%	20%	25%	30%	35%	DIRECTORATE GENERAL OF DISEASE CONTROL AND ENVIRONMENTAL HEALTH
			2	Percentage of decline in certain cases of Diseases Preventable by Immunization (PD3I)	(Number of certain PD3I cases in baseline) – (Number of certain PD3I cases in current year) / (Number of certain PD3I cases in baseline in 2013) x 100%	-	7%	10%	20%	30%	

NO	PROGRAM/ACTIVITY	OBJECTIVE	INDICATOR	CALCULATION METHOD	BASELINE (2014)	TARGET					EXECUTING ORGANIZATION UNIT
						2015	2016	2017	2018	2019	
(1)	(2)	(3)	(4)	(5)	(6)	(7)	(8)	(9)	(10)	(11)	(12)
			3 Percentage of Districts/cities that have policy of readiness in overcoming people's health emergencies that potentially have epidemic threats	Number of Districts/cities with harbors, airports, and PLBDN that have policy of readiness in overcoming PHEIC divided by Number of Districts/cities with harbors, airports, and PLBDN times 100% Note: Criteria of PLBDN harbors, airports : 1. International 2. Routine work all year long 3. Health quarantine, Immigration, and Customs elements available (Number of Districts/cities with the above criteria in 2014) / 106 Districts/cities	11%	29%	46%	64%	82%	100%	DIRECTORATE GENERAL OF DISEASE CONTROL AND ENVIRONMENTAL HEALTH
			4 Percentage of decline in smoking prevalence of ≤ 18 years old	(Number of population aged five to 18 years with smoking behaviors) / (all populations aged five to 18 years old) x 100%	7.2%	6.9%	6.4%	5.9%	5.6%	5.4%	

NO	PROGRAM/ACTIVITY	OBJECTIVE	INDICATOR		CALCULATION METHOD	BASELINE (2014)	TARGET					EXECUTING ORGANIZATION UNIT
							2015	2016	2017	2018	2019	
(1)	(2)	(3)	(4)		(5)	(6)	(7)	(8)	(9)	(10)	(11)	(12)
1	Development of Surveillance, Immunization, Quarantine, and Matra Health	To reduce the number of illness caused by preventable diseases through immunization, development of surveillance, quarantine, and matra health	1	Percentage of children aged zero to 11 months old who has received complete basic immunization	(Numbers of babies who have received one Hepatitis B immunization; one BCG immunization; three DPT,HB and Hib immunizations); four polio immunizations; and one chickenpox immunization in a one year period) / (total number of babies in the same period) x 100%	90%	91%	91.5%	92%	92.5 %	93%	DIRECTORATE OF SURVEILLANCE, IMMUNIZATION, QUARANTINE, AND MATRAT HEALTH
			2	Percentage of early warning signals that have been responded to	(Numbers of early warning signals that have been responded to by Regency/ Municipality Health Service and its ranks in one month period) / (number of early warning signals received by Regency/Municipality Health Service and its ranks in the same period) x 100%	-	65%	70%	75%	80%	90%	DIRECTORATE OF SURVEILLANCE, IMMUNIZATION, QUARANTINE, AND MATRA HEALTH
			3	Percentage of Districts/cities with diving territories that perform matra health intervention	(Number of Districts/cities that perform diving health intervention) / (Number of Districts/cities with diving territories) x 100%	-	30%	36%	42%	51%	60%	

NO	PROGRAM/ACTIVITY	OBJECTIVE	INDICATOR		CALCULATION METHOD	BASELINE (2014)	TARGET					EXECUTING ORGANIZATION UNIT
							2015	2016	2017	2018	2019	
(1)	(2)	(3)	(4)		(5)	(6)	(7)	(8)	(9)	(10)	(11)	(12)
2	Vector Borne Disease Control	Improvement in Prevention and Eradication of Vector Borne Disease	1	Percentage of Districts/cities that perform integrated vector control	(Numbers of Districts/cities that perform vector control divided by Number of Districts/cities with endemic vector communicable disease and other zoonotic diseases) x 100%	30%	40%	50%	60%	70%	80%	DIRECTORATE OF VECTOR BORNE DISEASE CONTROL
			2	Numbers of Districts/cities with API <1 per 1.000 population	Cumulative numbersof Districts/cities with API <1 per 1.000 population	337	340	360	375	390	400	
			3	Number of Filariasis endemic Districts/cities that successfully cut microfilaria number to < 1%	Accumulation of Districts/cities that successfully cut microfilaria number to < 1%	29	35	45	55	65	75	
			4	Percentage of Districts/cities with IR DBD < 49 per 100.000 population	Numbers of Districts/cities with IR DBD < 49/100.000 population divided by total number of Districts/cities with endemic DBD (Dengue Fever) in the same year	58%	60%	62%	64%	66%	68%	
			5	Percentage of Districts/cities that eliminate Rabies	(Number of Districts/cities with endemic Rabies yang that eliminate Rabies) / (Number of Districts/cities endemic) x 100% in current year	10	25	40	55	70	85.0	

NO	PROGRAM/ACTIVITY	OBJECTIVE	INDICATOR		CALCULATION METHOD	BASELINE (2014)	TARGET					EXECUTING ORGANIZATION UNIT
							2015	2016	2017	2018	2019	
(1)	(2)	(3)	(4)		(5)	(6)	(7)	(8)	(9)	(10)	(11)	(12)
3	Direct Transferable Disease Control	Decline in numbers of illness and mortality caused by directly communicable diseases	1	Percentage of new discoveries of non-deforming leprosy cases	Number of non-deforming leprosy cases discovered divided by number of new cases discovered	80%	82%	85%	88%	91%	95%	DIRECTORATE OF DIRECT TRANSFERABLE DISEASE CONTROL
			2	Percentage of Districts/cities with pulmonary TB BTA treatment success rate of minimum 85%	Districts/cities with pulmonary TB BTA treatment success rate of minimum 85% compared to all Districts/cities x 100%	75%	78%	81%	84%	87%	90%	
			3	Percentage of HIV cases treated	(Numbers of ODHA who still receive ARV therapy) / (Numbers of ODHA who meet requirements to start ARV therapy) x 100%	42%	45%	47%	50%	52%	55%	
			4	Percentage of Districts/cities whose 50% of their Public Health Centers/Puskesmas perform Pneumonia examination and governance through MTBS programs	(Numbers of Districts/cities that discovered and perform governance according to standard of minimum 80%) / (Total numbers of all Districts/cities in Indonesia)	15%	20%	30%	40%	50%	60%	
			5	Percentage of Districts/cities that perform Hepatitis B early detection activities on risk groups	(Numbers of Districts/cities that perform Hepatitis B early detection activities on risk groups / (Total numbers of all Districts/cities in Indonesia)	2.5%	5%	10%	30%	60%	80%	

NO	PROGRAM/ACTIVITY	OBJECTIVE	INDICATOR		CALCULATION METHOD	BASELINE (2014)	TARGET					EXECUTING ORGANIZATION UNIT
							2015	2016	2017	2018	2019	
(1)	(2)	(3)	(4)		(5)	(6)	(7)	(8)	(9)	(10)	(11)	(12)
4	Non-Communicable Disease Control	Decline in number of illness and mortality caused by non-communicable disease; Improvement in prevention and eradication of non-communicable disease	1	Percentage of Puskesmas that perform integrated non-communicable disease (PTM) control	(Number of Puskesmas that perform integrated non- communicable disease (PTM) control / (Total number of Puskesmas in Indonesia) x 100%	7%	10%	20%	30%	40%	50%	DIRECTORATE OF NON-COMMUNICABLE DISEASE CONTROL
			2	Percentage of Districts/cities that implement Non Smoking Area (KTR) policy in 50% schools minimally	(Number of Districts/cities that have regulation and proof of implementation in 50% teaching-learning locations in schools) / (Total number of all Districts/cities in Indonesia) x 100%	3%	10%	20%	30%	40%	50%	
			3	Percentage of Villages/Urban Villages that carry out Non-Communicable Disease Integrated development post (PTM-Posbindu) activities	Villages/Urban Villages that carry out Non-Communicable Disease Integrated development post (PTM-Posbindu) activities / (Total number of all Villages in Indonesia) x 100%	8.4%	10%	20%	30%	40%	50%	DIRECTORATE OF NON-COMMUNICABLE DISEASE CONTROL
			4	Percentage of women aged between 30 and 50 years subject to early detection of cervix and breast cancer	(Numbers of women aged between 30 and 50 years subject to early detection of cervix and breast cancer) / (Total numbers of women aged between 30 and 50 years old in Indonesia) x 100%	1.75%	10%	20%	30%	40%	50%	

NO	PROGRAM/ACTIVITY	OBJECTIVE	INDICATOR	CALCULATION METHOD	BASELINE (2014)	TARGET					EXECUTING ORGANIZATION UNIT
						2015	2016	2017	2018	2019	
(1)	(2)	(3)	(4)	(5)	(6)	(7)	(8)	(9)	(10)	(11)	(12)
			5 Percentage of Districts/cities that perform drivers' health check at main terminals	(Numbers of Districts/cities that perform drivers' health check at main terminals) / (Total numbers of all Districts/cities in Indonesia) x 100%	-	10%	20%	30%	40%	50%	DIRECTORATE OF NON-COMMUNICABLE DISEASE CONTROL
5	Environmental Health	Improvement in environmental health and quality environmental monitoring	1 Numbers of Villages/Urban Villages that carry out STBM	Calculating total sum of Villages/Urban Villages that have been verified for carrying out STBM	18,339	25,000	30,000	35,000	40,000	45,000	DIRECTORATE OF ENVIRONMENT HEALTH
			2 Percentage of drinking water facilities monitored	(Numbers of samples checked on drinking water providers) / (numbers of samples that must be checked) x 100%	22.7%	30%	35%	40%	45%	50%	

NO	PROGRAM/ACTIVITY	OBJECTIVE	INDICATOR	CALCULATION METHOD	BASELINE (2014)	TARGET					EXECUTING ORGANIZATION UNIT
						2015	2016	2017	2018	2019	
(1)	(2)	(3)	(4)	(5)	(6)	(7)	(8)	(9)	(10)	(11)	(12)
			3	Percentage of Public Sites that meet health requirements	(Numbers of TTU that meet health requirements based on Environment Health Inspection result according to standards in 1 (one) year period) / (numbers of TTU registered in Districts/cities in the same 1 year period) x 100%	30%	50%	52%	54%	56%	DIRECTORATE OF ENVIRONMENT HEALTH
			4	Percentage of hospitals that carry out medical waste processing according to standards	(Numbers of hospitals that carry out medical waste processing according to regulations) / (numbers of hospitals) x 100%	5%	10%	15%	21%	28%	
			5	Percentage of Food processing facility (TPM) that meet health requirements	(Number of TPM that meet sanitation hygiene requirements) / (number of registered TPM) x 100%	25 (TPM meet hygiene requirements according to sanitation inspection result)	8%	14%	20%	26%	
			6	Number of Districts/cities that administer healthy area order	Total cumulative number of Districts/cities that administer healthy area order	336	346	356	366	376	

NO	PROGRAM/ACTIVITY	OBJECTIVE	INDICATOR		CALCULATION METHOD	BASELINE (2014)	TARGET					EXECUTING ORGANIZATION UNIT
							2015	2016	2017	2018	2019	
(1)	(2)	(3)	(4)		(5)	(6)	(7)	(8)	(9)	(10)	(11)	(12)
6	Management Support and Implementation of Other Technical Tasks in Disease Control and Environmental Health Programs	Improvement in management support and implementation of other technical tasks in disease control and environmental health programs	1	Percentage of Working Units (Satker) in PP and PL programs that have SAKIP assessment with AA minimum score	(Number of Working Units with AA score) / (number of Working Units with SAKIP assessment)	-	35%	40%	55%	70%	85%	SECRETARIAT OF DIRECTORATE GENERAL OF DISEASE CONTROL AND ENVIRONMENTAL HEALTH I
			2	Percentage of Central and Provincial/Municipal Working Units (Satker) whose equipment/infrastructure have been improved to meet the standards	(Number of Central Working Units and UPT that meet equipment/infrastructure standards) / (Number of Central Working Units and UPT)	-	50%	55%	60%	64%	69%	
VI	HEALTHCARE PROVISION DEVELOPMENT PROGRAM	The increasing access to quality primary and referral healthcare provision for community	1	Numbers of sub-districts have at minimum one certified accredited Puskesmas each	Total number of all Sub-districts with minimally one accredited health centers/Puskesmas in current year	-	350	700	1,400	2,800	5,600	DIRECTORATE GENERAL OF HEALTHCARE PROVISION
			2	Districts/cities have minimally one accredited local hospital (RSUD) each.	Total cumulative number of accredited Local/Provincial Public Hospital/RSUD in Districts/cities achieved each year	10	94	190	287	384	481	

NO	PROGRAM/ACTIVITY	OBJECTIVE	INDICATOR		CALCULATION METHOD	BASELINE (2014)	TARGET					EXECUTING ORGANIZATION UNIT
							2015	2016	2017	2018	2019	
(1)	(2)	(3)	(4)		(5)	(6)	(7)	(8)	(9)	(10)	(11)	(12)
1	Medical Support Intervention and Health Facilities Development	The increased equality, and quality service of medical support, health facilities and medical devices	1	Percentage of regional hospitals as the provider of telemedicine services.	(Number of regional transfer hospitals that deliver services as the provider of telemedicine) / (total number of all regional hospitals) x 100%	-	3%	6%	12%	20%	32%	DIRECTORATE OF MEDICAL SUPPORT INTERVENTION AND HEALTH FACILITIES
			2	vertically technical implementing units (UPT) and regional referral hospitals have operation cooperation (KSO) arrangement	Numbers of vertical UPTs and regional hospitals that implement operation cooperation arrangement on facilities and equipmen	10	10	10	10	10	10	
2	Nursing Care and Medical Technicians' Services Development	The improvement of quality and access to nursing, midwifery care and medical technician services	1	Numbers of Health Centers that implement Nursing care on public healthcare provision (Perkesmas)		567	637	721	812	914	1,015	DIRECTORATE OF NURSING CARE AND MEDICAL TECHNICIANS
3	Development of Basic Healthcare Provision	Improvement in people's access to quality basic healthcare provision	1	Number of Puskesmas with outpatient and inpatient care that deliver standardized services	Number of Puskesmas with outpatient and inpatient care that deliver standardized services within a current year	288	700	1,400	2,800	5,600	6,000	DIRECTORATE OF BASIC HEALTHCARE PROVISION
			2	Numbers of districts/cities that organize mobile health services (PKB) in remote and the most remote areas.		96	107	118	128	139	150	

NO	PROGRAM/ACTIVITY	OBJECTIVE	INDICATOR	CALCULATION METHOD	BASELINE (2014)	TARGET					EXECUTING ORGANIZATION UNIT
						2015	2016	2017	2018	2019	
(1)	(2)	(3)	(4)	(5)	(6)	(7)	(8)	(9)	(10)	(11)	(12)
			3 Numbers of puskesmas that have applied puskesmas' management		-	6,706	8,280	8,698	9,033	9,414	DIRECTORATE OF BASIC HEALTHCARE PROVISION
			4 Numbers of districts/cities with remote/most remote areas (T/ST) have regulations on stipulation of Puskesmas with T/ST areas.		214	229	247	265	282	318	
			5 Numbers of districts/cities are ready to undergo accreditation on primary health facilities.		-	86	210	266	313	366	
			6 Numbers of Puskesmas that have collaborated with blood transfusion unit (UTD) and hospitals through the health office.	Number of Puskesmas that with blood transfusion unit (UTD) and hospitals through the health office for donor recruitment and selection in order preparing blood supplies for mothers in labor	-	200	1600	3,000	4,400	5,600	

NO	PROGRAM/ACTIVITY	OBJECTIVE	INDICATOR		CALCULATION METHOD	BASELINE (2014)	TARGET					EXECUTING ORGANIZATION UNIT
							2015	2016	2017	2018	2019	
(1)	(2)	(3)	(4)		(5)	(6)	(7)	(8)	(9)	(10)	(11)	(12)
4	Referral Healthcare Intervention Development	Availability of quality referral healthcare facilities that accessible by the people	1	Numbers of national and regional referral hospitals implement the integration of medical records		-	-	15	30	45	60	DIRECTORATE OF REFERRAL HEALTHCARE PROVISION
			2	Number of regional referral hospitals facilities and devices (SPA) meet the specified standard		-	125	125	125	125	125	
			3	Percentages of districts/cities are prepared with referral services access.	(Numbers of districts/cities are prepared with referral services access.) / (total numbers of Districts/cities within a current year) x 100 %	50%	60%	70%	80%	90%	95%	
			4	Number of national referral central hospitals facilities are being upscale		-	14	14	14	14	14	
			5	Number of document concerning the needs of hospital boat in archipelagic districts		-	1	1	-	-	-	
			6	Number of regional hospitals meet the standard and specific criteria.		-	94	96	97	97	97	
			7	Number of constructed primary hospitals (cumulative)		24	24	34	44	54	64	

NO	PROGRAM/ACTIVITY	OBJECTIVE	INDICATOR	CALCULATION METHOD	BASELINE (2014)	TARGET					EXECUTING ORGANIZATION UNIT	
						2015	2016	2017	2018	2019		
(1)	(2)	(3)	(4)	(5)	(6)	(7)	(8)	(9)	(10)	(11)	(12)	
5	Development of Mental Healthcare	The improvement of quality and access to mental healthcare and narcotics, psychotropic, and addictive substances healthcare	1	Percentages of healthcare facilities at the recipient obligation report institution (IPWL) of active narcotic drug users	(IPWL that report their activities) x 100 % / (numbers of established IPWL within a current year)	16.5%	25%	30%	35%	40%	50%	DIRECTORATE OF MENTAL HEALTHCARE PROVISION
			2	Numbers of districts/cities Puskesmas administer the mental healthcare		50	80	130	180	230	280	
			3	Percentages of regional referral general hospitals provide mental health/psychiatry care	(Number of regional referral hospitals that administer medical psychiatric care with outpatient and inpatient service by competent medical workers) / (Number of established referral hospitals) x 100 %	13.5%	20%	30%	40%	50%	60%	

NO	PROGRAM/ACTIVITY	OBJECTIVE	INDICATOR		CALCULATION METHOD	BASELINE (2014)	TARGET					EXECUTING ORGANIZATION UNIT
							2015	2016	2017	2018	2019	
(1)	(2)	(3)	(4)		(5)	(6)	(7)	(8)	(9)	(10)	(11)	(12)
6	Management Support and Implementation of other technical tasks in Healthcare Provision Development Program	The improvement of management support and implementation of other technical tasks in healthcare Provision development program	1	the integrated monitoring and evaluation that runs effectively	(Numbers of integrated evaluation executions that run effectively) / (all integrated evaluation executions) x 100%	-	30%	40%	60%	80%	100%	SECRETARIAT DIRECTORATE GENERAL OF HEALTHCARE PROVISION
			2	Percentage of working units receive budget allocation according to the priorities criteria	(Numbers of Working Units (Satker) that receive budget allocation according to priorities criteria) / (Numbers of Working Units (Satker) that receive budget allocation within a current year) x 100 %	100%	100%	100%	100%	100%	100%	
			3	Percentage of vertically technical implementing units have Renstra-based performance management system	(Number of vertically technical implementing unit that have Renstra-based performance management system) / (numbers of all UPT) x 100%	-	30%	40%	50%	60%	70%	
			4	Percentage of organized vertically technical implementing units based on good performance index that are in line with the performance contract.	(Numbers of Vertically UPT with AA score) / (total numbers of Vertically UPT (49 UPT)) x 100 %	-	60%	70%	80%	90%	100%	
			5	Percentages of directorate programs refer to national target areas.	(Directorate programs or activities that refer to national target area) / (total numbers of programs or activities in the directorate) x 100%	-	50%	60%	70%	80%	90%	

NO	PROGRAM/ACTIVITY	OBJECTIVE	INDICATOR	CALCULATION METHOD	BASELINE (2014)	TARGET					EXECUTING ORGANIZATION UNIT	
						2015	2016	2017	2018	2019		
(1)	(2)	(3)	(4)	(5)	(6)	(7)	(8)	(9)	(10)	(11)	(12)	
VII	PHARMACEUTICAL AND MEDICAL DEVICES PROGRAM	The improved access and quality of pharmacy, and medical devices stocks as well as the household medical supplies	1	Percentages of drugs and vaccines availabilities at Puskesmas level	a. In Districts/cities: (Total cumulative number of medicinal items available in (n) Puskesmas) x (100 divided by (n x total numbers of indicators medicinal items) b. In Province (Total cumulative numbers of medicinal items available in (n) Puskesmas in (y) Districts/cities) x 100 divided by (n x y) x (total numbers of indicators medicinal items)	75.5%	77%	80%	83%	86%	90%	DIRECTORATE GENERAL OF PHARMACEUTICAL CARE AND MEDICAL DEVICES
			2	Numbers of types of medicinal raw materials, traditional medicines and medical devices that are manufactured in the country (cumulative)	Increment in medicinal raw materials (BBO) ready to be manufactured, and/or made in Indonesia; and types of medical tools manufactured domestically, every year cumulatively	-	7	14	21	28	35	

NO	PROGRAM/ACTIVITY	OBJECTIVE	INDICATOR	CALCULATION METHOD	BASELINE (2014)	TARGET					EXECUTING ORGANIZATION UNIT
						2015	2016	2017	2018	2019	
(1)	(2)	(3)	(4)	(5)	(6)	(7)	(8)	(9)	(10)	(11)	(12)
			3 Percentage of medical devices products and household medical supplies in the market are qualified	(Number of Medical devices and household medical supplies tested and qualified) x 100% divided by total numbers of medical devices and household medical supplies samples tested	-	75%	77%	79%	81%	83%	DIRECTORATE GENERAL OF PHARMACEUTICAL CARE
1	Improvement of Pharmaceutical Care	The improvement of pharmacy service and the rational drug use at healthcare facilities	1 Percentage of Puskesmas delivers pharmaceutical care according to the standard	(Numbers of Puskesmas that provide pharmaceutical care) divided by (total number of Puskesmas) x 100%	30%	40%	45%	50%	55%	60%	DIRECTORATE OF PHARMACEUTICAL CARE

NO	PROGRAM/ACTIVITY	OBJECTIVE	INDICATOR	CALCULATION METHOD	BASELINE (2014)	TARGET					EXECUTING ORGANIZATION UNIT
						2015	2016	2017	2018	2019	
(1)	(2)	(3)	(4)	(5)	(6)	(7)	(8)	(9)	(10)	(11)	(12)
			2	Percentage of rational drug use at Puskesmas $= \frac{\left(\frac{100 - P_{\text{antibiotic}}}{100} \right) + \left(\frac{100 - P_{\text{antibiotic}}}{100} \right) + \left(\frac{100 - P_{\text{antibiotic}}}{100} \right) + \left(\frac{100 - P_{\text{antibiotic}}}{100} \right)}{4}$ <i>Total percentage of each indicators prescription attainment</i> <i>Total Component of indicators' prescription</i> A sigma of : 1)Percentage of antibiotics use in non-Pneumonia ISPA case; 2)Percentage of antibiotics use in non-specific Diarrhea case; 3)Percentage of injection use in Myalgia case; 4) Average drug item type per prescription in 3 cases.	60%	62%	64%	66%	68%	70%	DIRECTORATE OF PHARMACEUTICAL CARE
2	Increasing Availability of Public Medicines and Healthcare Supplies	The provision of quality, well-distributed and affordable drugs, vaccines and medical supplies at state healthcare facilities	1	Percentage of drugs and vaccines availability at Puskesmas a. In Districts/cities: Total cumulative numbers of drug items available in (n) Puskesmas divided by (n x total number of indicator drug item) x 100%; b. In province: (Total cumulative number of drug items available in (n) Puskesmas in (y) Districts/cities divided by (n x y x total number of indicator drug item) x 100%	75.5%	77%	80%	83%	86%	90%	DIRECTORATE OF PUBLIC MEDICINE AND MEDICAL SUPPLIES
			2	Percentages of Pharmaceutical Installations in Districts/cities that perform medicine and vaccine management according to standards (Numbers of Pharmaceutical Installations in Districts/cities that perform medicine management according to standards) divided by (numbers of Pharmaceutical Installations in Districts/cities all over Indonesia) x 100%	53.50%	55%	60%	65%	70%	75%	

NO	PROGRAM/ACTIVITY	OBJECTIVE	INDICATOR	CALCULATION METHOD	BASELINE (2014)	TARGET					EXECUTING ORGANIZATION UNIT	
						2015	2016	2017	2018	2019		
(1)	(2)	(3)	(4)	(5)	(6)	(7)	(8)	(9)	(10)	(11)	(12)	
3	Medical Devices Production and Distribution Development	The improvement in control of pre and post marketing of medical devices and household health supplies (PKRT)	1	Percentage of qualified medical devices products and household health supplies in the market	(Numbers of qualified PKRT and medical devices samples tested) divided by (numbers of PKRT and medical devices samples tested) x 100%	-	75%	77%	79%	81%	83%	DIRECTORATE OF MEDICAL DEVICES PRODUCTION AND DISTRIBUTION
			2	Number of medical devices are manufactured in the country (cumulative)	Addition of medical devices types manufactured every year, accumulatively	-	2	4	6	8	10	
			3	Percentage of medical devices and household medical supplies meet the good manufacturing standard (GMP/CPAKB).	Percentage of medical devices and household medical supplies that meet the good manufacturing standard compared to number of production equipment that already have production certificates	30%	35%	40%	45%	50%	55%	
			4	Percentage of pre-market assessments are in line with the good review practices	(Numbers of processed applications that meet service promises) divided by (Numbers of submitted applications) x 100%	60%	63%	66%	69%	72%	75%	

NO	PROGRAM/ACTIVITY	OBJECTIVE	INDICATOR		CALCULATION METHOD	BASELINE (2014)	TARGET					EXECUTING ORGANIZATION UNIT
							2015	2016	2017	2018	2019	
(1)	(2)	(3)	(4)		(5)	(6)	(7)	(8)	(9)	(10)	(11)	(12)
4	Improvement on Pharmacy Production and Distribution of Medical Devices	The increasing production of raw materials and local medicines and quality improvement of pharmaceutical production and distribution facilities	1	Numbers of medicinal raw materials and traditional medicines are manufactured in the country. (cumulative)	Addition of medicinal raw materials and traditional medicinal raw materials (BBO/BBOT) types ready to be manufactured, and/or made in Indonesia, yearly basis, accumulatively	-	5	10	15	20	25	DIRECTORATE OF PHARMACEUTICAL PRODUCTION AND DISTRIBUTION
			2	Numbers of industries utilize medicinal raw materials and traditional medicines that are manufactured in the country. (cumulative)	Additional numbers of industries that use BBO/BBOT manufactured domestically, yearly basis, accumulatively	-	2	4	6	8	10	
5	Management Support and Implementation of Other Technical Tasks in Pharmaceutical and Medical Devices Programs	The increasing management support and implementation of other technical tasks in pharmaceutical and medical devices program		Percentage of client satisfaction on the management support	Percentage of satisfaction: (Number of items that satisfied the clients) / (numbers of services rendered) x 100%	75%	80%	85%	87%	89%	95%	SECRETARIAT DIRECTORATE GENERAL OF PHARMACEUTICAL AND MEDICAL DEVICES

NO	PROGRAM/ACTIVITY	OBJECTIVE	INDICATOR	CALCULATION METHOD	BASELINE (2014)	TARGET					EXECUTING ORGANIZATION UNIT	
						2015	2016	2017	2018	2019		
(1)	(2)	(3)	(4)	(5)	(6)	(7)	(8)	(9)	(10)	(11)	(12)	
VII	Human Resources for Health Development and Empowerment Program	The improvement on availability and quality of human resource for health according to the health care standard	1	Numbers of Puskesmas that has at minimum 5 types of human resource of health each	Puskesmas' absolute score of having standardized HRHs such as sanitarians, pharmacist, nutritionist, public health officers, and lab analyst	1,015	1,200	2,000	3,000	4,200	5,600	HUMAN RESOURCE FOR HEALTH DEVELOPMENT AND EMPOWERMENT INSTITUTE
			2	Percentage of class-C district/city hospitals that have had four basic specialist doctor and three ancillary specialist doctor	Numbers of class-C distric/city hospitals with four basic specialist doctors (Ob-Gyn, Pediatric, Internist, and Surgeon) and Three ancillary specialists divided by total numbers of Class-C district/city Hospitals	25%	30%	35%	40%	50%	60%	
			3	Numbers of HRHs with improved competency (cumulative)	Number of public officials, teachers and teacher' education and private HRHs and communities with improved competence through certification from accredited educations and trainings	25,000 (cumulative)	10,200	21,510	33,060	44,850	56,910	

NO	PROGRAM/ACTIVITY	OBJECTIVE	INDICATOR	CALCULATION METHOD	BASELINE (2014)	TARGET					EXECUTING ORGANIZATION UNIT
						2015	2016	2017	2018	2019	
(1)	(2)	(3)	(4)	(5)	(6)	(7)	(8)	(9)	(10)	(11)	(12)
1	Standardization, Certification, and Continuous Education of Human Resource for Health	The organization of standardization, certification, and continuous education of human resource for health	Numbers of registered human resources of health	Numbers of Letter of registration (STR) per year	164,000 (cumulative)	100,000	115,000	175,000	150,000	150,000	CENTER FOR STANDARDIZATION, CERTIFICATION, CONTINUOUS EDUCATION OF HUMAN RESOURCE FOR HEALTH
2	Higher Education and Improvement of Human Resource for Health Quality	The increasing organization of advanced education and quality improvement of human resource for health	Numbers of new participants of education assistance aid and scholarship	Numbers of education assistance aid and scholarship recipients (Diploma/Higher Graduates and PPDS/PPDGS per year)	9,500 (cumulative)	1,000	1,000	1,000	1,000	1,000	
3	Public Officials' Education and Training	The improvement of public officials for health education and training	Number of public officials for health obtaining certificate from accredited trainings (cumulative)	Based on number of certificates issued for training participants who have undergone accredited training	5,000 (25,000 baseline based on trainings delivered from 2010-2014/5,000 per year)	9,000	18,000	27,000	36,000	45,000	CENTER FOR PUBLIC OFFICIALS EDUCATION AND TRAINING
4	Human Resource for Health Education and Training	The increasing number of education and training for Human Resource for Health	Number of educators, human resources for health and members of community with improved competence through training	Number of certificates issued for training participants who have undergone accredited training	3,747	1,200	2,310	2,550	2,790	3,060	CENTER FOR HUMAN RESOURCE FOR HEALTH EDUCATION AND TRAINING

NO	PROGRAM/ACTIVITY	OBJECTIVE	INDICATOR	CALCULATION METHOD	BASELINE (2014)	TARGET					EXECUTING ORGANIZATION UNIT
						2015	2016	2017	2018	2019	
(1)	(2)	(3)	(4)	(5)	(6)	(7)	(8)	(9)	(10)	(11)	(12)
5	The Management of Higher Education Quality	The quality improvement of higher education	Percentage of study programs in Health of Polytechnic (Poltekes) of the Ministry of Health that have been accredited.	Number of study program/Health Polytechnics with good accreditation compared to number of expired study programs and new study programs times one hundred percent	-	50%	60%	70%	75%	80%	CENTER FOR HUMAN RESOURCE FOR HEALTH EDUCATION AND TRAINING
6	Planning and Efficient use of Human resource for health	Having a better planning and efficient use of human resource for health	Numbers of human resource for health are absorbed to work in health services facilities	Absolute score of health workers assigned in Puskesmas or government hospitals through mechanisms of PNS, PPPK, and special assignments (team based & individual)	-	950	20,600	21,700	22,800	24,000	CENTER FOR HUMAN RESOURCE FOR HEALTH EDUCATION AND TRAINING
7	Human resource for health Planning	The increasing implementation of health professional planning	Numbers of planning documents of human resource for health	Absolute score of Human Resource for Health policy and planning documents	2	2	3	3	3	4	
8	Internship for Human Resources of Health	Established internship opportunity for human resources of health	Number of HRHs who undergo internship	Absolute score of HRHs who undergo internship	4,677	6,500	6,500	6,500	6,500	6,500	

NO	PROGRAM/ACTIVITY	OBJECTIVE	INDICATOR	CALCULATION METHOD	BASELINE (2014)	TARGET					EXECUTING ORGANIZATION UNIT
						2015	2016	2017	2018	2019	
(1)	(2)	(3)	(4)	(5)	(6)	(7)	(8)	(9)	(10)	(11)	(12)
9	Management Support and Implementation of Other Technical Tasks in Health Manpower Development and Empowerment Program	Improvement in management support and implementation of other technical tasks in Health Manpower development and empowerment program	1	Number of documents, norms, standards, procedures, and criteria of Health Manpower Development Program (PPSDM Kesehatan)	Calculation of number of regulation draft documents Designed/made/created by centers and secretariats in the environment of Health PPSDM Institutions In the form of drafts of bills, Government Regulations (PP), Presidential Regulations (Perpres), Health Minister Regulations (Permenkes), Health Minister Resolutions (Kepmenkes), technical guidance, Technical instructions, partnership agreements, and standard procedure related to Health PPSDM.	166 (cumulative)	20	20	20	20	SECRETARIAT OF HEALTH MANPOWER DEVELOPMENT AND EMPOWERMENT INSTITUTE
			2	Number of documents, data and information of Health Manpower Development Program (PPSDM Kesehatan)	Creation of documents based on data processing And information from Central and Provincial Health PPSDM	-	-	34	34	34	

NO	PROGRAM/ACTIVITY	OBJECTIVE	INDICATOR	CALCULATION METHOD	BASELINE (2014)	TARGET					EXECUTING ORGANIZATION UNIT
						2015	2016	2017	2018	2019	
(1)	(2)	(3)	(4)	(5)	(6)	(7)	(8)	(9)	(10)	(11)	(12)
10	High Education Development and Management	Improvement in high education development and management	1	Number of health workers graduated from Health Polytechnics of the Republic of Indonesia (Poltekkes Kemenkes RI)	15,000	20,000	20,000	20,000	20,000	20,000	SECRETARIAT OF HEALTH MANPOWER DEVELOPMENT AND EMPOWERMENT INSTITUTE
			2	Number of working units with improved equipment and infrastructure (cumulative)	49 (cumulative)	6	14	21	31	38	

NO	PROGRAM/ACTIVITY	OBJECTIVE	INDICATOR	CALCULATION METHOD	BASELINE (2014)	TARGET					EXECUTING ORGANIZATION UNIT	
						2015	2016	2017	2018	2019		
(1)	(2)	(3)	(4)	(5)	(6)	(7)	(8)	(9)	(10)	(11)	(12)	
IX	HEALTH RESEARCH AND DEVELOPMENT	The quality improvement of research and development in health sector	1	Numbers of documents containing research results are registered under HKI	Calculation of cumulative number of health research and development results registered by HKI proven with a registration number	10 (cumulative)	13	21	26	31	35	NATIONAL HEALTH RESEARCH AND DEVELOPMENT INSTITUTE
			2	Number of policy recommendations that are based on health research and development which are being advocated to health programme manager and or to stakeholders	Calculation of cumulative number of policy recommendations (policy brief/policy paper) written based on health research and development results presented in a forum or meeting to program managers and/or stakeholders proven with a policy paper and forum/meeting report (Calculation of target/baseline based on recommendation calculation according to strategic issues advocated)	-	24	48	72	96	120	
			3	Numbers of report documents on national health research (Riskesnas) in health and public nutrition	Calculation of cumulative number of Riskesnas reports written based on health research and development results, proven with a National Riskesnas Report	4 (cumulative)	1	2	3	4	5	

NO	PROGRAM/ACTIVITY	OBJECTIVE	INDICATOR	CALCULATION METHOD	BASELINE (2014)	TARGET					EXECUTING ORGANIZATION UNIT	
						2015	2016	2017	2018	2019		
(1)	(2)	(3)	(4)	(5)	(6)	(7)	(8)	(9)	(10)	(11)	(12)	
1	Research and Development in Biomedical and Basic Medical Technology	The increasing number of researches and developments in biomedical and basic medical technology	1	Number of policies recommendations which are derived from researches and development in biomedical and basic medical technology	Calculation of cumulative number of policy recommendations (policy brief/policy paper) written based on health research and development in the form of proven output of policy brief/policy paper draft (Calculation of target/baseline based on based on recommendation calculation according to strategic issues in Biomedical and Basic Health Technology)	-	5	10	15	20	25	CENTER FOR BIOMEDIC AND BASIC HEALTH TECHNOLOGY
			2	Numbers of publications of scientific papers on biomedical and basic medical technology are published in national and international printed and or electronic media	Calculation of cumulative number of health research and development articles in biomedical and basic health technology published in national and international print media and/or electronic media which is accredited and written by researchers of National Health Research and Development Institution (Badan Litbangkes) as the first author	95 (cumulative)	20	40	60	80	100	

NO	PROGRAM/ACTIVITY	OBJECTIVE	INDICATOR		CALCULATION METHOD	BASELINE (2014)	TARGET					EXECUTING ORGANIZATION UNIT
							2015	2016	2017	2018	2019	
(1)	(2)	(3)	(4)		(5)	(6)	(7)	(8)	(9)	(10)	(11)	(12)
2	Research and Development in People's Health Intervention Technology	The increasing number of researches and developments on public health intervention technology	1	Numbers of policies recommendations which are derived from researches and development in public health intervention technology	Calculation of cumulative number of policy recommendations (policy brief/policy paper) written based on health research and development in the form of proven output of policy brief/policy paper draft (Calculation of target/baseline based on based on recommendation calculation according to strategic issues in People's Health Intervention Technology)	-	8	16	24	32	40	CENTER OF PUBLIC HEALTH INTERVENTION TECHNOLOGY
			2	Numbers of publications of scientific papers on public health intervention technology are published in national and international printed and or electronic media	Calculation of cumulative number of health research and development articles in People's Health Intervention Technology published in national and international print media and/or electronic media which is accredited and written by researchers of National Health Research and Development Institute (Badan Litbangkes) as the first author	73 (cumulative)	33	75	119	169	219	

NO	PROGRAM/ACTIVITY	OBJECTIVE	INDICATOR		CALCULATION METHOD	BASELINE (2014)	TARGET					EXECUTING ORGANIZATION UNIT
							2015	2016	2017	2018	2019	
(1)	(2)	(3)	(4)		(5)	(6)	(7)	(8)	(9)	(10)	(11)	(12)
3	Research and Development on Medical Applied Technology and Clinical Epidemiology	The increasing number of researches and developments in medical applied technology and clinical epidemiology	1	Numbers of policies recommendations which are derived from researches and development in medical applied technology and clinical epidemiology	Calculation of cumulative number of policy recommendations (policy brief/policy paper) written based on health research and development in the form of proven output of policy brief/policy paper draft (Calculation of target/baseline based on based on recommendation calculation according to strategic issues in Applied Health Technology and Clinical Epidemiology)	-	8	16	24	32	40	CENTER FOR APPLIED HEALTH TECHNOLOGY AND CLINICAL EPIDEMIOLOGY
			2	Numbers of publications of scientific papers on medical applied technology and clinic epidemiology are published in national and international printed and or electronic media.	Calculation of cumulative number of health research and development articles in Applied Health Technology and Clinical Epidemiology published in national and international print media and/or electronic media which is accredited and written by researchers of National Health Research and Development Institute (Badan Litbangkes) as the first author	69 (cumulative)	15	32	51	72	93	

NO	PROGRAM/ACTIVITY	OBJECTIVE	INDICATOR		CALCULATION METHOD	BASELINE (2014)	TARGET					EXECUTING ORGANIZATION UNIT
							2015	2016	2017	2018	2019	
(1)	(2)	(3)	(4)		(5)	(6)	(7)	(8)	(9)	(10)	(11)	(12)
4	Research and Development on Humanity, Health Policies and Community Empowerment	The increasing number of researches and developments on humanity, health policies and community empowerment	1	Numbers of policies recommendations which are derived from researches and development in humanity, health policies and community empowerment	Calculation of cumulative number of policy recommendations (policy brief/policy paper) written based on health research and development in the form of proven output of policy brief/policy paper draft (Calculation of target/baseline based on based on recommendation calculation according to strategic issues in Ethics, Health Policies, and People’s Empowerment)	-	9	18	27	36	45	CERENTER FOR ETHICS, HEALTH POLICIES, AND PEOPLE’S EMPOWERMENT
			2	Numbers of publications of scientific papers on humanity, health policies and community empowerment are published in national and international printed and or electronic media.	Calculation of cumulative number of health research and development articles in Ethics, Health Policies, and People’s Empowerment published in national and international print media and/or electronic media which is accredited and written by researchers of National Health Research and Development Institute (Badan Litbangkes) as the first author	95 (cumulative)	25	50	75	100	125	

NO	PROGRAM/ACTIVITY	OBJECTIVE	INDICATOR		CALCULATION METHOD	BASELINE (2014)	TARGET					EXECUTING ORGANIZATION UNIT
							2015	2016	2017	2018	2019	
(1)	(2)	(3)	(4)		(5)	(6)	(7)	(8)	(9)	(10)	(11)	(12)
5	Research and Development on Medicinal Plants and Traditional Medicines	The increasing number of research and development on medicinal plants and traditional medicines	1	Numbers of policies recommendations which are derived from researches and development on medicinal plants and traditional medicines	Calculation of cumulative number of policy recommendations (policy brief/policy paper) written based on health research and development in the form of proven output of policy brief/policy paper draft (Calculation of target/baseline based on based on recommendation calculation according to strategic issues in Medicine Plants and Traditional Medicine)	-	2	4	6	8	10	GRAND HOUSE OF RESEARCH AND DEVELOPMENT IN MEDICINE PLANTS AND TRADITIONAL MEDICINE
			2	Numbers of publications of scientific papers on medicinal plants and traditional medicines are published in national and international printed and or electronic media	Calculation of cumulative number of health research and development articles in Medicine Plants and Traditional Medicine published in national and international print media and/or electronic media which is accredited and written by researchers of National Health Research and Development Institute (Badan Litbangkes) as the first author	70 (cumulative)	24	48	72	96	120	

NO	PROGRAM/ACTIVITY	OBJECTIVE	INDICATOR		CALCULATION METHOD	BASELINE (2014)	TARGET					EXECUTING ORGANIZATION UNIT
							2015	2016	2017	2018	2019	
(1)	(2)	(3)	(4)		(5)	(6)	(7)	(8)	(9)	(10)	(11)	(12)
6	Research and Development on Vector and Reservoir of Diseases	The increasing number of research and development on vector and reservoir of diseases	1	Numbers of policies recommendations which are derived from researches and development on vector and reservoir of diseases.	Calculation of cumulative number of policy recommendations (policy brief/policy paper) written based on health research and development in the form of proven output of policy brief/policy paper draft (Calculation of target/baseline based on based on recommendation calculation according to strategic issues in Disease Vector and Reservoir)	-	2	4	6	8	10	GRAND HOUSE OF RESEARCH AND DEVELOPMENT IN DISEASE VECTOR AND RESERVOIR
			2	Numbers of publications of scientific papers on vector and reservoir of diseases are published in national and international printed and or electronic media	Calculation of cumulative number of health research and development articles in Disease Vector and Reservoir published in national and international print media and/or electronic media which is accredited and written by researchers of National Health Research and Development Institute (Badan Litbangkes) as the first author	20 (cumulative)	10	25	45	65	85	

NO	PROGRAM/ACTIVITY	OBJECTIVE		INDICATOR	CALCULATION METHOD	BASELINE (2014)	TARGET					EXECUTING ORGANIZATION UNIT
							2015	2016	2017	2018	2019	
(1)	(2)	(3)		(4)	(5)	(6)	(7)	(8)	(9)	(10)	(11)	(12)
7	Management Support and Implementation of other technical tasks in Research and Development Program in Health Sector	The improvement of management support and implementation of other technical tasks towards research and development program	1	Numbers of concerning management support towards research and development in health sector	Calculation of cumulative number of reports on implementation in the areas of Planning and Budgeting; Policy Recommendations (Policy Brief/Policy Paper); Publication and Dissemination; Finance and General Affairs; Organization and Employee Affairs Law; Scientific and Ethics Management	30 (Cumulative)	5	10	15	20	25	SECRETARIAT OF NATIONAL HEALTH RESEARCH AND DEVELOPMENT INSTITUTE

HEALTH MINISTER
REPUBLIC OF INDONESIA,

[SIGNED]

NILA FARID MOELOEK

APPENDIX II

MINISTER OF HEALTH DECREE
NUMBER HK.02.02/MENKES/52/2015
ON
THE MINISTRY OF HEALTH STRATEGIC PLAN
YEARS 2015-2019

FUND ALLOCATION MATRIX OF MINISTRY OF HEALTH STRATEGIC PLAN 2015-2019

APPENDIX II
MINISTER OF HEALTH DECREE
NUMBER HK.02.02/MENKES/52/2015
ON
THE MINISTRY OF HEALTH STRATEGIC PLAN
YEARS 2015-2019

MINISTRY OF HEALTH STRATEGIC PLAN 2015-2019 BUDGET ALLOCATION MATRIX

NO	PROGRAM/ ACTIVITY	OBJECTIVE	INDICATOR	CALCULATION METHOD	ALLOCATION (Rp. Billion)					TOTAL ALLOCATION 2015-2019	EXECUTING ORGANIZATION UNIT
					2015	2016	2017	2018	2019		
(1)	(2)	(3)	(4)	(5)	(6)	(7)	(8)	(9)	(10)	(11)	(12)
I	SUPPORTING PROGRAM FOR MANAGEMENT AND IMPLEMENTATIO N OF OTHER TECHNICAL TASKS OF MOH	improved coordination of tasks implementation, development and delivery of management support of the MOH	1 The number of formulation health-oriented public policies		24,288.9	32,179.0	36,786.3	37,710.8	41,441.1	172,220.7	SECRETARIAT GENERAL
			2 The percentage of the alignment between management support and implementation of other technical tasks	Number of Center/Bureau performance achievement divided by total number of Center/Bureau							
1	Planning and Budgeting of Health Development Program	The increasing quality of planning and budgeting of health development program	1 The number of provinces that have five years terms plan and an integrated health budget from various sources	Provinces with a five-year plan and health budget integrated from various sources	13.1	11.7	15.1	8.4	6.7	55.0	PLANNING AND BUDGETING BUREAU

NO	PROGRAM/ ACTIVITY	OBJECTIVE	INDICATOR	CALCULATION METHOD	ALLOCATION (Rp. Billion)					TOTAL ALLOCATION 2015-2019	EXECUTING ORGANIZATION UNIT	
					2015	2016	2017	2018	2019			
(1)	(2)	(3)	(4)	(5)	(6)	(7)	(8)	(9)	(10)	(11)	(12)	
			2	The number of quality document of planning policies, budget and evaluation of health development	Documents are grouped according to duties and functions of working units	93.7	103.1	113.4	124.7	137.2	434.9	PLANNING AND BUDGETING BUREAU
			3	The number of recommendations of integrated monitoring-evaluation	Integrated monitoring and evaluation recommendations made	13.1	14.4	15.9	17.5	19.2	80.1	
2	Personnel Administration Development	The improving service delivery of personnel administration	1	The percentage of the fulfillment for public officials for health	Realization of Civil Government Employee Candidates (CPNS) and PTT/P3K appointment to number of CPNS and PTT/P3K formations per year ratio	74.7	78.4	82.2	85.9	85.9	407.1	BUREAU OF PERSONNEL
			2	The percentage of structural officials in the scope of Ministry of Health whose competency are relevant to the job requirement	Number of structural officials who meet position competence standards to total number of structural officials ratio	3.9	3.9	4.0	4.2	4.2	20.2	

NO	PROGRAM/ ACTIVITY	OBJECTIVE	INDICATOR	CALCULATION METHOD	ALLOCATION (Rp. Billion)					TOTAL ALLOCATION 2015-2019	EXECUTING ORGANIZATION UNIT
					2015	2016	2017	2018	2019		
(1)	(2)	(3)	(4)	(5)	(6)	(7)	(8)	(9)	(10)	(11)	(12)
			3 The percentages of Ministry of Health personnel whose performance values are at minimum satisfactory level	The number of CPNS and PNS with an SKP score of, at minimum satisfactory level to the total number of CPNS and PNS	21.5	22.7	23.8	24.9	24.9	117.7	BUREAU OF PERSONNEL
3	Development of Financial Administration and State Assets Management	The increasing quality of Financial and State Assets (BMN) management of the Ministry of Health that is effective and efficient and reported according to the rules	1 The percentage of work units that submit a timely and quality financial statement according to the Government Accounting System (SAP) in order to maintain the Unqualified Opinion (WTP)	Number of Working Units (Satker) in Central Office and Local Office that submit financial reports divided by total number of all Working Units (Satker) in Central Office and Local Office	41.0	43.1	45.2	47.4	49.8	226.5	BUREAU OF FINANCE AND STATE ASSETS
			2 The percentages of fixed asset values that have been granted by the Stipulation of Utilization Status (PSP)	Value of fixed asset that has received Usage Status Determination (Penetapan Status Penggunaan/PSP) divided by value of fixed asset in audited financial report							

NO	PROGRAM/ ACTIVITY	OBJECTIVE	INDICATOR	CALCULATION METHOD	ALLOCATION (Rp. Billion)					TOTAL ALLOCATION 2015-2019	EXECUTING ORGANIZATION UNIT
					2015 (6)	2016 (7)	2017 (8)	2018 (9)	2019 (10)		
(1)	(2)	(3)	(4)	(5)	(6)	(7)	(8)	(9)	(10)	(11)	(12)
			3 The percentage of procurement that uses e-procurement	Number of Working Units in Central Office and Local Office that use LPSE divided by total number of all Working Units (Satker) in Central Office and Local Office							BUREAU OF FINANCE AND STATE ASSETS
4	The Formulation of Laws and Regulation and Organization	The increasing number of laws and regulation, delivery of legal services, organization and procedures	1 The number of legal products in health sector:								BUREAU OF LAW ORGANIZATION
			The nuber of drafts bill (RUU), drafts of government regulations (RPP), draft of presidential regulation/decrees (Keppres)/presidential instruction (Inpres) that have been completed	Comparing result trends in previous years with needs estimation for the future years	4.5	4.8	5.0	5.0	5.5	24.8	
			The number of drafts of the Minister of Health Regulations/Decrees	Comparing result trends in previous years with needs estimation for the future years	4.0	4.3	4.5	4.5	4.5	21.8	

NO	PROGRAM/ ACTIVITY	OBJECTIVE	INDICATOR	CALCULATION METHOD	ALLOCATION (Rp. Billion)					TOTAL ALLOCATION 2015-2019	EXECUTING ORGANIZATION UNIT	
					2015	2016	2017	2018	2019			
(1)	(2)	(3)	(4)	(5)	(6)	(7)	(8)	(9)	(10)	(11)	(12)	
			2	Number of case handling related to assets	Comparing result trends in previous years with needs estimation for the future years	4.0	4.0	4.0	4.5	5.0	21.5	BUREAU OF LAW ORGANIZATION
				Number of legal case handling	Comparing result trends in previous years with needs estimation for the future years							
				The number of work agreements in the health sector	Comparing result trends in previous years with needs estimation for the future years							
			3	The number of products of organization and job procedures and analysis	Comparing result trends in previous years with needs estimation for the future years	6.4	6.5	6.8	6.8	7.0	33.4	
			4	The number of procedures, state affairs organization products in health sector, performance accountability and functional position	Comparing result trends in previous years with needs estimation for the future years	3.2	3.5	3.8	3.8	4.0	18.2	

NO	PROGRAM/ACTIVITY	OBJECTIVE	INDICATOR		CALCULATION METHOD	ALLOCATION (Rp. Billion)					TOTAL ALLOCATION 2015-2019	EXECUTING ORGANIZATION UNIT
						2015	2016	2017	2018	2019		
(1)	(2)	(3)	(4)		(5)	(6)	(7)	(8)	(9)	(10)	(11)	(12)
5	The Management of Administration, Protocols, Household Affairs, Financial and Payroll	The increasing quality of correspondence administration, well-organization of events and activities of the leadership	1	The Percentage of correspondence administrations, organization of events and activities for the decision maker are well and smoothly implemented according to the rules	A = Number of correspondence completed divided by total number of all correspondence times one hundred percent B = Number of daily events successfully held divided by total number of all daily events times one hundred percent. Cumulative A plus B divided by two equals target	20	21	23	24	26	114.0	BUREAU OF GENERAL AFFAIRS
		The increasing quality of documentation service for overseas official travel, official paperwork and archive management in the Ministry of Health environment	1	The percentage of documents service for overseas official travel is delivered in timely manner	a.Number of employees' overseas official travel document completed in maximum 10 days after proposition	22	23	24	25	28	122.0	

NO	PROGRAM/ACTIVITY	OBJECTIVE	INDICATOR	CALCULATION METHOD	ALLOCATION (Rp. Billion)					TOTAL ALLOCATION 2015-2019	EXECUTING ORGANIZATION UNIT
					2015	2016	2017	2018	2019		
(1)	(2)	(3)	(4)	(5)	(6)	(7)	(8)	(9)	(10)	(11)	(12)
			2	The percentage of central work unit's archives are well-developed and The percentage of regional technical implementing unit's archives and official paperwork are well-developed	b. Number of Central Working Units' (Satker Pusat) archives successfully managed divided by total number of all Working Units times one hundred percent						BUREAU OF GENERAL AFFAIRS
					c. Number of Local UPT Working Units' (Satker Daerah) archives and official documents successfully managed divided by total number of all Working Units times one hundred						

NO	PROGRAM/ ACTIVITY	OBJECTIVE	INDICATOR		CALCULATION METHOD	ALLOCATION (Rp. Billion)					TOTAL ALLOCATION 2015-2019	EXECUTING ORGANIZATIO N UNIT
						2015	2016	2017	2018	2019		
(1)	(2)	(3)	(4)		(5)	(6)	(7)	(8)	(9)	(10)	(11)	(12)
		An improve management of MOH’s office		The percentage provision of office means and facilities	SP-1 = Number of sqM of office building development, renovation, rehabilitation divided by Number of sqM realization of office building development, renovation, rehabilitation times one hundred percent. SP-2 = Number of office equipment procurement divided by number of office equipment procurement realization times one hundred percent. SP-3 = Number of office equipment and infrastructure maintenance divided by Number of office equipment and infrastructure maintenance realization times one hundred percent. IKK-2 = Total SP divided by three	101	107	112	118	129	567.0	BUREAU OF GENERAL AFFAIRS

NO	PROGRAM/ ACTIVITY	OBJECTIVE	INDICATOR		CALCULATION METHOD	ALLOCATION (Rp. Billion)					TOTAL ALLOCATION 2015-2019	EXECUTING ORGANIZATION UNIT
						2015	2016	2017	2018	2019		
(1)	(2)	(3)	(4)		(5)	(6)	(7)	(8)	(9)	(10)	(11)	(12)
		improving the quality of management of the salaries payment and/or strategic human resources of health in order to support the achievement indicators of health development program 2015-2019		Total percentage of payment and/or incentives for strategic human resources of health are well-targeted.	Number of effective strategic health workers divided by total number of all employees times 100 percent	2,697.4	2,831	2,972	3,120	3,433	15,053.4	BUREAU OF GENERAL AFFAIRS
6	Health Data and Management Information	The improving management of health data and information	1	The percentage of districts/cities that report prioritized health data	Number of Regencies/Municipalities that send report divided by total number of all of Regencies/Municipalities	74.0	77.7	81.6	85.7	90.0	409.0	CENTER FOR HEALTH DATA AND INFORMATION
			2	The percentage of provision of data communication network that is designated for e-health service access	Number of Regencies/Municipalities that have connection divided by total number of all of Regencies/Municipalities							
7	Health Promotion and Community Empowerment	the improving implementation of community empowerment and health promotion	1	The formulation of health-oriented public policies to enhance the life quality of Indonesian people.	Number of public policies with health insight	88.5	93.0	97.6	102.5	107.6	489.2	CENTER OF HEALTH PROMOTION

NO	PROGRAM/ ACTIVITY	OBJECTIVE	INDICATOR		CALCULATION METHOD	ALLOCATION (Rp. Billion)					TOTAL ALLOCATION 2015-2019	EXECUTING ORGANIZATION UNIT
						2015	2016	2017	2018	2019		
(1)	(2)	(3)	(4)		(5)	(6)	(7)	(8)	(9)	(10)	(11)	(12)
			2	The percentage of districts/cities that implement clean and healthy lifestyle policies	(Number of Regencies/Municipalities that make policies that support PHBS at least 1 New policy per year divided by total number of all of Regencies/Municipalities) x 100%	44.3	66.3	72.1	82.9	95.1	360.7	CENTER OF HEALTH PROMOTION
			3	The percentage of villages that utilize the 10% village fund for community based healthcare program	(Number of Villages that utilize minimum 10 percent village funding for Health Intervention by the People (Upaya Kesehatan Bersumberdaya Masyarakat/UKBM) divided by total number of villages) x 100%	44.3	66.3	72.1	82.9	95.1	360.7	
			4	The number of business entities utilize their CSR for health program	Number of businesses that have collaborations (MOU) with Health Ministry in supporting health programs	22.1	23.2	24.4	25.6	26.9	122.2	

NO	PROGRAM/ ACTIVITY	OBJECTIVE	INDICATOR		CALCULATION METHOD	ALLOCATION (Rp. Billion)					TOTAL ALLOCATION 2015-2019	EXECUTING ORGANIZATION UNIT
						2015	2016	2017	2018	2019		
(1)	(2)	(3)	(4)		(5)	(6)	(7)	(8)	(9)	(10)	(11)	(12)
			5	The number of civil society organizations utilize their resources to support health sector	Number of people’s organizations that have collaborations (MOU) with Health Ministry in supporting health programs	22.1	23.2	24.4	25.6	26.9	122.2	CENTER OF HEALTH PROMOTION
8	Health Crisis Management	The improving intervention of health crisis risks reduction	1	The number of districts/cities receive support in order to implement health crisis risk reduction intervention in their areas	Calculating number of Regencies/Municipalities that have received support in health crisis risk reduction intervention in their areas	69.0	66.1	70.7	75.9	81.6	363.4	CENTER FOR HEALTH CRISIS MANAGEMENT
			2	The number of provinces receive advocacy and outreach to support the implementation of health crisis risk reduction intervention in their areas	Calculating number of Provinces that have received support and socialization in health crisis risk reduction intervention in their areas	6.0	4.9	5.3	4.1	4.4	24.6	

NO	PROGRAM/ ACTIVITY	OBJECTIVE	INDICATOR		CALCULATION METHOD	ALLOCATION (Rp. Billion)					TOTAL ALLOCATION 2015-2019	EXECUTING ORGANIZATIO N UNIT
						2015	2016	2017	2018	2019		
(1)	(2)	(3)	(4)		(5)	(6)	(7)	(8)	(9)	(10)	(11)	(12)
9	Public Communication Management	The improving management of public communication	1	The number of publications on health are disseminated to public	Calculating total number of publications distributed to the society by Public Communication Center (Pusat Komunikasi Publik) through print and electronic media, releases, social media (facebook, twitter, YouTube), website, publishing and face-to-face media (socializations/meetin gs)	28.6	29.3	30.0	30.9	31.1	149.9	CENTER OF PUBLIC COMMUNICATION
			2	The percentage of information request and complaint services that are completed	Comparison between number of information requests and complaints from the people handled and resolved divided by total number of information requests and complaints made through SIAP applications (Halo Kemkes, email, pojok info, PPID, LAPOR, incoming letters and SMS)	12.4	13.9	16.1	17.5	18.6	78.4	CENTER OF PUBLIC COMMUNICATION

NO	PROGRAM/ ACTIVITY	OBJECTIVE	INDICATOR		CALCULATION METHOD	ALLOCATION (Rp. Billion)					TOTAL ALLOCATION 2015-2019	EXECUTING ORGANIZATION UNIT
						2015	2016	2017	2018	2019		
(1)	(2)	(3)	(4)		(5)	(6)	(7)	(8)	(9)	(10)	(11)	(12)
10	Health Intelligence Enhancement	The optimum improvement on intellectual health in order to prepare quality manpower.		The number of production of instruments for increasing and countering intellectual health problems according to the lifecycle stages to support the development of public education in preparing quality manpower	According to 7 stages of life cycle, namely: 1. fetus/pregnancy stage; 2. infant stage; 3. toddler stage; 4. child stage; 5. teenage stage; 6. mature stage; 7. elderly stage	20.0	21.0	22.1	23.2	24.3	86.2	CENTER OF HEALTH INTELLIGENCE
11	The Improving Health of the Hajj Pilgrims	The increasing preparedness of effective and required health services in Saudi Arabia		The percentages of health checkup results of the pilgrims (3 months before the operation)	Number of results of Hajj Pilgrims’ health checks entered into SISKOHATKES three months before operations divided by number of Hajj Pilgrims quota in current year times 100%	200.0	270.0	311.0	357.0	411.0	1,548.0	HAIJ HEALTH CENTER

NO	PROGRAM/ ACTIVITY	OBJECTIVE	INDICATOR		CALCULATION METHOD	ALLOCATION (Rp. Billion)					TOTAL ALLOCATION 2015-2019	EXECUTING ORGANIZATION UNIT
						2015	2016	2017	2018	2019		
(1)	(2)	(3)	(4)		(5)	(6)	(7)	(8)	(9)	(10)	(11)	(12)
12	Enhancement International Cooperation	The better role and position of Indonesia in international cooperation in health sector		The number of formulation of cooperation agreements in health sector	Number of international agreement documents signed including agreements in international negotiations of governmental nature and have been implemented by the Health Ministry in order to support the achievement of health development's strategic objectives measured through periodic and comprehensive monitoring reports and evaluations in one year	20.0	23.5	21.0	18.5	21.0	104.0	CENTER FOR INTERNATIONAL COOPERATION
13	The Management of Indonesian Medical Council	The improvement of registration service and standardization of professional education, development and handling of disciplinary violation cases on doctors and dentists	1	The number of cases of disciplinary violation of doctors and dentists are solved	Average number of reports in the last 3 years from 30 to 40 reports and acted upon as violations of Doctors' and Dentists' discipline	2.5	2.6	2.8	2.9	3.0	13.8	INDONESIAN MEDICAL COUNCIL

NO	PROGRAM/ ACTIVITY	OBJECTIVE	INDICATOR		CALCULATION METHOD	ALLOCATION (Rp. Billion)					TOTAL ALLOCATION 2015-2019	EXECUTING ORGANIZATIO N UNIT
						2015	2016	2017	2018	2019		
(1)	(2)	(3)	(4)		(5)	(6)	(7)	(8)	(9)	(10)	(11)	(12)
			2	The number of Letters of Registration (STR) of doctors and dentists are registered and delivered in timely manner	Estimated number of newly graduated Doctors and Dentists, plus repeat registrations, plus overseas-graduate Indonesian citizens (WNI), domestic-graduate foreign citizens (WNA), and domestic-graduate Doctors and Dentists who want to work or study abroad	32.5	46.5	40.6	36.8	38.4	194.9	INDONESIAN MEDICAL COUNCIL
II	ENHANCEMENT PROGRAM FOR THE IMPLEMENTATION OF NATIONAL HEALTH INSURANCE (JKN)/ HEALTHY INDONESIA CARD (KIS) DEVELOPMENT OF HEALTH FINANCING AND NATIONAL HEALTH INSURANCE (JKN)/HEALTHY INDONESIA CARD (KIS)	The enhancement of National Health Insurance (JKN)/Healthy Indonesia Card (KIS) The formulation of technical policies on development of Health Financing and National Health Insurance (JKN)/Healthy Indonesia Card (KIS)	1	The number of people as Contribution Beneficiaries (PBI) by participating in national health insurance (JKN)/Healthy Indonesia Card (KIS) The number of productions of documents study/monitoring and evaluation results on JKN/KIS implementation	Number of PBI participants determined according to laws times amount of funds determined times twelve months	20,479.2	28,069.1	32,440.1	33,114.4	36,396.1	150,476.1	CENTER FOR HEALTH FUNDING AND HEALTH INSURANCE
						20,355.1	27,945.0	32,314.0	32,987.0	36,267.0	149,868.1	

NO	PROGRAM/ ACTIVITY	OBJECTIVE	INDICATOR		CALCULATION METHOD	ALLOCATION (Rp. Billion)					TOTAL ALLOCATION 2015-2019	EXECUTING ORGANIZATION UNIT
						2015	2016	2017	2018	2019		
(1)	(2)	(3)	(4)		(5)	(6)	(7)	(8)	(9)	(10)	(11)	(12)
1	Enhancement Program for the Implementation of National Health Insurance (JKN)/ Healthy Indonesia Card (KIS) Development of Health Financing and National Health Insurance (JKN)/Healthy Indonesia Card (KIS)	The enhancement of National Health Insurance (JKN)/Healthy Indonesia Card (KIS) The formulation of technical policies on development of Health Financing and National Health Insurance (JKN)/Healthy Indonesia Card (KIS)	2	The number of documents of Health Technology Assessment (HTA) results are submitted to the Minister of Health The number of policies documents on the realization of contribution fee of the JKN/KIS participants are produced	Documents are grouped based on study/research/monitoring and evaluation of health funding and JKN	124.1	124.1	126.1	127.4	129.1	608.0	CENTER FOR HEALTH FUNDING AND HEALTH INSURANCE
			1	The number of people as Contribution Beneficiaries (PBI) by participating in national health insurance (JKN)/Healthy Indonesia Card (KIS)	Documents are grouped based on HTA research/analysis result							
			1	The number of productions of documents study/monitoring and evaluation results on JKN/KIS implementation	Documents are grouped according to policy for PBI JKN/KIS funding							

NO	PROGRAM/ACTIVITY	OBJECTIVE	INDICATOR	CALCULATION METHOD	ALLOCATION (Rp. Billion)					TOTAL ALLOCATION 2015-2019	EXECUTING ORGANIZATION UNIT
					2015	2016	2017	2018	2019		
(1)	(2)	(3)	(4)	(5)	(6)	(7)	(8)	(9)	(10)	(11)	(12)
III	PROGRAM FOR ENHANCING THE INTERNAL CONTROL AND ACCOUNTABILITY OF THE MINISTRY OF HEALTH' PUBLIC OFFICIALS	The increasing transparency of good governance and the implementation of bureaucratic reform	The percentage of of working units have <1% of state losses	(Number of Working Units managing Health Ministry State Budget (APBN) with state loss finding ≤ 1% based on audit result) x 100% number of Working Units managing Health Ministry State Budget (APBN) audited	103.0	111.3	116.2	121.3	126.6	578.4	INSPECTORATE GENERAL
1	Reinforce the internal control on program/activities in working unit under Inspectorate I	The increasing transparency of state governance and the implementation of bureaucratic reform at working units under Inspectorate I	The percentage of working units in Inspectorate I have < 1% of state losses	(Number of Working Units managing Health Ministry State Budget (APBN) under Inspectorate I with state loss finding ≤ 1% based on audit result) x 100% total number of Working Units managing Health Ministry State Budget (APBN) under Inspectorate I audited	12.2	13.6	14.8	16.3	18.1	75.0	INSPECTORATE I

NO	PROGRAM/ACTIVITY	OBJECTIVE	INDICATOR	CALCULATION METHOD	ALLOCATION (Rp. Billion)					TOTAL ALLOCATION 2015-2019	EXECUTING ORGANIZATION UNIT
					2015	2016	2017	2018	2019		
(1)	(2)	(3)	(4)	(5)	(6)	(7)	(8)	(9)	(10)	(11)	(12)
2	Reinforce the internal control on program/activities in working unit under Inspectorate II	The increasing transparency of state governance and the implementation of bureaucratic reform at working units under Inspectorate II	The percentage of working units in Inspectorate II have < 1% of state losses	(Number of Working Units managing Health Ministry State Budget (APBN) under Inspectorate II with state loss finding $\leq 1\%$ based on audit result) x 100% total number of Working Units managing Health Ministry State Budget (APBN) under Inspectorate II audited	5.1	5.4	5.7	6.0	6.7	28.9	INSPECTORATE II
3	Reinforce the internal control on program/activities in working unit under Inspectorate III	The increasing transparency of state governance and the implementation of bureaucratic reform at working units under Inspectorate III	The percentage of working units in Inspectorate III have < 1% of state losses	(Number of Working Units managing Health Ministry State Budget (APBN) under Inspectorate III with state loss finding $\leq 1\%$ based on audit result) x 100% total number of Working Units managing Health Ministry State Budget (APBN) under Inspectorate III audited	9.1	10.1	11.3	12.6	14.1	57.1	INSPECTORATE III

N O	PROGRAM/ACTIVITY	OBJECTIVE	INDICATOR	CALCULATION METHOD	ALLOCATION (Rp. Billion)					TOTAL ALLOCATION 2015-2019	EXECUTING ORGANIZATION UNIT
					2015	2016	2017	2018	2019		
(1)	(2)	(3)	(4)	(5)	(6)	(7)	(8)	(9)	(10)	(11)	(12)
4	Reinforce the internal control on program/activities in working unit under Inspectorate IV	The increasing transparency of state governance and the implementation of bureaucratic reform at working units under Inspectorate IV	The percentage of working units in Inspectorate IV have < 1% of state losses	(Number of Working Units managing Health Ministry State Budget (APBN) under Inspectorate IV with state loss finding $\leq 1\%$ based on audit result) x 100% total number of Working Units managing Health Ministry State Budget (APBN) under Inspectorate IV audited	9.2	10.1	11.0	12.0	13.2	55.5	INSPECTORATE IV
5	Improvement of Public Complaint Handling Service at Ministry of Health	Improvement in handling public complaints that indicate any state losses	The percentage of public complaints are handled according to the authority of the Inspectorate General	(Number of handling of people's reports indicating state loss accepted according to the authority of the Inspectorate General) x 100% Number of people's reports indicating state loss accepted according to the authority of the Inspectorate General	9.2	9.7	10.1	10.6	10.6	50.1	INSPECTORATE OF INVESTIGATION

N O	PROGRAM/ACTIVITY	OBJECTIVE	INDICATOR	CALCULATION METHOD	ALLOCATION (Rp. Billion)					TOTAL ALLOCATION 2015-2019	EXECUTING ORGANIZATION UNIT
					2015	2016	2017	2018	2019		
(1)	(2)	(3)	(4)	(5)	(6)	(7)	(8)	(9)	(10)	(11)	(12)
6	Management support and implementation of other technical tasks in the enhancement program of Internal Control and Accountability of the Ministry of Health Public Officials	The increasing support on management and implementation of other technical tasks in the enhancement program of Internal Control and accountability of the ministry of health's public officials	The percentage of working units have implemented the prevention and eradication of corruption program	(Number of Working Units in Central Office and Local Office that have implemented corruption prevention and eradication programs) x 100% Number of Working Units in Central Office and Local Office in Health Ministry environment	58.2	62.5	63.3	63.8	64.0	311.8	SECRETARIATE OF THE INSPECTORATE GENERAL
IV	MATERNAL AND CHILD HEALTH AND NUTRITION PROGRAM	Increased availability and reach of high-quality health services for the whole society.	1 The percentage of deliveries in healthcare facilities	The number of women giving birth in Puskesmas operational areas and receiving standard assistance by health workers in health facilities within a one year period divided by the number of targeted deliveries in Puskesmas operational areas within the same one year period) x 100%	2,682.6	9,263.7	10,800.3	12,110.2	13,350.9	48,207.7	DIRECTORATE GENERAL OF MATERNAL, CHILD HEALTH AND NUTRITION

N O	PROGRAM/ACTIVITY	OBJECTIVE	INDICATOR	CALCULATION METHOD	ALLOCATION (Rp. Billion)					TOTAL ALLOCATION 2015-2019	EXECUTING ORGANIZATION UNIT
					2015	2016	2017	2018	2019		
(1)	(2)	(3)	(4)	(5)	(6)	(7)	(8)	(9)	(10)	(11)	(12)
			2 The percentage of pregnant women with Chronic Energy Deficiency (CED)	The number of pregnant women with upper arm circumference of < 23,5cm / number of pregnant women with measured upper arm circumference x 100%							DIRECTORATE OF GENERAL OF MATERNAL, CHILD HEALTH AND NUTRITION
1	Nutrition Improvement Program	Improved nutritional services to the community	1 The percentage of pregnant women with CED receiving food supplement (PMT)	(The percentage of pregnant women with CED that receive food supplement (PMT) within a certain area/the total number of pregnant women with CED within the same area) x 100%	500.3	4,500.0	5,100.0	5,600.0	6,100.0	21,800.3	
			2 The percentage of pregnant women receiving Tablet for Blood Supplement (TTD)	The number of pregnant women receiving TTD within a certain area/the entire number of pregnant women within the same area) x 100%							
			3 The percentage of Infants under six months receiving an exclusive breastfeeding	The number of infants under six months receiving exclusive breastfeeding within a certain area / total number of infants under six months within the							

N O	PROGRAM/ACTIVITY	OBJECTIVE	INDICATOR	CALCULATION METHOD	ALLOCATION (Rp. Billion)					TOTAL ALLOCATION 2015-2019	EXECUTING ORGANIZATION UNIT	
					2015	2016	2017	2018	2019			
(1)	(2)	(3)	(4)	(5)	(6)	(7)	(8)	(9)	(10)	(11)	(12)	
			4	The percentage of newborn infants receiving Early Initiation of Breastfeeding (IMD)	The number of newborn infants receiving IMD / total number of babies x 100%						DIRECTORATE OF NUTRITION	
			5	The percentage of underweight children under five receiving food supplement	The number of underweight children under five receiving food supplement / the total number of weighed children under five) x 100%							
			6	The percentage of adolescent girls receiving Tablet for Blood Supplement (TTD)	(The number of adolescent girls receiving TTD in a certain area / total number of adolescent girls in the area) x 100%							
2	Healthcare provision for Infants, Children and Adolescents	Improved access and quality of healthcare provision for infants, children, and adolescents	1	The percentage of first prenatal visit (KN1)	(The number of newborn infants receiving 1 standard service in a Prenatal Visit at the age of six to 48 hours in a certain area within a certain period/the total number of targeted infants in the area within the same period of time) x 100%	161.9	658.4	884.4	982.8	1,010.7	8,698.2	DIRECTORATE OF CHILD HEALTH

N O	PROGRAM/ACTIVIT Y	OBJECTIVE	INDICATOR	CALCULATION METHOD	ALLOCATION (Rp. Billion)					TOTAL ALLOCATION 2015-2019	EXECUTING ORGANIZATION UNIT
					201 5	2016	2017	2018	2019		
(1)	(2)	(3)	(4)	(5)	(6)	(7)	(8)	(9)	(10)	(11)	(12)
			2 The percentage of Puskesmas conducting health screenings for grade-one students	The number of Puskesmas conducting health screening for grade-one elementary school (SD/MI) students in an area within one year/the number of Puskesmas in the area within the same year) x 100%							DIRECTORATE OF CHILD HEALTH
			3 The percentage of Puskesmas conducting health screening for grade seven and grade ten students	(Number of Puskesmas that administer health screening to 7th grade students of Junior High School (SMP/MTs) and 10th grade students of Senior High School (SMA/MA) in an area in 1 year/ number of all Puskesmas in an area in the same 1 year period) x 100 %							DIRECTORATE OF CHILD HEALTH
			4 The percentage of Puskesmas conducting health services for adolescents	The number of Puskesmas qualified to conduct health services for adolescents in their operational areas in a year/the number of Puskesmas in the area in the same year) x 100%							DIRECTORATE OF CHILD HEALTH

N O	PROGRAM/ACTIVIT Y	OBJECTIVE	INDICATOR	CALCULATION METHOD	ALLOCATION (Rp. Billion)					TOTAL ALLOCATION 2015-2019	EXECUTING ORGANIZATION UNIT	
					201 5	2016	2017	2018	2019			
(1)	(2)	(3)	(4)	(5)	(6)	(7)	(8)	(9)	(10)	(11)	(12)	
3	Maternal and Reproductive Healthcare	Improved access and quality of Maternal and Reproductive Healthcare Provision	1	Percentage of Puskesmas conducting maternity classes	(The number of Puskesmas conducting maternal classes / total number of District Puskesmas x 100%	252. 1	500. 0	600. 0	700.0	800. 0	2,852.1	DIRECTORATE OF MATERNAL HEALTH
			2	The percentage of Puskesmas conducting orientations for the Birth Planning and Complication Prevention Program (P4K)	(The number of Puskesmas implementing orientations for the Birth Planning and Complication Prevention Program / total number of Puskesmas) x 100%							
			3	Percentage of pregnant women receiving antenatal services minimum 4 times (K4)	(The number of pregnant women that have received antenatal services minimum 4 times by health workers in an operation area within 1 year)/(number of targeted pregnant women in the area within 1 year) x100%							

NO	PROGRAM/ACTIVITY	OBJECTIVE	INDICATOR		CALCULATION METHOD	ALLOCATION (Rp. Billion)					TOTAL ALLOCATION 2015-2019	EXECUTING ORGANIZATION UNIT
						2015	2016	2017	2018	2019		
(1)	(2)	(3)	(4)		(5)	(6)	(7)	(8)	(9)	(10)	(11)	(12)
4	Development of occupational health and sports	Improved efforts for occupational health and sports	1	Percentage of Puskesmas implementing basic occupational health	(The number of Puskesmas that have applied Work Safety and Security (K3) and conducted promotive and/or preventive and/or curative and/or rehabilitative measures for workers in their area of operation)/(number of Puskesmas in Indonesia) x 100%	87.1	200.0	250.0	300.0	350.0	1,187.1	DIRECTORATE OF OCCUPATIONAL HEALTH AND SPORTS
			2	Number of UKK posts established in PPI/TPI areas	The Numbers of UKK posts established by communities and facilitated by Puskesmas in PPI/TPI areas							
			3	The percentage of qualified facilities for the health examination of Indonesian migrant workers (TKI)	(Numbers of TKI health check facilities that meet the standards) / (number of TKI health check facilities) x 100%							

NO	PROGRAM/ACTIVITY	OBJECTIVE	INDICATOR	CALCULATION METHOD	ALLOCATION (Rp. Billion)					TOTAL ALLOCATION 2015-2019	EXECUTING ORGANIZATION UNIT
					2015	2016	2017	2018	2019		
(1)	(2)	(3)	(4)	(5)	(6)	(7)	(8)	(9)	(10)	(11)	(12)
			4 The percentage of Puskesmas conducting sports health activities in communities within their areas of operation	(The numbers of Puskesmas conducting qualified sports and health activities / The number of Puskesmas in Indonesia x 100%							DIRECTORATE OF OCCUPATIONAL HEALTH AND SPORTS
5	Traditional and Complementary Health Development	Improved efforts in the management, development, and monitoring of traditional and complementary health	The percentage of Puskesmas implementing traditional health	(The percentage of Puskesmas implementing traditional health) / (the total number of Puskesmas) x 100%	62.8	105.3	115.9	127.4	140.2	551.6	DIRECTORATE OF TRADITIONAL AND COMPLEMENTARY HEALTH SERVICE DEVELOPMENT

NO	PROGRAM/ACTIVITY	OBJECTIVE	INDICATOR	CALCULATION METHOD	ALLOCATION (Rp. Billion)					TOTAL ALLOCATION 2015-2019	EXECUTING ORGANIZATION UNIT
					2015	2016	2017	2018	2019		
(1)	(2)	(3)	(4)	(5)	(6)	(7)	(8)	(9)	(10)	(11)	(12)
6	Health Operational Assistance (BOK)	The provision of Health Operational Assistance (BOK) for Puskesmas	1 The number of Puskesmas that receives BOK	Number of Puskesmas that utilizes BOK funds	1,410.5	3,000.0	3,500.0	4,000.0	4,500.0	16,410.5	SECRETARIAT OF DIRECTORATE GENERAL OF MOTHER AND CHILD HEALTH NUTRITION DEVELOPMENT
			2 The number of Puskesmas that publish BOK utilization reports on the Puskesmas message board or at the district office	The number of Puskesmas that publish the utilization of BOK reports on the Puskesmas message board or at the district office							
7	Management Support and Implementation of Other Technical Tasks in Maternal, Child Health and Nutrition Program	Improvement in management support and implementation of other technical tasks in Maternal, Child Health and Nutrition Program	Realization percentage of administration activities in management support and implementation of other technical tasks in Maternal, Child Health and Nutrition Program	(Amount of used budget and amount of activities carried out) / (total budget and total activity output) x 100	207.9	300.0	350.0	400.0	450.0	1,707.9	
V	DISEASE CONTROL AND ENVIRONMENTAL HEALTH PROGRAM	Decreasing numbers in communicable disease, non-communicable disease and improvement of the environment quality	1 Percentage of Districts/cities that meet environmental health quality	(Cumulative number of Districts/cities that meet at least 4 criteria) / (total number of Districts/cities) x 100% in a certain period	2,202.0	3,300.0	3,650.0	3,950.0	4,250.0	16,864.6	DIRECTORATE GENERAL OF DISEASE CONTROL AND ENVIRONMENTAL HEALTH

NO	PROGRAM/ACTIVITY	OBJECTIVE	INDICATOR	CALCULATION METHOD	ALLOCATION (Rp. Billion)					TOTAL ALLOCATION 2015-2019	EXECUTING ORGANIZATION UNIT
					2015	2016	2017	2018	2019		
(1)	(2)	(3)	(4)	(5)	(6)	(7)	(8)	(9)	(10)	(11)	(12)
			2	Percentage of decline in certain cases of Diseases Preventable by Immunization (PD3I) (Numbers of certain PD3I cases in baseline) – (Number of certain PD3I cases in current year) / (Number of certain PD3I cases in baseline in 2013) x 100%							DIRECTORATE GENERAL OF DISEASE CONTROL AND ENVIRONMENTAL HEALTH
				Percentage of Districts/cities that have policy of readiness in overcoming people's health emergencies that potentially have epidemic threats Number of Districts/cities with harbors, airports, and PLBDN that have policy of readiness in overcoming PHEIC divided by Number of Districts/cities with harbors, airports, and PLBDN times 100% Note: Criteria of PLDBN harbors, airports : 1. International 2. Routine work all year long 3. Health quarantine, Immigration, and Customs elements available (Number of Districts/cities with the above criteria in 2014) / 106 Districts/cities							

NO	PROGRAM/ACTIVITY	OBJECTIVE	INDICATOR		CALCULATION METHOD	ALLOCATION (Rp. Billion)					TOTAL ALLOCATION 2015-2019	EXECUTING ORGANIZATION UNIT
						2015	2016	2017	2018	2019		
(1)	(2)	(3)	(4)		(5)	(6)	(7)	(8)	(9)	(10)	(11)	(12)
1	Development of Surveillance, Immunization, Quarantine, and Matra Health	To reduce the number of illness caused by preventable diseases through immunization, development of surveillance, quarantine, and matra health	1	Percentage of children aged zero to 11 months old who has received complete basic immunization	(Numbers of babies who have received one Hepatitis B immunization; one BCG immunization; three DPT,HB and Hib immunizations); four polio immunizations; and one chickenpox immunization in a one year period) / (total number of babies in the same period) x 100%	292.8	600.0	700.0	750.0	800.0	3,142.8	DIRECTORATE OF SURVEILLANCE, IMMUNIZATION, QUARANTINE, AND MATRAT HEALTH
			2	Percentage of early warning signals that have been responded to	(Numbers of early warning signals that have been responded to by Regency/Municipa lity Health Service and its ranks in one month period) / (number of early warning signals received by Regency/Municipa lity Health Service and its ranks in the same period) x 100%							

NO	PROGRAM/ACTIVITY	OBJECTIVE	INDICATOR		CALCULATION METHOD	ALLOCATION (Rp. Billion)					TOTAL ALLOCATION 2015-2019	EXECUTING ORGANIZATION UNIT
						2015	2016	2017	2018	2019		
(1)	(2)	(3)	(4)		(5)	(6)	(7)	(8)	(9)	(10)	(11)	(12)
			3	Percentage of Districts/cities with diving territories that perform matra health intervention	(Number of Districts/cities that perform diving health intervention) / (Number of Districts/cities with diving territories) x 100%							DIRECTORATE OF SURVEILLANCE, IMMUNIZATION, QUARANTINE, AND MATRAT HEALTH
2	Vector Borne Disease Control	Improvement in Prevention and Eradication of Vector Borne Disease	1	Percentage of Districts/cities that perform integrated vector control	(number of District/City implementing vector control divided by number of District/City endemic of vector-borne and other zoonotic diseases) x 100%	240.0	600.0	650.0	700.0	750.0	2,940.0	DIRECTORATE OF VECTOR BORNE DISEASE CONTROL
			2	Numbers of Districts/cities with API <1 per 1.000 population	Cumulative numbers of Districts/cities with API <1 per 1.000 population							
			3	Number of Filariasis endemic Districts/cities that successfully cut microfilaria number to < 1%	Accumulation of Districts/cities that successfully cut microfilaria number to < 1%							

NO	PROGRAM/ACTIVITY	OBJECTIVE	INDICATOR	CALCULATION METHOD	ALLOCATION (Rp. Billion)					TOTAL ALLOCATION 2015-2019	EXECUTING ORGANIZATION UNIT	
					2015	2016	2017	2018	2019			
(1)	(2)	(3)	(4)	(5)	(6)	(7)	(8)	(9)	(10)	(11)	(12)	
			4	Percentage of Districts/cities with IR DBD < 49 per 100.000 population	Numbers of Districts/cities with IR DBD < 49/100.000 population divided by total number of Districts/cities with endemic DBD (Dengue Fever) in the same year						DIRECTORATE OF VECTOR BORNE DISEASE CONTROL	
			5	Percentage of Districts/cities that eliminate Rabies	(Number of Districts/cities with endemic Rabies yang that eliminate Rabies) / (Number of Districts/cities endemic) x 100% in current year							
3	Direct Transferable Disease Control	Decline in numbers of illness and mortality caused by directly communicable diseases	1	Percentage of new discoveries of non-deforming leprosy cases	Number of non-deforming leprosy cases discovered divided by number of new cases discovered	260.0	600.0	650.0	700.0	750.0	2,960.0	DIRECTORATE OF DIRECT TRANSFERABLE DISEASE CONTROL
			2	Percentage of Districts/cities with pulmonary TB BTA treatment success rate of minimum 85%	Districts/cities with pulmonary TB BTA treatment success rate of minimum 85% compared to all Districts/cities x 100%							

NO	PROGRAM/ACTIVITY	OBJECTIVE	INDICATOR	CALCULATION METHOD	ALLOCATION (Rp. Billion)					TOTAL ALLOCATION 2015-2019	EXECUTING ORGANIZATION UNIT
					2015	2016	2017	2018	2019		
(1)	(2)	(3)	(4)	(5)	(6)	(7)	(8)	(9)	(10)	(11)	(12)
			3	Percentage of HIV cases treated (Numbers of ODHA who still receive ARV therapy) / (Numbers of ODHA who meet requirements to start ARV therapy) x 100%.							DIRECTORATE OF DIRECT TRANSFERABLE DISEASE CONTROL
			4	Percentage of Districts/cities whose 50% of their Public Health Centers/Puskesmas perform Pneumonia examination and governance through MTBS programs (Numbers of Districts/cities that discovered and perform governance according to standard of minimum 80%) / (Total numbers of all Districts/cities in Indonesia)							
			5	Percentage of Districts/cities that perform Hepatitis B early detection activities on risk groups ((Numbers of Districts/cities that perform Hepatitis B early detection activities on risk groups / (Total numbers of all Districts/cities in Indonesia)							

NO	PROGRAM/ACTIVITY	OBJECTIVE	INDICATOR		CALCULATION METHOD	ALLOCATION (Rp. Billion)					TOTAL ALLOCATION 2015-2019	EXECUTING ORGANIZATION UNIT
						2015	2016	2017	2018	2019		
(1)	(2)	(3)	(4)		(5)	(6)	(7)	(8)	(9)	(10)	(11)	(12)
4	Non-Communicable Disease Control	Decline in number of illness and mortality caused by non-communicable disease; Improvement in prevention and eradication of non-communicable disease	1	Percentage of Puskesmas that perform integrated non-communicable disease (PTM) control	(Number of Puskesmas that perform integrated non- communicable disease (PTM) control / (Total number of Puskesmas in Indonesia) x 100%	325.0	600.0	650.0	700.0	750.0	3,025.0	DIRECTORATE OF NON-COMMUNICABLE DISEASES CONTROL
			2	Percentage of Districts/cities that implement Non Smoking Area (KTR) policy in 50% schools minimally	(Number of Districts/cities that have regulation and proof of implementation in 50% teaching-learning locations in schools) / (Total number of all Districts/cities in Indonesia) x 100%							
			3	Percentage of Villages/Urban Villages that carry out Non-Communicable Disease Integrated development post (PTM-Posbindu) activities	Villages/Urban Villages that carry out Non-Communicable Disease Integrated development post (PTM-Posbindu) activities / (Total number of all Villages in Indonesia) x 100%							

NO	PROGRAM/ACTIVITY	OBJECTIVE	INDICATOR	CALCULATION METHOD	ALLOCATION (Rp. Billion)					TOTAL ALLOCATION 2015-2019	EXECUTING ORGANIZATION UNIT	
					2015	2016	2017	2018	2019			
(1)	(2)	(3)	(4)	(5)	(6)	(7)	(8)	(9)	(10)	(11)	(12)	
			4	Percentage of women aged between 30 and 50 years subject to early detection of cervix and breast cancer	(Numbers of women aged between 30 and 50 years subject to early detection of cervix and breast cancer) / (Total numbers of women aged between 30 and 50 years old in Indonesia) x 100%						DIRECTORATE OF NON-COMMUNICABLE DISEASES CONTROL	
			5	Percentage of Districts/cities that perform drivers' health check at main terminals	(Numbers of Districts/cities that perform drivers' health check at main terminals) / (Total numbers of all Districts/cities in Indonesia) x 100%							
5	Environmental Health	Improvement in environmental health and quality environmental monitoring	1	Numbers of Villages/Urban Villages that carry out STBM	Calculating total sum of Villages/Urban Villages that have been verified for carrying out STBM	384.2	600.0	650.0	700.0	750.0	3,084.2	DIRECTORATE OF ENVIRONMENT HEALTH
			2	Percentage of drinking water facilities monitored	(Numbers of samples checked on drinking water providers) / (numbers of samples that must be checked) x 100%							

NO	PROGRAM/ACTIVITY	OBJECTIVE	INDICATOR	CALCULATION METHOD	ALLOCATION (Rp. Billion)					TOTAL ALLOCATION 2015-2019	EXECUTING ORGANIZATION UNIT
					2015	2016	2017	2018	2019		
(1)	(2)	(3)	(4)	(5)	(6)	(7)	(8)	(9)	(10)	(11)	(12)
			3	Percentage of Public Sites that meet health requirements (Numbers of TTU that meet health requirements based on Environment Health Inspection result according to standards in 1 (one) year period) / (numbers of TTU registered in Districts/cities in the same 1 year period) x 100%							DIRECTORATE OF ENVIRONMENT HEALTH
			4	Percentage of hospitals that carry out medical waste processing according to standards (Numbers of hospitals that carry out medical waste processing according to regulations) / (numbers of hospitals) x 100%							
			5	Percentage of Food processing facility (TPM) that meet health requirements (Number of TPM that meet sanitation hygiene requirements) / (number of registered TPM) x 100%							
			6	Number of Districts/cities that administer healthy area order Total cumulative number of Districts/cities that administer healthy area order							

NO	PROGRAM/ ACTIVITY	OBJECTIVE	INDICATOR		CALCULATION METHOD	ALLOCATION (Rp. Billion)					TOTAL ALLOCATION 2015-2019	EXECUTING ORGANIZATION UNIT
						2015	2016	2017	2018	2019		
(1)	(2)	(3)	(4)		(5)	(6)	(7)	(8)	(9)	(10)	(11)	(12)
6	Management Support and Implementation of Other Technical Tasks in Disease Control and Environmental Health Programs	Improvement in management support and implementation of other technical tasks in disease control and environmental health programs	1	Percentage of Working Units (Satker) in PP and PL programs that have SAKIP assessment with AA minimum score	(Number of Working Units with AA score) / (number of Working Units with SAKIP assessment)	700.0	300.0	350.0	400.0	450.0	2,200.0	SECRETARIAT OF DIRECTORATE GENERAL OF DISEASES CONTROL AND ENVIRONMENT HEALTH
			2	Percentage of Central and Provincial/Municipal Working Units (Satker) whose equipment/infrastructure have been improved to meet the standards	(Number of Central Working Units and UPT that meet equipment/infrastructure standards) / (Number of Central Working Units and UPT)							

NO	PROGRAM/ ACTIVITY	OBJECTIVE	INDICATOR		CALCULATION METHOD	ALLOCATION (Rp. Billion)					TOTAL ALLOCATION 2015-2019	EXECUTING ORGANIZATION UNIT
						2015	2016	2017	2018	2019		
(1)	(2)	(3)	(4)		(5)	(6)	(7)	(8)	(9)	(10)	(11)	(12)
VI	HEALTHCARE PROVISION DEVELOPMENT PROGRAM	The increasing access to quality primary and referral healthcare provision for community	1	Numbers of sub-districts have at minimum one certified accredited Puskesmas each	Total number of all Sub-districts with minimally one accredited health centers/Puskesmas in current year	15,697.9	22,841.4	24,737.8	39,570.6	53,840.4	154,202.6	DIRECTORATE GENERAL OF HEALTHCARE PROVISION
			2	Districts/cities have minimally one accredited local hospital (RSUD) each	Total cumulative number of accredited Local/Provincial Public Hospital/RSUD in Districts/cities achieved each year							
1	Medical Support Intervention and Health Facilities Development	The increased equality, and quality service of medical support, health facilities and medical devices	1	Percentage of regional hospitals as the provider of telemedicine services.	(Number of regional transfer hospitals that deliver services as the provider of telemedicine) / (total number of all regional hospitals) x 100%	148.2	196.9	206.0	215.1	224.4	1,030.3	DIRECTORATE OF MEDICAL SUPPORT INTERVENTION AND HEALTH FACILITIES
			2	vertically technical implementing units (UPT) and regional referral hospitals have operation cooperation (KSO) arrangement	Numbers of vertical UPTs and regional hospitals that implement operation cooperation arrangement on facilities and equipment							
2	Nursing Care and Medical Technicians' Services Development	The improvement of quality and access to nursing, midwifery care and medical technician services		Numbers of Health Centers that implement Nursing care on public healthcare provision (Perkesmas)		31.2	32.8	34.4	35.9	37.6	171.8	DIRECTORATE OF NURSING CARE AND MEDICAL TECHNICIANS

NO	PROGRAM/ ACTIVITY	OBJECTIVE	INDICATOR		CALCULATION METHOD	ALLOCATION (Rp. Billion)					TOTAL ALLOCATION 2015-2019	EXECUTING ORGANIZATION UNIT
						20 15	2016	2017	2018	2019		
(1)	(2)	(3)	(4)		(5)	(6)	(7)	(8)	(9)	(10)	(11)	(12)
3	Development of Basic Healthcare Provision	Improvement in people's access to quality basic healthcare provision	1	Number of Puskesmas with outpatient and inpatient care that deliver standardized services	Number of Puskesmas with outpatient and inpatient care that deliver standardized services within a current year	757.1	1,100.5	1,034.7	4,702.3	7,811.5	15,406.1	DIRECTORATE OF BASIC HEALTHCARE PROVISION
			2	Numbers of districts/cities that organize mobile health services (PKB) in remote and the most remote areas								
			3	Numbers of puskesmas that have applied puskesmas' management								
			4	Numbers of districts/cities with remote/most remote areas (T/ST) have regulations on stipulation of Puskesmas with T/ST areas.								
			5	Numbers of districts/cities are ready to undergo accreditation on primary health facilities.								
			6	Numbers of Puskesmas that have collaborated with blood transfusion unit (UTD) and hospitals through the health office.	Number of Puskesmas that with blood transfusion unit (UTD) and hospitals through the health office for donor recruitment and selection in order preparing blood supplies for mothers in labor							

NO	PROGRAM/ ACTIVITY	OBJECTIVE	INDICATOR	CALCULATION METHOD	ALLOCATION (Rp. Billion)					TOTAL ALLOCATION 2015-2019	EXECUTING ORGANIZATION UNIT
					2015	2016	2017	2018	2019		
(1)	(2)	(3)	(4)	(5)	(6)	(7)	(8)	(9)	(10)	(11)	(12)
4	Referral Healthcare Intervention Development	Availability of quality referral healthcare facilities that accessible by the people	1	Numbers of national and regional referral hospitals implement the integration of medical records	11,614.1	19,356.3	20,890.7	31,554.9	42,126.4	124,465.4	DIRECTORATE OF REFERRAL HEALTHCARE PROVISION
			2	Number of regional referral hospitals facilities and devices (SPA) meet the specified standard							
			3	Percentages of districts/cities are prepared with referral services access. (Numbers of districts/cities are prepared with referral services access.) / (total numbers of Districts/cities within a current year) x 100 %							
			4	Number of national referral central hospitals facilities are being upscale							
			5	Number of document concerning the needs of hospital boat in archipelagic districts							
			6	Number of regional hospitals meet the standard and specific criteria.							
			7	Number of constructed primary hospitals (cumulative)							

NO	PROGRAM/ ACTIVITY	OBJECTIVE	INDICATOR	CALCULATION METHOD	ALLOCATION (Rp. Billion)					TOTAL ALLOCATION 2015-2019	EXECUTING ORGANIZATION UNIT
					2015	2016	2017	2018	2019		
(1)	(2)	(3)	(4)	(5)	(6)	(7)	(8)	(9)	(10)	(11)	(12)
5	Development of Mental Healthcare	The improvement of quality and access to mental healthcare and narcotics, psychotropic, and addictive substances healthcare	1 Percentages of healthcare facilities at the recipient obligation report institution (IPWL) of active narcotic drug users	(IPWL that report their activities) x 100 % / (numbers of established IPWL within a current year)	24.0	25.6	26.9	28.1	29.4	134.0	DIRECTORATE OF MENTAL HEALTHCARE PROVISION
			2 Numbers of districts/cities Puskesmas administer the mental healthcare								
			3 Percentages of regional referral general hospitals provide mental health/psychiatry care	(Number of regional referral hospitals that administer medical psychiatric care with outpatient and inpatient service by competent medical workers) / (Number of established referral hospitals) x 100 %							

NO	PROGRAM/ ACTIVITY	OBJECTIVE	INDICATOR		CALCULATION METHOD	ALLOCATION (Rp. Billion)					TOTAL ALLOCATION 2015-2019	EXECUTING ORGANIZATION UNIT
						201 5	201 6	2017	2018	2019		
(1)	(2)	(3)	(4)		(5)	(6)	(7)	(8)	(9)	(10)	(11)	(12)
6	Management Support and Implementation of other technical tasks in Healthcare Provision Development Program	The improvement of management support and implementation of other technical tasks in healthcare Provision development program	1	The integrated monitoring and evaluation that runs effectively	(Numbers of integrated evaluation executions that run effectively) / (all integrated evaluation executions) x 100%	3,123.3	2,129.3	2,545.1	3,034.2	3,611.0	12,995.0	SECRETARIAT DIRECTORATE GENERAL OF HEALTHCARE PROVISION
			2	Percentage of working units receive budget allocation according to the priorities criteria	(Numbers of Working Units (Satker) that receive budget allocation according to priorities criteria) / (Numbers of Working Units (Satker) that receive budget allocation within a current year) x 100 %							
			3	Percentage of vertically technical implementing units have Renstra-based performance management system	(Number of vertically technical implementing unit that have Renstra-based performance management system) / (numbers of all UPT) x 100%							
			4	Percentage of organized vertically technical implementing units based on good performance index that are in line with the performance contract.	(Numbers of Vertically UPT with AA score) / (total numbers of Vertically UPT (49 UPT)) x 100 %							
			5	Percentages of directorate programs refer to national target areas.	(Directorate programs or activities that refer to national target area) / (total numbers of programs or activities in the directorate) x 100%							

NO	PROGRAM/ ACTIVITY	OBJECTIVE	INDICATOR		CALCULATION METHOD	ALLOCATION (Rp. Billion)					TOTAL ALLOCATION 2015-2019	EXECUTING ORGANIZATION UNIT
						2015	2016	2017	2018	2019		
(1)	(2)	(3)	(4)		(5)	(6)	(7)	(8)	(9)	(10)	(11)	(12)
VII	PHARMACEUTICAL AND MEDICAL DEVICES PROGRAM	The improved access and quality of pharmacy, and medical devices stocks as well as the household medical supplies	1	Percentages of drugs and vaccines availabilities at Puskesmas level	a. In Districts/cities: (Total cumulative number of medicinal items available in (n) Puskesmas) x (100 divided by (n x total numbers of indicators medicinal items) b. In Province (Total cumulative numbers of medicinal items available in (n) Puskesmas in (y) Districts/cities) x 100 divided by (n x y) x (total numbers of indicators medicinal items)	1,746.5	2,828.2	3,443.4	3,680.4	4,032.4	15,730.9	DIRECTORATE GENERAL OF PHARMACEUTICAL CARE AND MEDICAL DEVICES
			2	Numbers of types of medicinal raw materials, traditional medicines and medical devices that are manufactured in the country (cumulative)	Increment in medicinal raw materials (BBO) ready to be manufactured, and/or made in Indonesia; and types of medical tools manufactured domestically, every year cumulatively							
			3	Percentage of medical devices products and household medical supplies in the market are qualified	(Number of Medical devices and household medical supplies tested and qualified) x 100% divided by total numbers of medical devices and household medical supplies samples tested							

NO	PROGRAM/ ACTIVITY	OBJECTIVE	INDICATOR	CALCULATION METHOD	ALLOCATION (Rp. Billion)					TOTAL ALLOCATION 2015-2019	EXECUTING ORGANIZATION UNIT
					2015	2016	2017	2018	2019		
(1)	(2)	(3)	(4)	(5)	(6)	(7)	(8)	(9)	(10)	(11)	(12)
1	Improvement of Pharmaceutical Care	The improvement of pharmacy service and the rational drug use at healthcare facilities	1	Percentage of Puskesmas delivers pharmaceutical care according to the standard (Numbers of Puskesmas that provide pharmaceutical care) divided by (total number of Puskesmas) x 100%	32.3	38.4	45.0	51.3	59.0	226.0	DIRECTORATE OF PHARMACEUTICAL CARE
			2	Percentage of rational drug use at Puskesmas  <i>Total percentage of each indicators prescription attainment</i> <i>Total Component of indicators' prescription</i> a sigma of : 1)Percentage of antibiotics use in non- Pneumonia ISPA case; 2)Percentage of antibiotics use in non- specific Diarrhea case; 3)Percentage of injection use in Myalgia case; 4) Average drug item type per prescription in 3 cases.							

NO	PROGRAM/ ACTIVITY	OBJECTIVE	INDICATOR		CALCULATION METHOD	ALLOCATION (Rp. Billion)					TOTAL ALLOCATION 2015-2019	EXECUTING ORGANIZATION UNIT
						2015	2016	2017	2018	2019		
(1)	(2)	(3)	(4)		(5)	(6)	(7)	(8)	(9)	(10)	(11)	(12)
2	Increasing Availability of Public Medicines and Healthcare Supplies	The provision of quality, well-distributed and affordable drugs, vaccines and medical supplies at state healthcare facilities	1	Percentage of drugs and vaccines availability at Puskesmas	a. In Districts/cities: Total cumulative numbers of drug items available in (n) Puskesmas divided by (n x total number of indicator drug item) x 100%; b. In province: (Total cumulative number of drug items available in (n) Puskesmas in (y) Districts/cities divided by (n x y x total number of indicator drug item) x 100%	1,500.0	2,556.1	3,151.3	3,367.9	3,689.7	14,265.0	DIRECTORATE OF PUBLIC MEDICINE AND MEDICAL SUPPLIES
			2	Percentages of Pharmaceutical Installations in Districts/cities that perform medicine and vaccine management according to standards	(Numbers of Pharmaceutical Installations in Districts/cities that perform medicine management according to standards) divided by (numbers of Pharmaceutical Installations in Districts/cities all over Indonesia) x 100%							

NO	PROGRAM/ ACTIVITY	OBJECTIVE	INDICATOR		CALCULATION METHOD	ALLOCATION (Rp. Billion)					TOTAL ALLOCATION 2015-2019	EXECUTING ORGANIZATION UNIT
						2015	2016	2017	2018	2019		
(1)	(2)	(3)	(4)		(5)	(6)	(7)	(8)	(9)	(10)	(11)	(12)
3	Medical Devices Production and Distribution Development	The improvement in control of pre and post marketing of medical devices and household health supplies (PKRT)	1	Percentage of qualified medical devices products and household health supplies in the market	(Numbers of qualified PKRT and medical devices samples tested) divided by (numbers of PKRT and medical devices samples tested) x 100%	35.5	39.5	41.4	43.1	47.4	206.9	DIRECTORATE OF MEDICAL DEVICES PRODUCTION AND DISTRIBUTION
			2	Number of medical devices are manufactured in the country (cumulative)	Addition of medical devices types manufactured every year, accumulatively							
			3	Percentage of medical devices and household medical supplies meet the good manufacturing standard (GMP/CPAKB).	Percentage of medical devices and household medical supplies that meet the good manufacturing standard compared to number of production equipment that already have production certificates							
			4	Percentage of pre- market assessments are in line with the good review practices	(Numbers of processed applications that meet service promises) divided by (Numbers of submitted applications) x 100%							

NO	PROGRAM/ ACTIVITY	OBJECTIVE	INDICATOR		CALCULATION METHOD	ALLOCATION (Rp. Billion)					TOTAL ALLOCATION 2015-2019	EXECUTING ORGANIZATION UNIT
						2015	2016	2017	2018	2019		
(1)	(2)	(3)	(4)		(5)	(6)	(7)	(8)	(9)	(10)	(11)	(12)
4	Improvement on Pharmacy Production and Distribution of Medical Devices	The increasing production of raw materials and local medicines and quality improvement of pharmaceutical production and distribution facilities	1	Numbers of medicinal raw materials and traditional medicines are manufactured in the country. (cumulative)	Addition of medicinal raw materials and traditional medicinal raw materials (BBO/BBOT) types ready to be manufactured, and/or made in Indonesia, yearly basis, accumulatively	79.8	88.3	92.4	96.8	106.5	463.8	DIRECTORATE OF PHARMACEUTICAL PRODUCTION AND DISTRIBUTION
			2	Numbers of industries utilize medicinal raw materials and traditional medicines that are manufactured in the country. (cumulative)	Additional numbers of industries that use BBO/BBOT manufactured domestically, yearly basis, accumulatively							
5	Management Support and Implementation of Other Technical Tasks in Pharmaceutical and Medical Devices Programs	The increasing management support and implementation of other technical tasks in pharmaceutical and medical devices program		Percentage of client satisfaction on the management support	Percentage of satisfaction: (Number of items that satisfied the clients) / (numbers of services rendered) x 100%	98.9	105.9	113.3	121.3	129.8	569.2	SECRETARIAT DIRECTORATE GENERAL OF PHARMACEUTICAL AND MEDICAL DEVICES

NO	PROGRAM/ ACTIVITY	OBJECTIVE	INDICATOR		CALCULATION METHOD	ALLOCATION (Rp. Billion)					TOTAL ALLOCATION 2015-2019	EXECUTING ORGANIZATION UNIT
						2015	2016	2017	2018	2019		
(1)	(2)	(3)	(4)		(5)	(6)	(7)	(8)	(9)	(10)	(11)	(12)
VIII	HUMAN RESOURCES FOR HEALTH DEVELOPMENT AND EMPOWERMENT PROGRAM	The improvement on availability and quality of human resource for health according to the health care standard	1	Numbers of Puskesmas that has at minimum 5 types of human resource of health each	Puskesmas’ absolute score of having standardized HRHs such as sanitarians, pharmacist, nutritionist, public health officers, and lab analyst	3,000.	6,505.	7,087.	7,677.	8,536.	32,807.4	HUMAN RESURCE FOR HEALTH DEVELOPMENT AND EMPOWERMENT INSTITUTE
			2	Percentage of class-C district/city hospitals that have had four basic specialist doctor and three ancillary specialist doctor	Numbers of class-C distric/city hospitals with four basic specialist doctors (Ob-Gyn, Pediatric, Internist, and Surgeon) and Three ancillary specialists divided by total numbers of Class-C district/city Hospitals							
			3	Numbers of HRHs with improved competency (cumulative)	Number of public officials, teachers and teacher’ education and private HRHs and communities with improved competence through certification from accredited educations and trainings							

NO	PROGRAM/ ACTIVITY	OBJECTIVE	INDICATOR	CALCULATION METHOD	ALLOCATION (Rp. Billion)					TOTAL ALLOCATION 2015-2019	EXECUTING ORGANIZATION UNIT
					2015	2016	2017	2018	2019		
(1)	(2)	(3)	(4)	(5)	(6)	(7)	(8)	(9)	(10)	(11)	(12)
1	Standardization, Certification, and Continuous Education of Human Resource for Health	The organization of standardization, certification, and continuous education of human resource for health	Numbers of registered human resources of health	Numbers of Letter of registration (STR) per year	23.0	25.0	31.0	27.0	29.0	135.1	CENTER FOR STANDARDIZATION, CERTIFICATION, CONTINUOUS EDUCATION OF HUMAN RESOURCE FOR HEALTH
2	Higher Education and Improvement of Human Resource for Health Quality	The increasing organization of advanced education and quality improvement of human resource for health	Numbers of new participants of education assistance aid and scholarship	Numbers of education assistance aid and scholarship recipients(Diploma/Higher Graduates and PPDS/PPDGS per year)	446.0	463.6	486.0	507.1	529.3	2,432.0	
3	Public Officials' Education and Training	The improvement of public officials for health education and training	Number of public officials for health obtaining certificate from accredited trainings (cumulative)	Based on number of certificates issued for training participants who have undergone accredited training	173.4	179.0	188.0	196.4	205.3	942.1	CENTER FOR PUBLIC OFFICIALS EDUCATION AND TRAINING
4	Human Resource for Health Education and Training	The increasing number of education and training for Human Resource for Health	Number of educators, human resources for health and members of community with improved competence through training	Number of certificates issued for training participants who have undergone accredited training	20.0	21.0	22.1	23.0	24.1	110.2	CENTER FOR HUMAN RESOURCE FOR HEALTH EDUCATION AND TRAINING
5	The Management of Higher Education Quality	The quality improvement of higher education	Percentage of study programs in Health of Polytechnic (Poltekkes) of the Ministry of Health that have been accredited.	Number of study program/Health Polytechnics with good accreditation compared to number of expired study programs and new study programs times one hundred percent	24.0	25.2	26.5	27.7	28.9	132.2	

NO	PROGRAM/ ACTIVITY	OBJECTIVE	INDICATOR	CALCULATION METHOD	ALLOCATION (Rp. Billion)					TOTAL ALLOCATION 2015-2019	EXECUTING ORGANIZATION UNIT
					2015	2016	2017	2018	2019		
(1)	(2)	(3)	(4)	(5)	(6)	(7)	(8)	(9)	(10)	(11)	(12)
6	Planning and Efficient use of Human resource for health	Having a better planning and efficient use of human resource for health	Numbers of human resource for health are absorbed to work in health services facilities	Absolute score of health workers assigned in Puskesmas or government hospitals through mechanisms of PNS, PPPK, and special assignments (team based & individual)	18.7	2,550.3	2,949.4	3,348.4	3,947.5	12,814.3	CENTER FOR PLANNING AND EMPOWERMENT OF HUMAN RESOURCE FOR HEALTH
7	Human resource for health Planning	The increasing implementation of health professional planning	Numbers of planning documents of human resource for health	Absolute score of Human Resource for Health policy and planning documents	0.0	49.7	50.6	51.6	52.5	204.4	
8	Internship for Human Resources of Health	Established internship opportunity for human resources of health	Number of HRHs who undergo internship	Absolute score of HRHs who undergo internship	250.0	750.0	800.0	850.0	900.0	3,550.00	
9	Management support and implementation of other technical tasks in the Development and Empowerment of Human resource for health	The improvement on management support and implementation of other technical tasks in the development and empowerment of human resource for health.	1 Numbers of documents regulating norms, standards, procedures and criteria of the development and empowerment of human resource for health criteria of PPSPDMK	Calculation of number of regulation draft documents Designed/made/created by centers and secretariats in the environment of Health PPSPDM Institutions In the form of drafts of bills, Government Regulations (PP), Presidential Regulations (Perpres), Health Ministerial Regulations (Permenkes), Health Ministerial Decree (Kepmenkes), technical guidance, Technical instructions, partnership agreements,	1,315.5	1,512.3	1,569.8	1,648.2	1,786.4	7,832.2	SECRETARIATE OF AGENCY FOR DEVELOPMENT AND EMPOWERMENT OF HEALTH MANPOWER

NO	PROGRAM/ ACTIVITY	OBJECTIVE	INDICATOR	CALCULATION METHOD	ALLOCATION (Rp. Billion)					TOTAL ALLOCATION 2015-2019	EXECUTING ORGANIZATION UNIT
					2015	2016	2017	2018	2019		
(1)	(2)	(3)	(4)	(5)	(6)	(7)	(8)	(9)	(10)	(11)	(12)
				and standard procedure related to BPPSDM							
			2 Numbers of documents containing data and information of PPSDM programs	Creation of documents based on data processing And information from Central and Provincial Health PPSDM	-	20.0	25.0	30.0	35.0	110.0	SECRETARIATE OF AGENCY FOR DEVELOPMENT AND EMPOWERMENT OF HEALTH MANPOWER
10	Development and Management of Higher Education	Improvement in higher education development and management	1 Numbers of graduates from MOH's Poltekkes	Recapitulation of number of education participants who have graduated/completed their education in 38 Health Polytechnics from various study programs of various education levels	730.0	909.0	939.5	968.2	998.3	4,545.0	
			2 Numbers of working units have their facilities upscale (cumulative)	Recapitulation of number of work units who have carried out building and environment construction/rehabilitation, ABBM procurement, and operational vehicle procurement							

NO	PROGRAM/ ACTIVITY	OBJECTIVE	INDICATOR		CALCULATION METHOD	ALLOCATION (Rp. Billion)					TOTAL ALLOCATION 2015-2019	EXECUTING ORGANIZATION UNIT
						2015	2016	2017	2018	2019		
(1)	(2)	(3)	(4)		(5)	(6)	(7)	(8)	(9)	(10)	(11)	(12)
IX	HEALTH RESEARCH AND DEVELOPMENT	The quality improvement of research and development in health sector	1	Numbers of documents containing research results are registered under HKI	Calculation of cumulative number of health research and development results registered by HKI proven with a registration number	744.7	1,450.0	1,850.0	2,200.0	2,650.0	8,894.7	NATIONAL HEALTH RESEARCH AND DEVELOPMENT INSTITUTE
			2	Number of policy recommendations that are based on health research and development which are being advocated to health programme manager and or to stakeholders	Calculation of cumulative number of policy recommendations (policy brief/policy paper) written based on health research and development results presented in a forum or meeting to program managers and/or stakeholders proven with a policy paper and forum/meeting report (Calculation of target/baseline based on recommendation calculation according to strategic issues advocated)culating cumulative number of policy brief/policy paper formulated based on the result of litbangkes presented in the forums or meetings to program manager and or stakeholders and it's proven by the presence of policy paper and forum/meeting reports (calculating target/baseline based on the calculation of recommendations that correspond to advocated strategic issues)							

NO	PROGRAM/ ACTIVITY	OBJECTIVE	INDICATOR	CALCULATION METHOD	ALLOCATION (Rp. Billion)					TOTAL ALLOCATION 2015-2019	EXECUTING ORGANIZATION UNIT	
					2015	2016	2017	2018	2019			
(1)	(2)	(3)	(4)		(5)	(6)	(7)	(8)	(9)	(10)	(11)	(12)
			3	Numbers of report documents on national health research (Riskesnas) in health and public nutrition	Calculation of cumulative number of Riskesnas reports written based on health research and development results, proven with a National Riskesnas Report							NATIONAL HEALTH RESEARCH AND DEVELOPMENT INSTITUTE
1	Research and Development in Biomedical and Basic Medical Technology	The increasing number of researches and developments in biomedical and basic medical technology	1	Number of policies recommendations which are derived from researches and development in biomedical and basic medical technology	Calculation of cumulative number of policy recommendations (policy brief/policy paper) written based on health research and development in the form of proven output of policy brief/policy paper draft (Calculation of target/baseline based on based on recommendation calculation according to strategic issues in Biomedical and Basic Health Technology)	111.6	197.3	214.2	378.6	415.8	1,317.4	CENTER FOR BIOMEDIC AND BASIC HEALTH TECHNOLOGY
			2	Numbers of publications of scientific papers on biomedical and basic medical technology are published in national and international printed and or electronic media	Calculation of cumulative number of health research and development articles in biomedical and basic health technology published in national and international print media and/or electronic media which is accredited and written by researchers of National Health Research and Development Institution (Badan Litbangkes) as the first author	23.7	86.0	51.8	71.7	87.7	320.9	

NO	PROGRAM/ ACTIVITY	OBJECTIVE	INDICATOR		CALCULATION METHOD	ALLOCATION (Rp. Billion)					TOTAL ALLOCATION 2015-2019	EXECUTING ORGANIZATION UNIT
						2015	2016	2017	2018	2019		
(1)	(2)	(3)	(4)		(5)	(6)	(7)	(8)	(9)	(10)	(11)	(12)
2	Research and Development in People’s Health Intervention Technology	The increasing number of researches and developments on public health intervention technology	1	Numbers of policies recommendations which are derived from researches and development in public health intervention technology	Calculation of cumulative number of policy recommendations (policy brief/policy paper) written based on health research and development in the form of proven output of policy brief/policy paper draft (Calculation of target/baseline based on based on recommendation calculation according to strategic issues in People’s Health Intervention Technology)	69.3	169.4	331.7	353.5	458.3	1,382.1	CENTER OF PUBLIC HEALTH INTERVENTION TECHNOLOGY
			2	Numbers of publications of scientific papers on public health intervention technology are published in national and international printed and or electronic media.	Calculation of cumulative number of health research and development articles in People’s Health Intervention Technology published in national and international print media and/or electronic media which is accredited and written by researchers of National Health Research and Development Institute (Badan Litbangkes) as the first author	14.9	37.5	48.3	55.9	56.2	212.7	

NO	PROGRAM/ ACTIVITY	OBJECTIVE	INDICATOR	CALCULATION METHOD	ALLOCATION (Rp. Billion)					TOTAL ALLOCATION 2015-2019	EXECUTING ORGANIZATION UNIT	
					2015	2016	2017	2018	2019			
(1)	(2)	(3)	(4)	(5)	(6)	(7)	(8)	(9)	(10)	(11)	(12)	
3	Research and Development on Medical Applied Technology and Clinical Epidemiology	The increasing number of researches and developments in medical applied technology and clinical epidemiology	1	Numbers of policies recommendations which are derived from researches and development in medical applied technology and clinical epidemiology	Calculation of cumulative number of policy recommendations (policy brief/policy paper) written based on health research and development in the form of proven output of policy brief/policy paper draft (Calculation of target/baseline based on based on recommendation calculation according to strategic issues in Applied Health Technology and Clinical Epidemiology)	28.7	137.8	225.0	340.1	449.0	1,180.5	CENTER FOR APPLIED HEALTH TECHNOLOGY AND CLINICAL EPIDEMIOLOGY
			2	Numbers of publications of scientific papers on medical applied technology and clinic epidemiology are published in national and international printed and or electronic media	Calculation of cumulative number of health research and development articles in Applied Health Technology and Clinical Epidemiology published in national and international print media and/or electronic media which is accredited and written by researchers of National Health Research and Development Institute (Badan Litbangkes) as the first author	9.4	31.4	24.7	35.3	52.3	153.1	

NO	PROGRAM/ ACTIVITY	OBJECTIVE	INDICATOR	CALCULATION METHOD	ALLOCATION (Rp. Billion)					TOTAL ALLOCATION 2015-2019	EXECUTING ORGANIZATION UNIT	
					2015	2016	2017	2018	2019			
(1)	(2)	(3)	(4)	(5)	(6)	(7)	(8)	(9)	(10)	(11)	(12)	
4	Research and Development on Humanity, Health Policies and Community Empowerment	The increasing number of researches and developments on humanity, health policies and community empowerment	1	Numbers of policies recommendations which are derived from researches and development in humanity, health policies and community empowerment	Calculation of cumulative number of policy recommendations (policy brief/policy paper) written based on health research and development in the form of proven output of policy brief/policy paper draft (Calculation of target/baseline based on based on recommendation calculation according to strategic issues in Ethics, Health Policies, and People's Empowerment)	66.2	119.1	164.5	288.6	305.6	943.9	CERNTER FOR ETHICS, HEALTH POLICIES, AND PEOPLE'S EMPOWERMENT
			2	Numbers of publications of scientific papers on humanity, health policies and community empowerment are published in national and international printed and or electronic media.	Calculation of cumulative number of health research and development articles in Ethics, Health Policies, and People's Empowerment published in national and international print media and/or electronic media which is accredited and written by researchers of National Health Research and Development Institute (Badan Litbangkes) as the first author	3.5	6.7	9.0	21.6	29.7	70.5	

NO	PROGRAM/ ACTIVITY	OBJECTIVE	INDICATOR		CALCULATION METHOD	ALLOCATION (Rp. Billion)					TOTAL ALLOCATION 2015-2019	EXECUTING ORGANIZATION UNIT
						2015	2016	2017	2018	2019		
(1)	(2)	(3)	(4)		(5)	(6)	(7)	(8)	(9)	(10)	(11)	(12)
5	Research and Development on Medicinal Plants and Traditional Medicines	The increasing number of research and development on medicinal plants and traditional medicines	1	Numbers of policies recommendations which are derived from researches and development on medicinal plants and traditional medicines	Calculation of cumulative number of policy recommendations (policy brief/policy paper) written based on health research and development in the form of proven output of policy brief/policy paper draft (Calculation of target/baseline based on based on recommendation calculation according to strategic issues in Medicine Plants and Traditional Medicine)	98.9	68.5	158.7	157.1	270.4	753.6	GRAND HOUSE OF RESEARCH AND DEVELOPMENT IN MEDICINE PLANTS AND TRADITIONAL MEDICINE
			2	Numbers of publications of scientific papers on medicinal plants and traditional medicines are published in national and international printed and or electronic media	Calculation of cumulative number of health research and development articles in Medicine Plants and Traditional Medicine published in national and international print media and/or electronic media which is accredited and written by researchers of National Health Research and Development Institute (Badan Litbangkes) as the first author	14.7	57.6	51.1	67.7	40.9	232.0	

NO	PROGRAM/ ACTIVITY	OBJECTIVE	INDICATOR		CALCULATION METHOD	ALLOCATION (Rp. Billion)					TOTAL ALLOCATION 2015-2019	EXECUTING ORGANIZATION UNIT
						2015	2016	2017	2018	2019		
(1)	(2)	(3)	(4)		(5)	(6)	(7)	(8)	(9)	(10)	(11)	(12)
6	Research and Development on Vector and Reservoir of Diseases	The increasing number of research and development on vector and reservoir of diseases	1	Numbers of policies recommendations which are derived from researches and development on vector and reservoir of diseases.	Calculation of cumulative number of policy recommendations (policy brief/policy paper) written based on health research and development in the form of proven output of policy brief/policy paper draft (Calculation of target/baseline based on based on recommendation calculation according to strategic issues in Disease Vector and Reservoir)	90.9	240.7	322.8	168.8	203.8	1,026.9	GRAND HOUSE OF RESEARCH AND DEVELOPMENT IN DISEASE VECTOR AND RESERVOIR
			2	Numbers of publications of scientific papers on vector and reservoir of diseases are published in national and international printed and or electronic media	Calculation of cumulative number of health research and development articles in Disease Vector and Reservoir published in national and international print media and/or electronic media which is accredited and written by researchers of National Health Research and Development Institute (Badan Litbangkes) as the first author	17.4	24.3	24.8	26.7	34.1	127.2	

NO	PROGRAM/ ACTIVITY	OBJECTIVE	INDICATOR	CALCULATION METHOD	ALLOCATION (Rp. Billion)					TOTAL ALLOCATION 2015-2019	EXECUTING ORGANIZATION UNIT	
					2015	2016	2017	2018	2019			
(1)	(2)	(3)	(4)	(5)	(6)	(7)	(8)	(9)	(10)	(11)	(12)	
7	Management Support and Implementation of other technical tasks in Research and Development Program in Health Sector	The improvement of management support and implementation of other technical tasks towards research and development program	1	Numbers of concerning management support towards research and development in health sector	Calculation of cumulative number of reports on implementation in the areas of Planning and Budgeting; Policy Recommendations (Policy Brief/Policy Paper); Publication and Dissemination; Finance and General Affairs; Organization and Employee Affairs Law; Scientific and Ethics Management	162.6	218.8	161.6	181.7	187.6	912.4	SECRETARIAT OF NATIONAL HEALTH RESEARCH AND DEVELOPMENT INSTITUTE
			2	Number of reports concerning management support towards technical research and development in health sector.	Calculation of cumulative number of reports on management of National Research, Health Development Research, Medicine Science and Technology (IPTEKDOK) Research, Contingency Research	33.0	55.2	61.8	52.8	58.7	261.4	
TOTAL BUDGET OF MINISTRY OF HEALTH 2015-2019						50,466.1	78,478.8	88,471.8	107,020.9	128,227.6	449,506.9	

MINISTER OF HEALTH OF THE REPUBLIC OF INDONESIA,

[SIGNED]

NILA FARID MOELOEK

APPENDIX III

MATRIX OF FRAMEWORK REGULATORY REQUIREMENTS



APPENDIX III
MINISTER OF HEALTH DECREE
NUMBER CONCERNING HK.02.02/MENKES/52/2015
ON
THE MINISTRY OF HEALTH STRATEGIC PLAN
YEARS 2015-2019

MATRIX OF FRAMEWORK REGULATORY REQUIREMENTS

No	STRATEGIC TARGET /PROGRAM	LEVEL/CONTENT OF REGULATION	REASONS FOR URGENT ISSUANCE OF REGULATION-NSPK	MAIN UNIT/WORKING UNIT	PLAN OF ISSUANCE YEAR
1	The improvement of community health	1. Permenkes on Quality standard of Environment Health and Health Conditions for Water Medium and Sanitation Intervention.		DGDCEH	2015
		2. Permenkes on Quality standard of Environment Health and Health Conditions for Air Medium and Sanitation Intervention.		DGDCEH	2015
		3. Permenkes on Quality standard of Environment Health and Health Conditions for Soil Medium and Sanitation Intervention.		DGDCEH	2016
		4. Permenkes on Quality standard of Environment Health and Health Conditions for Food Medium and Sanitation Intervention.		DGDCEH	2016
		5. Permenkes on Quality standard of Environment Health and Health Conditions for Facilities, Buildings and Sanitation Intervention		DGDCEH	2017
		6. Permenkes on Quality standard of Environment Health in Vector borne diseases		DGDCEH	2015
		7. Permenkes on public health protection intervention from chemical substances, physics disruption to the air and pesticides		DGDCEH	2016

No	STRATEGIC TARGET/PROGRAM	LEVEL/CONTENT OF REGULATION	REASONS FOR URGENT ISSUANCE OF REGULATION- NSPK	MAIN UNIT/WORKING UNIT	PLAN OF ISSUANCE YEAR
		8. Permenkes on Monitoring of medical waste at Fsyankes/ health service facility through surveillance, laboratory testing, and risk analysis		DGDCEH	2016
		9. Permenkes on hotels' health certificates		DGDCEH	2015
		10. Permenkes on environmental health services in Puskesmas		DGDCEH	2015
		11. Permenkes on standard and requirements of food sanitation and hygiene		DGDCEH	2016
		12. Permenkes on posbindu PTM		DGDCEH	2016
		13. Permenkes on control of PTM		DGDCEH	2015
		14. Permenkes on thalasemia		DGDCEH	2017
		15. Permenkes on erythematosus systemic lupus		DGDCEH	2018
		16. Permenkes on unregulated tobacco production		DGDCEH	2016
		17. Permenkes on additional substances on tobacco products		DGDCEH	2017
		18. Permenkes on guideline for early findings of cancer on children		DGDCEH	2016
		19. Permenkes on guideline for palliative care for cancer		DGDCEH	2017
		20. Permenkes on guideline of injuries control		DGDCEH	2017
		21. SKB/Joint Ministerial Decree with the Minister of National Education, other LS (Lintas Sektor/Cross Sectoral)/RPM (Registered Portfolio Manager) on guideline for UKS acceleration		DGDCEH	2018
		22. Perpres on National Action Plan for school-age children and adolescent health		DG of MCH and Nutrition Development (DGMCHND)	2016
		23. RPM on integrated service for adolescent health		DGDCEH	2016
		24. RPM on national standard guideline of PKPR		DGDCEH	2017
		25. RPM on PKPR management guideline		DGDCEH	2017

No	STRATEGIC TARGET/PROGRAM	LEVEL/CONTENT OF REGULATION	REASONS FOR URGENT ISSUANCE OF REGULATION- NSPK	MAIN UNIT/WORKING UNIT	PLAN OF ISSUANCE YEAR
		26. SKB with the minister of national education, other LS/lesson study to fulfill nutrition needs at school		DGDCEH	2016
		27. RPM on types of complementary traditional healthcare at Fasyankes		DGDCEH	2015
		28. RPM on occupational health surveillance		DGDCEH	2015
		29. RPM on health workers		DGDCEH	2018
		30. Permenkes on improvement of health and prevention of diseases		DGDCEH	2015
		31. Permenkes on advocacy guidance		DGDCEH	2018
		32. Permenkes on campaign/KIE (Komunikasi, Informasi dan Edukasi/Communication, Information and Education) guidance		DGDCEH	2019
		33. Permenkes/joint regulation on improving healthy behavior		DGDCEH	2018
		34. PP on Occupational Health	CHAPTER XII on Occupational Health, Article 164 Paragraph (5)	DGMCHND	2015
		35. PP on School Health	Mandate of Law Number 36/2009 CHAPTER VI on Health Intervention, Section Eighth teen on School Health	DGMCHND	2016
		36. Completion of derivatives regulation from PP Number. 61/2013 on reproductive health		DGMCHND	2016-2019
		37. Perpres on National Action Plan for Acceleration of		DGMCHND	2017
		38. AKI 2016 – 2030		DGMCHND	2017
		39. RPM Holding of training for assessing growth monitoring		DGMCHND	2016-2019
		40. RPM assessment standard for monitoring under-five growth		DGMCHND	2016-2019
		41. Perpres on guidelines of screening and regular examination		DGMCHND	2016-2019
		42. Perpres on adolescent integrated healthcare		DGMCHND	2016-2019

No	STRATEGIC TARGET/PROGRAM	LEVEL/CONTENT OF REGULATION	REASONS FOR URGENT ISSUANCE OF REGULATION- NSPK	MAIN UNIT/WORKING UNIT	PLAN OF ISSUANCE YEAR
		43. Perpres on guidance for acceleration of UKS		DGMCHND	2016-2019
		44. Perpres on Guidance for National Standard of PKPR		DGMCHND	2016-2019
		45. Perpres on Guidance of PKPR trainings for health workers		DGMCHND	2016-2019
		46. Perpres on guidance of PKPR management		DGMCHND	2016-2019
		47. Perpres on Health Report Book		DGMCHND	2016-2019
		48. RPM on types of complementary traditional healthcare in Fasyankes		DGMCHND	2016-2019
		49. RPM on permission of use of equipments and techNumberlogy		DGMCHND	2017
		50. RPM on complementary traditional healthcare		DGMCHND	2017
		51. RPM on traditional health workers training		DGMCHND	2016
		52. RPM on traditional healthcare facilities		DGMCHND	2016
		53. RPM on traditional health worker licenses		DGMCHND	2015
		54. RPP on mental health intervention		DG of Healthcare Provision (DGHP)	2016
2	The improvements in controlling diseases	1. R. Perpres on implementation of healthy district/city		DGDCEH	2015
		2. RUU on health quarantines		DGDCEH	2015
		3. RUU on outbreaks		DGDCEH	2016
		4. Control of TB 2015-2019		DGDCEH	2015
		5. Kepmenkes on national strategy to control TB 2015-2019		DGDCEH	2015
		6. Kepmenkes on guidance for integrated management to control drug-resistant-TB		DGDCEH	2016
		7. Kepmenkes on guidance for collaboration management of		DGDCEH	2016
		8. TB HIV			
		9. Control of acute respiratory tract infection (ISPA) pneumonia and alert against influenza pandemics		DGDCEH	2016
		10. SUFA-PPIA		DGDCEH	2016
		11. Elimination of leprosy		DGDCEH	2016
		12. Eradication of Framboesia		DGDCEH	2016

No	STRATEGIC TARGET/PROGRAM	LEVEL/CONTENT OF REGULATION	REASONS FOR URGENT ISSUANCE OF REGULATION- NSPK	MAIN UNIT/WORKING UNIT	PLAN OF ISSUANCE YEAR
		13. Committee of Experts in Leprosy Elimination and Eradication of Framboesia		DGDCEH	2017
		14. Roadmap of Leprosy		DGDCEH	2017
		15. Roadmap of Framboesia		DGDCEH	2017
		16. 2015-2019 Action plan for controlling Hepatitis, diarrhea and ISPA (Infeksi Saluran Pernafasan Akut/Acute Respiratory Infection)		DGDCEH	2015
		17. Permenkes on dengue fever control		DGDCEH	2015
		18. Permendagri on dengue fever operational working group (Pokjnal/operational working group)		DGDCEH	2016
		19. Permenkes on vector control		DGDCEH	2015
		20. Permenkes on zooNumbertic control		DGDCEH	2016
		21. Permenkes on implementing instruction of national campaign of filariasis prevention of mass drug (POMP/ Pemberian Obat Masal Pencegahan)		DGDCEH	2016
		22. Permenkes on guidelines of schistosomiasis elimination		DGDCEH	2018-2019
		23. Permenkes on guideline of helminthes diseases control		DGDCEH	2018-2019
		24. Perpres on neglected tropical diseases control in Indonesia		DGDCEH	2018-2019
		25. Document on health quarantine		DGDCEH	2016
		26. Surveillance on quarantine in entry points		DGDCEH	2017
		27. Implementation of health quarantine		DGDCEH	2017
		28. Guideline on comprehensive health		DGDCEH	2018-2019
		29. Guideline on surveillance		DGDCEH	2017
		30. Utilization of inactivated polio vaccines in immunization program		DGDCEH	2017
		31. Elimination of measles and control of rubella and syndrome of rubella congenital		DGDCEH	2017
		32. Surveillance on comprehensive health		DGDCEH	2016
		33. Guidance on healthcare administration for the hajj pilgrims in Indonesia		DGDCEH	2016

No	STRATEGIC TARGET/PROGRAM	LEVEL/CONTENT OF REGULATION	REASONS FOR URGENT ISSUANCE OF REGULATION- NSPK	MAIN UNIT/WORKING UNIT	PLAN OF ISSUANCE YEAR
		34. Technical guidance of medical examination for hajj pilgrims, in quality control framework		DGDCEH	2017-2019
		35. Guidance of hajj pilgrim healthcare in Indonesia and Saudi Arabia in quality control framework (Numberte : ISO 9001;2008)		DGDCEH	2017-2019
		36. Guidance on health development for Indonesian hajj pilgrims, implemented integrally with other program		DGDCEH	2017-2019
		37. Health information system of Indonesian Hajj pilgrims, which is connected with hajj information system of the Ministry of Religious Affairs of the Republic of Indonesia		DGDCEH	2017-2019
		38. RPM on health professionals for workers		DGDCEH	2017-2019
		39. Permenkes on improvement of health and diseases control		DGDCEH	2015
3	Better access and improved quality of health service facilities	1. RPM on types of complementary traditional healthcare provision in Fasyankes		DGHP	2016
		2. RPM on complementary traditional healthcare provision		DGHP	2016
		3. RPP on plastic surgery and reconstruction	Mandate of Law Number 36/2009 CHAPTER IV on Healthcare Provision, Section five on healing and health recovery	DGHP	2016
		4. RPM on traditional healthcare provision facilities		DGHP	2017
		5. RPM on occupational health examination		DGHP	2017

No	STRATEGIC TARGET/PROGRAM	LEVEL/CONTENT OF REGULATION	REASONS FOR URGENT ISSUANCE OF REGULATION- NSPK	MAIN UNIT/WORKING UNIT	PLAN OF ISSUANCE YEAR
		6. PP on quality standard of healthcare provision	Mandate of Law Number 36/2009 CHAPTER VI on Health Provision, Section Two on Healthcare, Article 55 paragraph (2)	DGHP	2017
		7. RPP on transplantation of organ, tissues and/or cells	Mandate of Law Number 36/2009 CHAPTER VI on Health Provision, Section five on healing and health recovery, Article 65 paragraph (3)	DGHP	2015
		8. RPP on healthcare provision facilities	Mandate of Law Number 36/2009 CHAPTER V on resources in Health sector, Article 35 paragraph (5)	DGHP	2015
		9. RPP on drug implant and/or medical devices	Mandate of Law Number 36/2009 CHAPTER VI on Health Provision, Section four on improvement of health and diseases control, Article 68 paragraph (2)	DGHP	2016
		10. RPP on monitoring on service facilities outside health sector and community based health facilities		DGHP	2016
		11. RPP on teaching hospitals		DGHP	2015
		12. RPP on subsidy and government aid for hospital financing		DGHP	2015
		13. R.Perpres on coordination of mental healthcare Provision		DGHP	2015

No	STRATEGIC TARGET/PROGRAM	LEVEL/CONTENT OF REGULATION	REASONS FOR URGENT ISSUANCE OF REGULATION-NSPK	MAIN UNIT/WORKING UNIT	PLAN OF ISSUANCE YEAR
		14. R.Permenkes on ODGJ (person with mental disorder) management with other number-medical science method		DGHP	2016
		15. R.Permenkes on psychiatric or psychosocial rehabilitation program		DGHP	2016
		16. R.Kepmenkes on Center of Research, Development and Use of Technology and its Product in Mental Health sector		DGHP	2017
		17. R.Permenkes on guideline in mental health examination for legal purposes, works, or certain positions		DGHP	2015
		18. Inpres draft on cooperation of Puskesmas with UTD in providing blood transfusion		DGHP	2015
		19. R.Permenkes on medical records		DGHP	2018
4	The increasing of number, types, and improvement of quality and equality of human resources for health (HRH)	1. PP on internship		Badan PPSDM (HRHDEI)	2017-2019
		2. PBM (joint ministerial regulation) on planning and distribution of HRHs in local government owned-healthcare facilities		HRHDEI	2016
		3. Permenkes on employment of skilled HRHs in all health institutions		HRHDEI	2016
		4. RPM on traditional HRHs licenses		HRHDEI	2016
		5. Permenkes on Guidelines of Planning and Career Development of HRHs		HRHDEI	2017
		6. Data and statistic of HRHs (Profile of SDM (Sumber Daya Manusia Kesehatan/Health Human Resource))		HRHDEI	2017
		7. Annual planning for the needs of HRHs (National Scale)		HRHDEI	2017
		8. Annual planning & evaluation of the utilization of HRHs (National Scale)		HRHDEI	2018
		9. Technical guidelines on HRHs long-term planning		HRHDEI	2018
		10. Technical guidelines on licensing of the employment of foreign HRHs in healthcare activities		HRHDEI	2018

No	STRATEGIC TARGET/PROGRAM	LEVEL/CONTENT OF REGULATION	REASONS FOR URGENT ISSUANCE OF REGULATION- NSPK	MAIN UNIT/WORKING UNIT	PLAN OF ISSUANCE YEAR
		11. Guidelines for Indonesian health professionals overseas graduates in health development		HRHDEI	2019
		12. Guidelines on the employment of Indonesian caregiver abroad		HRHDEI	2019
		13. Technical guidelines on implementing the Permenkes on planning, recruiting, employment and development and monitoring of traditional, alternative and complementary (Tradkom) HRHs		HRHDEI	2019
		14. Technical guidelines on employment of foreign HRHs in educational activities		HRHDEI	2017-2019
		15. Joint Regulation on Bilateral cooperation to fill the technology knowledge gap of Indonesian health professionals		HRHDEI	2017-2019
		16. Technical guidelines on employment of Indonesia Health worker abroad, for the region areas		HRHDEI	2017-2019
		17. Technical guidelines on licensing, employment of migrant health workers (TK-WNA/Tenaga Kerja-Warga Negara Asing/employment of foreign) for social works in health sector		HRHDEI	2017-2019
		18. Joint Regulation on the employment of Migrant Health worker abroad		HRHDEI	2017-2019
		19. Technical guidelines on the implementation of Permenkes on Guidelines of Planning and Career Development of HRHs		HRHDEI	2017-2019
		20. Technical guidelines on licensing, employment of TK-WNA in health research activities		HRHDEI	2017-2019
		21. Technical guidelines on Development and Monitoring of Indonesian migrant health worker		HRHDEI	2017-2019
		22. PBM on planning and distribution of HRHs in local government owned-healthcare facilities		HRHDEI	2016
		23. Distribution model of HRHs		HRHDEI	2016
		24. Financial and numbers financial incentive system		HRHDEI	2016

No	STRATEGIC TARGET/PROGRAM	LEVEL/CONTENT OF REGULATION	REASONS FOR URGENT ISSUANCE OF REGULATION- NSPK	MAIN UNIT/WORKING UNIT	PLAN OF ISSUANCE YEAR
		25. Guidelines on HRHs distribution in DTPK (underdeveloped, border territories and outlying regions)		HRHDEI	2016
		26. Permenkes on development of HRHs in DTPK		HRHDEI	2016
		27. Retention of HRHs in DTPK		HRHDEI	2017
		28. Guidelines on Facilities		HRHDEI	2017
		29. Guidelines on Competency Test		HRHDEI	2017
		30. Revision on HRHs Education Standard		HRHDEI	2017
		31. Guidelines on Curriculum Formulation for HRH's Education Institution		HRHDEI	2018
		32. Master plan of development and monitoring on quality of HRHs, regulation correspond to development and monitoring of quality of HRHs		HRHDEI	2016-2019
		33. Regulation related to certification and registration of HRHs, regulation related to disciplinary enforcement for HRHs, regulation related to professional development program for HRHs		HRHDEI	2016-2019
		34. Regulation related to the learning task, Regulation related to PDS/PDGS Program, Regulation related to professional development program in preparation of continuous education		HRHDEI	2016-2019
		35. RPP on equality, recognition of credit score the lecturers in nursing education		HRHDEI	2016
		36. RPP on planning, recruiting, employment of health worker (including foreign nurses - mandate of Law on Nursing, education and training for health worker (Diklatnakes) and Rengunakes – Mandate of Law on Health, employment of TKA - mandate of Law on Hospital)		HRHDEI	2015
		37. RPP on Procedure of administrative sanction enforcement for HRHs (includes in the mandate of Law on Nursing)		HRHDEI	2015
		38. R.Perpres on Indonesian HRHs council (including nurse council)		HRHDEI	2015

No	STRATEGIC TARGET/PROGRAM	LEVEL/CONTENT OF REGULATION	REASONS FOR URGENT ISSUANCE OF REGULATION- NSPK	MAIN UNIT/WORKING UNIT	PLAN OF ISSUANCE YEAR
		39. R.Permenkes on types, tasks, and authority of nurses (including nursing care in emergency situation and development and monitoring on nursing practices)		HRHDEI	2016
		40. R.Permenkes on procedures of competency evaluation process for Indonesian nurses graduates abroad		HRHDEI	2016
		41. R.Permenkes on health services needs and/or public health nursing care in a region		HRHDEI	2017
		42. R.Permenkes on HRHs licensing		HRHDEI	2015
		43. R.Permenkes on HRHs assistant		HRHDEI	2015
		44. R.Permenkes on tasks, functions and authorities of HRHs council		HRHDEI	2016
		45. R.Permenkes on disciplinary sanction for HRHs		HRHDEI	2017
		46. R.Permenkes on competency evaluation of Indonesian migrant workers (TKWNI) graduates abroad		HRHDEI	2017
		47. R.Permenkes on implementing practices correspond to authority based on competency		HRHDEI	2017
		48. R.Permenkes on practicing profession outside his/her authority		HRHDEI	2017
		49. R.Permenkes on action delegation		HRHDEI	2018-2019
		50. R.Permenkes on standard of profession, profession services and standard of operational procedure		HRHDEI	2018-2019
		51. R.Permenkes on approval for health worker actions		HRHDEI	2018-2019
		52. R.Permenkes on the confidentiality of healthcare beneficiaries (including the confidentiality of client's health-mandate of Law on Nursing)		HRHDEI	2018-2019
		53. RUU on pharmaceutical care		DG of Pharmaceutical care and Medical Devices (DGPCMD)	2016
5	The improvement on access, self-reliance and	1. Presidential Regulation Draft on Obligation to Use Domestic production of drug's raw materials for producing drugs for		DGPCMD	2017

No	STRATEGIC TARGET/PROGRAM	LEVEL/CONTENT OF REGULATION	REASONS FOR URGENT ISSUANCE OF REGULATION- NSPK	MAIN UNIT/WORKING UNIT	PLAN OF ISSUANCE YEAR
	quality of pharmaceutical care and medical devices	2. Presidential regulation draft on working group (POKJA) and consortium of development, production, and promotion of domestic production drug's raw materials		DGPCMD	2017
		3. Draft of Permenkes revision on pharmaceutical industry		DGPCMD	2015
		4. Pharmacopeia of Indonesia Edition V Supplement 2		DGPCMD	2016
		5. Draft of Kepmenkes on preparatory team		DGPCMD	
		6. Draft of Kepmenkes on the imposition of Pharmacopeia of Indonesia Edition V Supplement 2		DGPCMD	
		7. Herbal Pharmacopeia of Indonesia Edition II		DGPCMD	2017
		8. Draft of Kepmenkes on preparatory team		DGPCMD	
		9. Draft of Kepmenkes on the imposition of Herbal Pharmacopeia of Indonesia Edition II		DGPCMD	
		10. Pharmacopeia of Indonesia Edition V Supplement 3		DGPCMD	2018
		11. Draft of Kepmenkes on preparatory team		DGPCMD	
		12. Draft of Kepmenkes on the imposition of Pharmacopeia of Indonesia Edition V Supplement 3		DGPCMD	
		13. Draft of Kepmenkes on production of narcotics, psychotropic and pharmaceutical precursor		DGPCMD	2015
		14. Draft of Kepmenkes on labeling and publication of narcotics		DGPCMD	2015
		15. Draft of Permenkes on the provision of inflow and outflow of narcotics and psychotropic carried by tourists for personal medical		DGPCMD	2016

No	STRATEGIC TARGET/PROGRAM	LEVEL/CONTENT OF REGULATION	REASONS FOR URGENT ISSUANCE OF REGULATION- NSPK	MAIN UNIT/WORKING UNIT	PLAN OF ISSUANCE YEAR
		16. Draft of Permenkes on the changes of drugs classification		DGPCMD	2015
		17. Draft of Permenkes on the importation of drugs, traditional medicines, foods and beverages through special access scheme		DGPCMD	2015
		18. Draft of Permenkes on technical guidance of inflow and outflow of drugs, traditional medicines carried/sent for personal medical		DGPCMD	2015
		19. Draft of Permenkes on technical guidance for the inflow of donated drugs		DGPCMD	2015
		20. Draft of Permenkes on (Revision) the roadmap of development of drugs' raw materials		DGPCMD	2015
		21. Draft of Permenkes on (Revision) master plan on the development of raw materials for traditional medicines		DGPCMD	2015
		22. Draft of Permenkes on Indonesian foods codex		DGPCMD	2015
		23. Draft of Permenkes on Indonesian cosmetics codex Edition II Volume IV		DGPCMD	2015
		24. Kepmenkes draft on national formulary		DGPCMD	Revision every 2 years, in 2015, 2017, 2019
		25. Draft of Permenkes on technical guidance for the inflow of donated drugs		DGPCMD	2015
		26. Draft of Permenkes on (Revision) the roadmap of development of drugs' raw materials		DGPCMD	2015
		27. Draft of Permenkes on (Revision) master plan on the development of raw materials for traditional medicines		DGPCMD	2015
		28. Draft of Permenkes on Indonesian foods codex		DGPCMD	2015

No	STRATEGIC TARGET/PROGRAM	LEVEL/CONTENT OF REGULATION	REASONS FOR URGENT ISSUANCE OF REGULATION- NSPK	MAIN UNIT/WORKING UNIT	PLAN OF ISSUANCE YEAR
		29. Draft of Permenkes on Indonesian cosmetics codex Edition II Volume IV		DGPCMD	2015
		30. Kepmenkes draft on national formulary		DGPCMD	Revision every 2 years, in 2015, 2017, 2019
		25 Kepmenkes draft on the list of national essential drugs (DOEN)		DGPCMD	Revision every 2 years, in 2015, 2017, 2019
		26 Permenkes draft on implementation guidelines (Manlak) of national formulary		DGPCMD	Revision every 2 years in 2015, 2017, 2019
		27 Kepmenkes draft on Hajj formulary		DGPCMD	Revision for 2017
		28 Permenkes draft on guidelines of antibiotic uses		DGPCMD	2015
		29 Permenkes draft on pharmaceutical service standard in the clinic		DGPCMD	2015
		30 Permenkes draft on one-door-policy of drug management		DGPCMD	2016
		31 Permenkes draft on the guidelines of removals for drugs and medical supplies		DGPCMD	2015
		32 draft of Minister of Health Decree on the guidelines of drug management in government pharmaceutical installation		DGPCMD	2015
		33 Kepmenkes draft on the guidelines for drug quality test sampling in central and local government pharmaceutical installations		DGPCMD	2015
		34 Kepmenkes draft on vaccines and serum prices for immunization program		DGPCMD	2015-2019
		35 Kepmenkes draft on the highest retail prices for generic drugs		DGPCMD	2015-2019

No	STRATEGIC TARGET/PROGRAM	LEVEL/CONTENT OF REGULATION	REASONS FOR URGENT ISSUANCE OF REGULATION- NSPK	MAIN UNIT/WORKING UNIT	PLAN OF ISSUANCE YEAR
		31. Kepmenkes draft on the determination of drugs' price with special scheme 32. Permenkes draft on drugs procurement based on e-catalogue 33. Permenkes draft on the guidelines of medical devices stores 34. Permenkes draft on good method of medical devices production 35. Permenkes draft on the guidelines of the system for e-monitoring post market & surveillance on medical devices and PKRT 36. Permenkes draft on monitoring of medical devices and PKRT in the market 37. Permenkes draft on the reporting of medical devices and PKRT 38. Draft of Permenkes revision number 1189/2010 on the production of medical devices and PKRT 39. Draft of Permenkes revision number 1190/2010 on the distribution permit of medical devices and PKRT 40. Draft of Permenkes revision number 1191/2010 on the distribution of medical devices		DGPCMD DGPCMD DGPCMD DGPCMD DGPCMD DGPCMD DGPCMD DGPCMD DGPCMD	2015-2019 2015 2015 2015 2015 2015-2018 2015-2018 2015-2018 2015-2018 2015-2018
6	The improvement of synergy between ministries/agencies	1. PerGub (Peraturan Gubernur/Governor regulation)/PerBup (Peraturan Bupati/Regent regulation) that guarantees the mobilization of program at provincial, district, city and Puskesmas levels 2. Permenkes on the guidelines of advocacy 3. Kepmenkes on the formulation of national action plan of health development prioritized program		Center of Health Promotion Center of Health Promotion Bureau of Planning and Budgeting	2017 2017 2016
7	The improvement of utilization of partnership nationally and internationally	1. Permenkes on partnership guidance 2. Permenkes on partnership guidance with do numbers and international (counterparts)		Center of Health Promotion Center of International	2017 2018

No	STRATEGIC TARGET/PROGRAM	LEVEL/CONTENT OF REGULATION	REASONS FOR URGENT ISSUANCE OF REGULATION- NSPK	MAIN UNIT/WORKING UNIT	PLAN OF ISSUANCE YEAR
8	The improvement of integrated planning, technical guidance and monitoring-evaluation	3. Permenkes on guidance of formulation of integrated planning in the provincial level		Bureau of Planning and Budgeting	2015
		4. Kepmenkes on guidance of implementation of integrated bimtek and money to the regions		Bureau of Planning and Budgeting	2015
		5. PP on Health Minimum Service Standard	Mandate of Law Number 36/2009 CHAPTER VI on health intervention, Article 51 paragraph (3)	Bureau of Planning and Budgeting	2015
		6. PERPRES on advisory agency in health sector	Mandate of Law Number 36/2009 CHAPTER XVII on Advisory Agency in Health Sector, First Section, Article 177 paragraph (4)	Secretariat General	2016
9	The improvement of the effectiveness on health research and development	1. On the use of biological substances and prohibition of uses of biological substances as weapons (bill)		National Institute of Health Research and Development/ NIHRD	2016
		2. RPP on research and development in health sector		NIHRD	2015
		3. On cohort study of health sector in the scope of the ministry of health (verbal process for acquiring the signature of the minister of health)		NIHRD	2015
		4. On the ethic guidelines for research in health sector		NIHRD	2016
		5. On licensing for high risk and dangerous health researches and development		NIHRD	2017
		6. On death registration		NIHRD	2017
		7. On coordination of research and development in health sector		NIHRD	

No	STRATEGIC TARGET/PROGRAM	LEVEL/CONTENT OF REGULATION	REASONS FOR URGENT ISSUANCE OF REGULATION-NSPK	MAIN UNIT/WORKING UNIT	PLAN OF ISSUANCE YEAR
		8. On national policies for research and development in health sector		NIHRD	2016
		9. On the shipment of clinical specimen and MTA		NIHRD	2016
		10. On research of herbs using healthcare provision basis		NIHRD	2017
		11. on prioritized activities plan for research in health sector 2015-2019		NIHRD	2015
		12. On prioritized activities plan for research in health sector 2015-2019		NIHRD	2015
		13. On Council of Health Researcher's Ethic Council		NIHRD	2018
		14. On Health Researcher's competency standard		NIHRD	2018
10	The improvement of good and clean governance	1. Permenkes on the implementation of risk management in the Ministry of Health settings		Inspectorate General	2015
		2. Technical guidance on the implementation of risk management for working unit in the Ministry of Health settings		Inspectorate General	2016
		3. Guidance on risk management monitoring for APIP in the Ministry of Health settings		Inspectorate General	2016
11	The improvement of competency and performance of Ministry of Health's public officials	1. Government Regulation and Permenkes derivatives from Law Number 5/2014 on ASN (Aparatur Sipil Negara/government employee) related to P3K (pertolongan pertama pada kecelakaan/first aid)		Bureau of Personnel	2016
		2. Permenkes on the changes of position analysis (Anjab) especially related to the points of activities for doctor and nurse functional positions both at hospitals or KKP (Kantor Kesehatan Pelabuhan/Port Health Office)		Bureau of Personnel	2016

N0	STRATEGIC TARGET/PROGRAM	LEVEL/CONTENT OF REGULATION	REASONS FOR URGENT ISSUANCE OF REGULATION- NSPK	MAIN UNIT/WORKING UNIT	PLAN OF ISSUANCE YEAR
12	The improvement of the integrated Health Information System	1. Permenkes on health profile		Health Data and Information Center /HDIC	2017
		2. Permenkes on integration and management of health data bank, health data operator		HDIC	2016
		3. Management of health data and information in Fasyankes		HDIC	2015
		4. Permenkes on management of data and information in provincial, district/city levels		HDIC	2015
		5. Permenkes on roadmap of SIK		HDIC	2016

MINISTER OF HEALTH OF THE REPUBLIC OF INDONESIA

[SIGNED]

NILA FARID MOELOEK



APPENDIX IV

LIST OF ABBREVIATIONS



MINISTRY OF HEALTH
OF THE REPUBLIC OF INDONESIA

LIST OF ABBREVIATIONS

ABGC	: <i>Akademisi Swasta Pemerintah dan Masyarakat</i> Academy business government and community
AKB	: <i>Angka Kematian Bayi</i> Infant mortality rate
AKI	: <i>Angka Kematian Ibu</i> Maternal mortality rate
AKN	: <i>Angka Kematian Neonatal</i> Neonatal mortality rate
AKPN	: <i>Angka Kematian Pasca Neonatal</i> Post neonatal mortality rate
ALKES	: <i>Alat Kesehatan</i> Medical equipment
AMPL	: <i>Air Minum dan Penyehatan Lingkungan</i> Drinking water and environment sanitation
ANC	: Antenatal Care
ANJAB	: <i>Analisis Jabatan</i> The position analysis
APS	: <i>Angka Partisipasi Sekolah</i> School participation rate
API	: Annual Parasite Index
APIP	: <i>Aparatur Pengawas Internal Pemerintah</i> Public Officials for Internal Control of the Government
APBN	: <i>Anggaran Pendapatan dan Belanja Negara</i> State revenues and expenditure budget/State budget

APBD	: <i>Anggaran Pendapatan dan Belanja Daerah</i> Regional revenues and expenditure budget/Local budget
ARV	: Anti Retro Viral
ASI	: <i>Air Susu Ibu</i> Breast feeding
ASEAN	: Association of South East Asian Nation
BANSOS	: <i>Bantuan Sosial</i> Social assistance
BBLR	: <i>Berat Bayi Lahir Rendah</i> Underweight newborn
BIMTEK	: Bimbingan Teknis/Technical Guidance
BLUD	: <i>Badan Layanan Umum Daerah</i> Local public service agency
BMN	: <i>Barang Milik Negara</i> State-owned asset
BOK	: <i>Bantuan Operasional Kesehatan</i> Health operational assistance fund
BOR	: Bed Occupancy Rate
BPS	: <i>Badan Pusat Statistik</i> Central Bureau of Statistic
BSL	: Bio Safety Level
BTA	: <i>Basil Tahan Asam</i> Acid-alcohol resistant bacilli
BUK	: <i>Bina Upaya Kesehatan</i> Healthcare Provision
CDR	: Crude Detection Rate
CFR	: Case Fatality Rate
CSR	: Corporate Social Responsibility



MINISTRY OF HEALTH
OF THE REPUBLIC OF INDONESIA

CPAKB	: <i>Cara Produksi Alat Kesehatan yang Baik</i> Good practice manufacturing medical equipment
DAK	: <i>Dana Alokasi Khusus</i> Special allocation fund
DBD	: <i>Demam Berdarah Dengue</i> Dengue fever
DCEH	: Diseases Control and Environmental Health
DG	: Directorate General
DIKLAT NAKES	: <i>Pendidikan Pelatihan Tenaga Kesehatan</i> Health worker training
DKI	: <i>Daerah Khusus Ibukota</i> Special areas of the capital city
DLP	: <i>Dokter Layanan Primer</i> Primary healthcare doctors
DN	: <i>Dalam Negeri/Domestic</i>
DOEN	: <i>Daftar Obat Esensial Nasional</i> The list of national essential drugs
DTPK	: <i>Daerah Terpencil Perbatasan dan Kepulauan</i> Remote border areas and island
FASYANKES	: <i>Fasilitas Pelayanan Kesehatan</i> Health service facility
FCTC	: Framework Convention on Tobacco Control
FKTP	: <i>Fasilitas Kesehatan Tingkat Pertama</i> Primary healthcare
EWARS	: Early Warning and Response System
GMP	: Good Review Practices
GRTKF	: Genetic Resources, Traditional Knowledge and Folklores

HIV/AIDS	: Human Immunodeficiency Virus/Acquired Immune Deficiency Syndrome
HKI	: Helen Keller International
HP	: Health Provision
HRHDEI	Human Resource for Health Development and Empowerment Institute
HTA	: Health Technology Assessment
IHVCB	: Institute of Human Virology and Cancer Biology
IHR	: International Health Regulation
IKK	: <i>Indikator Kinerja Kegiatan</i> Activity Performance Indicator
IKP	: <i>Indikator Kinerja Program</i> Programme Performance Indicator
INA-CBG's	: Indonesia Case Base Groups
INPRES	: <i>Instruksi Presiden</i> Presidential Instruction
IMD	: <i>Inisiasi Menyusui Dini</i> Early breast feeding
IPG	: <i>Indeks Pemberdayaan Gender</i> Gender development index
IPWL	: <i>Institusi Penerima Wajib Lapo</i> The recipient obligation report institution
IR DBD	: Incidence Rate <i>Demam Berdarah Dengue</i> Incidence rate of dengue hemorrhagic fever
IT	: <i>Informasi Teknologi</i> Technology information
IUFD	: Intra Uterine Fetal Death
JKN	: <i>Jaminan Kesehatan Nasional</i> National Health Insurance

KEK	: <i>Kurang Energi Kronik</i> Chronic Energy Deficiency
KIA – KB	: <i>Kesehatan Ibu dan Anak - Keluarga Berencana</i> Maternal and child health/MCH
KLB	: <i>Kejadian Luar Biasa</i> Outbreaks
K/L	: <i>Kementerian/Lembaga</i> The Ministry/Agency
KIS	: <i>Kartu Indonesia Sehat</i> Healthy Indonesian Card
KEPRES	: <i>Kepetusan Presiden</i> Presidential Decree
KN	: <i>Kunjungan Neonatal</i> Neonatal visit
KSO	: <i>Kerja Sama Operasional</i> Operational cooperation
KTR	: <i>Kawasan Tanpa Rokok</i> Non smoking area
LANZIA	: <i>Lanjut Usia</i> Elderly people
LITBANG	: <i>Penelitian dan Pengembangan</i> Research and development
LHP	: <i>Laporan Hasil Pengawasan</i> Statutory audit report
LN	: <i>Luar Negeri/Abroad/Overseas</i>
MANLAK	: <i>Manajemen Pelaksanaan/Implementation</i> Guideline
MDGs	: Millenium Development Goals
MERS	: Middle East Respiratory Syndrom

MEA	: <i>Masyarakat Ekonomi ASEAN</i> ASEAN Economic of Community
MRA	: Mutual Recognition Agreement
MTBS	: <i>Manajemen Terpadu Balita Sakit</i> Integrated management childhood illness
MoU	: Memorandum of Understanding
MoH	: Ministry of Health
NAPZA	: <i>Narkotika Psikotropika dan Zat Adiktif</i> Narcotic psycho-tropical and other addictive Substances
NAKES	: <i>Tenaga Kesehatan</i> Human resource for health (HRH)
NSPK	: <i>Norma, Standar, Prosedur, dan Kriteria</i> Norm, standards, procedures and criteria
NTT	: <i>Nusa Tenggara Timur</i> East Nusa Tenggara
PBI	: <i>Penerima Bantuan Iuran</i> Contribution beneficiaries
PBF	: <i>Pedagang Besar Farmasi</i> Pharmacy whole supplier
P2B2	: <i>Pengendalian Penyakit Bersumber Binatang</i> Vector Borne Diseases Control
PCMD	: Pharmaceutical Care and Medical Devices
PD3I	: <i>Penyakit yang Dapat Dicegah Dengan</i> <i>Imunisasi</i> Diseases preventable by immunization
PERPRES	: <i>Peraturan Presiden</i> Presidential regulation
PERKESMAS	: <i>Perawatan Kesehatan Masyarakat</i> Public Health Nursing

PEMDA	: <i>Pemerintah Daerah</i> Local government
PHBS	: <i>Perilaku Hidup Bersih dan Sehat</i> Clean and healthy lifestyle behaviour
PHEIC	: Public Health Emergency of International Concern
P4K	: <i>Program Perencanaan Persalinan dan Pencegahan Komplikasi</i> Labor planning and complication prevention programme
PKPR	: <i>Pelayanan Kesehatan Peduli Remaja</i> Adolescent healthcare
PKB	: <i>Pelayanan Kesehatan Bergerak</i> Mobile healthcare
PKRT	: <i>Perbekalan Kesehatan Rumah Tangga</i> Household health supplies
POLTEKES	: <i>Politeknik Kesehatan</i> Health of polytechnic
POMP	: <i>Pemberian Obat Masal Pencegahan</i> Prevention of mass drug
PONED	: <i>Pelayanan Obstetri Neonatal Emergency Dasar</i> Basic emergency neonatal obstetric care
PONEK	: <i>Pelayanan Obstetri Neonatal Emergency Komprehensif</i> Comprehensive emergency neonatal obstetric care
POKJA	: <i>Kelompok Kerja</i> Working group
POSKESDES	: <i>Pos Kesehatan Desa</i> Village health post

POSBINDU	: <i>Pos Pembinaan Terpadu</i> Integrated development post
POSYANDU	: <i>Pos Pelayanan Terpadu</i> Integrated health post
PPI	: <i>Pencegahan dan Pengendalian Infeksi</i> Infection prevention and control
PPSDM	: <i>Pengembangan dan Pemberdayaan Sumber Daya Manusia</i> Human Resource for Health Development and Empowerment
PSP	: <i>Penetapan Status Penggunaan</i> The stipulation of utilization status
PTM	: <i>Pengendalian Penyakit Tidak Menular</i> Non-communicable diseases control
PTT	: <i>Pegawai Tidak Tetap</i> Contracted employees
PP	: <i>Peraturan Pemerintah</i> Government regulation
PPOK	: <i>Penyakit Paru Obstruktif Kronik</i> Chronic obstructive pulmonary disease
PP dan PL	: <i>Pengendalian Penyakit dan Penyehatan Lingkungan</i> Diseases Control and Environmental Health
PUSKESMAS	: <i>Pusat Kesehatan Masyarakat</i> Health Centre
RENSTRA	: <i>Rencana Strategis</i> Strategic plan
RIFASKES	: <i>Riset Fasilitas Kesehatan</i> Health facility research

RISKESDAS	: <i>Riset Kesehatan Dasar</i> Basic health research
RISKESNAS	: <i>Riset Kesehatan Nasional</i> National health research
RISTOJA	: <i>Riset Tumbuhan Obat dan Jamu</i> Research on medical plants and herbs
RIKUS	: <i>Riset Khusus Pencemaran Lingkungan</i> Specific research on environmental pollution
CEMARLING	
RPJPK	: <i>Rencana Pembangunan Jangka Panjang</i> <i>Bidang Kesehatan</i> Long Term Development Plan in Health Sector
RPJMN	: <i>Rencana Pembangunan Jangka Menengah</i> <i>Nasional</i> National Medium Term Development Plan
RPM	: <i>Rancangan Peraturan Menteri</i> Ministrial Regulation Draft
RENSTRA	: <i>Rencana Strategis</i> Strategic Plan
RENGUNAKES	: <i>Perencanaan Pendayagunaan Tenaga</i> <i>Kesehatan</i> Utilization Health Worker of Planning
RS	: <i>Rumah Sakit</i> Hospital
RSU	: <i>Rumah Sakit Umum</i> General hospitals
RSUD	: <i>Rumah Sakit Umum Daerah</i> Local hospital
RSK	: <i>Rumah Sakit Khusus</i> Specialized hospitals

R.PER	: <i>Rancangan Peraturan</i> Regulation draft
R.PERMENKES	: <i>Rancangan Peraturan Menteri Kesehatan</i> Regulation draft of the Minister of Health
R.KEPMENKES	: <i>Rancangan Keputusan Menteri Kesehatan</i> Draft of Minister of Health Decree
RPM	: <i>Rancangan Peraturan Menteri</i> Draft of Ministerial Regulation
RPP	: <i>Rancangan Peraturan Pemerintah</i> Draft of Government Regulation
RUU	: <i>Rancangan Undang-undang</i> Draft of Law
SAP	: <i>Sistem Akutansi Pemerintah</i> Governmental accounting system
SARS	: Severe Acute Respiratory Syndrome
SATKER	: <i>Satuan Kerja</i> Implementing Unit
SAKIP	: <i>Sistem Akuntabilitas Kinerja Instansi</i> <i>Pemerintah</i> Performance accountability system for government institution
SBS	: <i>Stop Buang Air Besar Sembarangan</i> Stop improper defecation
SDM	: <i>Sumber Daya Manusia</i> Human resources
SDGs	: Sustainable Development Goals
SDKI	: <i>Survei Demografi dan Kesehatan Indonesia</i> Indonesia Survey of Demographic and Health
SIK	: <i>Sistem Informasi Kesehatan</i> Health Information System

SIP	: <i>Sistem Informasi Puskesmas</i> Health Centre of Information System
SJSN	: <i>Sistem Jaminan Sosial Nasional</i> National Social Security System
SKDR	: <i>Sistem Kewaspadaan Dini dan Respon</i> Early warning and response system
SKD – KLB	: <i>Sistem Kewaspadaan Dini - Kejadian Luar Biasa</i> Early warning system and outbreak
SKPD	: <i>Satuan Kerja Perangkat Daerah</i> Regional working unit
SMA/SMK/MA	: <i>Sekolah Menengah Atas/Sekolah Menengah Kejuruan/Madrasah Aliyah</i> Senior High School/Vocational Senior High School/Islamic Senior High School
SPA	: <i>Sarana Prasarana dan Alat</i> Facilities and equipment
SPPN	: <i>Sistem Perencanaan Pembangunan Nasional</i> National Development Planning System
SPM	: <i>Standar Pelayanan Minimal</i> Minimum Standard Service
SP	: <i>Sensus Penduduk</i> Population census
SS	: <i>Sasaran Strategis</i> Strategic Target
STBM	: <i>Sanitasi Total Berbasis Masyarakat</i> Community Based Total Sanitation
STR	: <i>Surat Tanda Registrasi</i> Letter of registration
SUN	: <i>Scaling Up Nutrition</i>

SUSENAS	: <i>Survei Sosial Ekonomi Nasional</i> National Social Economic Survey
TB	: <i>Tuberkulosis</i> Tuberculosis
TKI	: <i>Tenaga Kerja Indonesia</i> Indonesian migrant workers
TK/RA	: <i>Taman Kanak-kanak/Raudhatul Anfal</i> Kindergarten/Islamic School
TK-WNA	: <i>Tenaga Kerja Warga Negara Asing</i> Employment of foreigners
TK-WNI	: <i>Tenaga Kerja Wanita Negara Indonesia</i> Employment of Indonesian Women
TPI	: <i>Tempat Pelelangan Ikan</i> The Fish Auction Place
TPM	: <i>Tempat Pengelolaan Makanan</i> Food processing facility
TP	: <i>Tugas Pembantuan</i> Co-Administration
TSR	: Treatment Success Rate
T/ST	: <i>Terpencil/ Sangat Terpencil</i> Remote areas
TTD	: <i>Tablet Tambah Darah</i> Iron supplementary tablet
TT	: <i>Tempat Tidur</i> The beds
TTU	: <i>Tempat-tempat Umum</i> Public Facilities
UHC	: Universal Health Coverage
UKK	: <i>Upaya Kesehatan Kerja</i> Occupational healthcare provision



MINISTRY OF HEALTH
OF THE REPUBLIC OF INDONESIA

UKS	: <i>Upaya Kesehatan Sekolah</i> Occupational health school
UKJBM	: <i>Upaya Kesehatan Jiwa Berbasis Masyarakat</i> Developing community based mental health
UKBM	: <i>Upaya Kesehatan Bersumberdaya Masyarakat</i> Community based healthcare
UU	: <i>Undang-undang</i> Law
UPT	: <i>Unit Pelaksana Teknis</i> Technical Unit
UTD	: <i>Unit Transfusi Darah</i> Blood Transfusion Unit
WHO	: World Health Organization
WTO	: World Trade Organization
WTP	: <i>Wajar Tanpa Pengecualian</i> Unqualified Audit Opinion



MINISTRY OF HEALTH
OF THE REPUBLIC OF INDONESIA



MINISTRY OF HEALTH
OF THE REPUBLIC OF INDONESIA

DIRECTORS

Nila Farid Moeloek, Nafsiah Mboi, Untung Suseno
Sutarjo, Supriyantoro, Yudhi Prayudha Ishak Djuarsa,
Akmal Taher, Anung Sugihantono, Maura Linda
Sitanggang, M.Subuh, Usman Sumantri, Tjandra Yoga
Aditama, Tini Suryanti Suhandi, Tritarayati,
Henni Setiawati Agus Purwadianto, Bambang Sarjono

CONTRIBUTORS

Pattiselanno Robert Johan, Wiwik Widarti, Barlian,
Embry Netty, Oscar Primadi, Lily Sriwahyuni Sulistyowati,
Donald Pardede, Murti Utami, Trisha Wahjudi Putri,
Fidiansjah, Budi Dhewajani, S.R.Mustikowati,
Wiyono Budiharjo, Heru Arnowo, Rahmaniar Brahim,
Mulyanto, Wayan Rai Suarthana, Kuwat Sri Hudoyo,
Gita Maya Koemara Sakti Soepono, Elizabeth Jane
Soepardi, Dedi Kuswenda, Doddy Izwardy, Muchtaruddin
Mansyur, Nurshanty S.Andi Sapada, Kartini Rustandi,
Chairul Radjab Nasution, Suhartati,
Deddy Tedjasukmana Basumi, Eka Viora, Purwadi,
Engko Sosialine Magdalene, Arianti Anaya Indradjidi,
Dettie Yuliati, Desak Made Wismarini, Ekowati Rahajeng,
Wilfried Hasiholan Purba, Ria Soekarno, Agus Suprpto,
Pretty Multihartina, Dede Anwar Musadad, Siswanto,
Asjikin Iman Hidayat Dachlan, Suhardjono, Kirana
Pritasari, Meinarwati Andi Saguni, Setyo Budi Hartono,
Azhar Jaya, Risca Ardhyaningtyas, Susiyo Luchito, Yudhi
Pramono, Zan Susilo, Johan Safari



PREPARATORY TEAM

Slamet, IGM. Wirabrata, Dede Mulyadi,
Grace Lovita Tewu,
Gede Ketut Wirakamboja,
Taufik, N. A. Ma'ruf, Asep Zaenal Mustofa,
Bambang Setiadji, Rahmat Kurniadi,
Iqbal Djakaria, Mayang Sari,
Roy Himawan,
Ockti Palupi Rahayuningtyas,
Ida Ayu Made Rai Astuti,
Anita Dwi Ingati, Tina Safaria,
Dian Shinta Fitriyanti, Muhammad Iqbal

TRANSLATORS

Anita Dwi Ingati, Okta Iskandaria, Dian Shinta Fitriyanti